Application Submitted to HRSA

Submitted to HRSA

Organization: THE REGENTS OF THE UNIVERSITY OF COLORADO, AURORA, Colorado

Grants.gov Tracking Number: GRANT14707548

EHB Application Number: 247833

Grant Number: 4 T73MC11044-18-03

Funding Opportunity Number: HRSA-26-019

Received Date: 7/9/2026 5:37:14 PM

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Table Of Contents

1. SF-424 RR Application for Federal Assistance (Grants.gov PDF)

2. Project Description

3. Project Abstract Summary (Grants.gov PDF)

4. RR Senior/Key Person Profile Expanded (Grants.gov PDF)

5. PD/PI Biographical Sketches

6. Senior/Key Person Biographical Sketches

7. Senior/Key Person Biographical Sketches-2

8. Project/Performance Site Location(s) (Grants.gov PDF)

9. SF-424RR Budget (Grants.gov PDF)

10. RR Other Project Information

11. Project Narrative

12. Attachment 1 (AttachmentForm_1_2-ATT1-UCO.T73MC11044.ATTACHMENT 1-Workplan.pdf)

13. Attachment 2 (AttachmentForm_1_2-ATT2-UCO.T73MC11044.ATTACHMENT 2.pdf)

14. Attachment 3 (AttachmentForm_1_2-ATT3-UCO.T73MC11044.ATTACHMENT 3.pdf)

15. Attachment 4 (AttachmentForm_1_2-ATT4-UCO.T73MC11044.ATTACHMENT 4-Org Chart.pdf)

16. Attachment 5 (AttachmentForm_1_2-ATT5-UCO.T73MC11044.ATTACHMENT 5.pdf)

17. Attachment 6 (AttachmentForm_1_2-ATT6-UCO.T73MC11044.Attachment 6 -Funding Tier.pdf)

18. Attachment 7 (AttachmentForm_1_2-ATT7-UCO.T73MC11044.ATTACHMENT 7 - Supplement.pdf)

19. Other (RR_Budget_3_0-UCO.T73MC11044.Budget Narrative -2026-31.pdf)

OMB Number: 4040-0001
Expiration Date: 01/31/2029

APPLICATION FOR FEDERAL ASSISTANCE

SF 424 (R&R)

3. DATE RECEIVED BY STATE

State Application Identifier

1. TYPE OF SUBMISSION

☐ Pre-application ☒ Application ☐ Changed/Corrected Application

4. a. Federal Identifier

b. Agency Routing Identifier

c. Previous Grants.gov Tracking ID

2. DATE SUBMITTED

07/09/2026

Applicant Identifier

264632

5. APPLICANT INFORMATION

UEI: MW8JHK6ZYEX8

Legal Name: University of Colorado Denver

Department: 21458--SOM-PEDS Dev Peds-SpPgms

Division: None

Street1: Mail Stop F428, Anschutz Medical Campus

Street2: Building 500, 13001 East 17th Place, Room W1124

City: Aurora

County / Parish: Adams

State: CO: Colorado

Province:

Country: USA: UNITED STATES

ZIP / Postal Code: 80045-2571

Person to be contacted on matters involving this application

Prefix:

First Name: Garrett

Middle Name: M

Last Name: Steed

Suffix:

Position/Title: Research Services Manager

Street1: Mail Stop F428, Anschutz Medical Campus

Street2: Building 500, 13001 East 17th Place, Room W1124

City: Aurora

County / Parish: Adams

State: CO: Colorado

Province:

Country: USA: UNITED STATES

ZIP / Postal Code: 80045-2571

Phone Number: 303/724-0090

Fax Number: 303-724-0814

Email: xenia@ucdenver.edu

6. EMPLOYER IDENTIFICATION *(EIN) or (TIN):* 1846000555A7

7. TYPE OF APPLICANT: H: Public/State Controlled Institution of Higher Education

Other (Specify):

Small Business Organization Type ☐ Women Owned ☐ Socially and Economically Disadvantaged

8. TYPE OF APPLICATION:

☒ New ☐ Resubmission

☐ Renewal ☐ Continuation ☐ Revision

If Revision, mark appropriate box(es).

☐ A. Increase Award ☐ B. Decrease Award ☐ C. Increase Duration ☐ D. Decrease Duration

☐ E. Other (specify):

Is this application being submitted to other agencies? Yes ☐ No ☒ What other Agencies?

9. NAME OF FEDERAL AGENCY:

Health Resources and Services Administration

10. ASSISTANCE LISTING NUMBER: 93.877

ASSISTANCE LISTING TITLE:

11. DESCRIPTIVE TITLE OF APPLICANT'S PROJECT:

LEND and LEND Audiology Supplement

12. PROPOSED PROJECT:

Start Date: 09/30/2026

Ending Date: 06/30/2031

13. CONGRESSIONAL DISTRICT OF APPLICANT

CO-006

14. PROJECT DIRECTOR/PRINCIPAL INVESTIGATOR CONTACT INFORMATION

Prefix: Dr. First Name: Judith Middle Name: A

Last Name: Reaven Suffix: Ph.D.

Position/Title: Professor

Organization Name: University of Colorado Denver

Department: 21458--SOM-PEDS Dev Peds-SpPgms

Division: None

Street1: CHCO Gary Pavilion Building

Street2: 13123 E. 16th Ave

City: Aurora County / Parish: Arapahoe

State: CO: Colorado Province:

Country: USA: UNITED STATES ZIP / Postal Code: 80045-7016

Phone Number: 303/724-7646 Fax Number: 303 724-7664

Email: judy.reaven@cuanschutz.edu

15. ESTIMATED PROJECT FUNDING

a. Total Federal Funds Requested	3,205,000.00
b. Total Non-Federal Funds	0.00
c. Total Federal & Non-Federal Funds	3,205,000.00
d. Estimated Program Income	0.00

16. IS APPLICATION SUBJECT TO REVIEW BY STATE EXECUTIVE ORDER 12372 PROCESS?

a. YES ☐ THIS PREAPPLICATION/APPLICATION WAS MADE AVAILABLE TO THE STATE EXECUTIVE ORDER 12372 PROCESS FOR REVIEW ON:

DATE:

b. NO ☒ PROGRAM IS NOT COVERED BY E.O. 12372; OR

☐ PROGRAM HAS NOT BEEN SELECTED BY STATE FOR REVIEW

17. By signing this application, I certify (1) to the statements contained in the list of certifications* and (2) that the statements herein are true, complete and accurate to the best of my knowledge. I also provide the required assurances * and agree to comply with any resulting terms if I accept an award. I am aware that any false, fictitious. or fraudulent statements or claims may subject me to criminal, civil, or administrative penalties. (U.S. Code, Title 18, Section 1001)

☒ **I agree**

**The list of certifications and assurances, or an Internet site where you may obtain this list, is contained in the announcement or agency specific instructions.*

18. SFLLL (Disclosure of Lobbying Activities) or other Explanatory Documentation

Add Attachment | Delete Attachment | View Attachment

19. Authorized Representative

Prefix: First Name: Muawia Middle Name: I

Last Name: Obeid Suffix:

Position/Title: Research Services Manager

Organization: University of Colorado Denver

Department: 20663--ADM AVCFC OGC Administration

Division:

Street1: Mail Stop F428, Anschutz Medical Campus

Street2: Building 500, 13001 East 17th Place, Room W1124

City: Aurora County / Parish: Adams

State: CO: Colorado Province:

Country: USA: UNITED STATES ZIP / Postal Code: 80045-2571

Phone Number: 303/724-6676 Fax Number: 303-724-0814

Email: xenia@ucdenver.edu

Signature of Authorized Representative	Date Signed
Muawia I Obeid	07/09/2026

20. Pre-application Add Attachment | Delete Attachment | View Attachment

21. Cover Letter Attachment Add Attachment | Delete Attachment | View Attachment

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OMB Number: 4040-0019
Expiration Date: 06/30/2028

Project Abstract Summary

This Project Abstract Summary form must be submitted or the application will be considered incomplete. Ensure the Project Abstract field succinctly describes the project in plain language that the public can understand and use without the full proposal. Use 4,000 characters or less. Do not include personally identifiable, sensitive or proprietary information. Refer to Agency instructions for any additional Project Abstract field requirements. If the application is funded, your project abstract information (as submitted) will be made available to public websites and/or databases including USAspending.gov.

Funding Opportunity Number

HRSA-26-019

Assistance Listing Number(s):

Applicant Name

Judith Reaven

Descriptive Title of Applicant's Project

LEND and LEND Audiology Supplement

Project Abstract

Population Served and Program Overview: JFK Partners' LEND serves individuals with autism and developmental disabilities (including dual diagnoses such as Deaf or Hard of Hearing plus autism) and their families, focusing on improving access to evidence-based screening, diagnosis, interventions and other supports by training trainees, the workforce, individuals with disabilities and their families, and strengthening systems of care through collaboration across the disability community.
Needs Addressed: Children, youth, and adults with autism and developmental disabilities and their families need disability-informed leaders, a workforce competent in evidence-based skills in disability, and respectful, evidence-informed systems of care that increase access to screening, diagnosis, interventions, and other supports to improve their health, well-being, informed decision-making, and meaningful community inclusion.
Proposed Services: Annually, JFK Partners will address three goals that respond to Colorado needs. GOAL A) Prepare health and other professionals, individuals with disabilities, and family trainees in interdisciplinary person-/family-centered approaches, including 18 long-, 10 medium-, and 100 short-term trainees, educated for 300 plus, 150-299, and 40 or fewer hours, respectively. The complete LEND curriculum includes didactic courses and clinical and other practical experiences focused on leadership competencies, autism and disability content and lifespan issues, disability policy, community health factors, emerging issues and public health threats, and evidence-based screening, diagnosis, interventions, and supports. Medium- and short-term trainees select LEND components that meet their specific training needs. GOAL B) Strengthen the developmental disability workforce and systems. JFK Partners will annually: a) Host at least 5 continuing education (CE) events for about 200 participants; b) provide technical assistance (TA) to at least 6 organizations; c) share project findings, products, and resources to inform partners and the community in formats accessible to the lay public. These efforts will be developed in collaboration with local, state, regional, and national partners, including family organizations, schools, public health programs, and national disability networks. GOAL C) Administer responsible stewardship of LEND resources to inform data-driven decision-making to maximize LEND impact. JFK Partners will maintain accountability in grant management by conducting accurate data collection and performance reporting, comprehensive program evaluation and continuous quality improvement strategies, and careful budget monitoring. A LEND Audiology Supplement increases training CE/TA activities and numbers.
Collaboration and Partnership: JFK Partners will implement the above goals in collaboration with Colorado universities, Title V maternal and child health agencies and other organizations (local to national levels), and community partner networks including individuals with disabilities and families. Partners will: help identify emerging or unmet needs; support identification of potential trainees, expert speakers and panelists, and research participants; and collaborate in design, implementation, and evaluation of projects that advance shared missions.
Program Evaluation: JFK Partners will annually: Assess project progress and outcomes by 1) monitoring project work plan implementation, 2) managing an internal data system, comprehensive evaluation, continuing quality improvement (QI) strategies, and budget monitoring. Data sources include: trainee data, evaluations, and graduate outcomes; continuing education and technical assistance outcomes and satisfaction and community ratings; and QI data, numbers and accessibility ratings of project products and resources, timeliness of all required reports, and project expenditures.

OMB Number: 4040-0001
Expiration Date: 01/31/2029

RESEARCH & RELATED Senior/Key Person Profile (Expanded)

PROFILE - Project Director/Principal Investigator

Prefix: Dr.
* First Name: Judith
Middle Name: A
* Last Name: Reaven
Suffix: Ph.D.
Position/Title: Professor
Department: 21458--SOM-PEDS Dev Peds-SpPgms
Organization Name: University of Colorado Denver
Division: None
* Street1: CHCO Gary Pavilion Building
Street2: 13123 E. 16th Ave
* City: Aurora
County/ Parish: Arapahoe
* State: CO: Colorado
Province:
* Country: USA: UNITED STATES
* Zip / Postal Code: 80045-7016
* Phone Number: 303/724-7646
Fax Number: 303 724-7664
* E-Mail: judy.reaven@cuanschutz.edu

Credential, e.g., agency login:

*** Project Role:** PD/PI
Other Project Role Category:

Degree Type:
Degree Year:

***Attach Biographical Sketch**	1-REAVEN - PI.pdf	Add Attachment	Delete Attachment	View Attachment
Attach Current & Pending Support		Add Attachment	Delete Attachment	View Attachment

PROFILE - Senior/Key Person 1

Prefix:
* First Name: Teresa
Middle Name: R
* Last Name: Charlifue-Smith
Suffix:
Position/Title: Senior Instructor
Department: 21458--SOM-PEDS Dev Peds-SpPgms
Organization Name: University of Colorado Denver
Division: None
* Street1: 13065 East 17th Avenue
Street2:
* City: Aurora
County/ Parish: Adams
* State: CO: Colorado
Province:
* Country: USA: UNITED STATES
* Zip / Postal Code: 80045-2532
* Phone Number: 303/724-7632
Fax Number:
* E-Mail: renee.charlifue-smith@cuanschutz.edu

Credential, e.g., agency login:

*** Project Role:** Other (Specify)
Other Project Role Category: Discipline Director - Speech / Training

Degree Type:
Degree Year:

Attach Biographical Sketch	2-CHARLIFUE-SMITH.pdf	Add Attachment	Delete Attachment	View Attachment
Attach Current & Pending Support		Add Attachment	Delete Attachment	View Attachment

RESEARCH & RELATED Senior/Key Person Profile (Expanded)

PROFILE - Senior/Key Person 2

Prefix:
* First Name: Dina
Middle Name: D
* Last Name: Johnson
Suffix:
Position/Title: Senior Instructor
Department: 21458--SOM-PEDS Dev Peds-SpPgms
Organization Name: University of Colorado Denver
Division: None
* Street1: 13065 East 17th Avenue
Street2:
* City: Aurora
County/ Parish: Adams
* State: CO: Colorado
Province:
* Country: USA: UNITED STATES
* Zip / Postal Code: 80045-2532
* Phone Number: 303/724-7673
Fax Number:
* E-Mail: dina.johnson@cuanschutz.edu

Credential, e.g., agency login:

*** Project Role:** Other (Specify)
Other Project Role Category: Training Co-Director - Ed/SpEd

Degree Type:
Degree Year:

Attach Biographical Sketch	3-JOHNSON.pdf	Add Attachment	Delete Attachment	View Attachment
Attach Current & Pending Support		Add Attachment	Delete Attachment	View Attachment

NAME Reaven, Judy	POSITION TITLE Professor of Psychiatry/Pediatrics LEND/UCEDD Director; Director of Psychology

EDUCATION/TRAINING

INSTITUTION AND LOCATION	DEGREE *(if applicable)*	Completion Date MM/YYYY	FIELD OF STUDY
Vanderbilt University	B.A.	05/1980	Psychology
University of Missouri – Columbia	M.A., Ph.D.	1983, 1985	Clinical Psychology
Denver Health (pre-doctoral internships)	N/A	1984-1985	Clinical Psychology internship
University of Colorado School of Medicine	Post-doc	1985-1987	Clinical Psychology/Developmental Disabilities

A. **Personal Statement.** I have worked in the field of developmental disabilities as clinician, teacher and researcher for nearly 40 years. I have supervised/mentored over 100 psychology graduate students, pre-doctoral interns, and post-doctoral fellows. My training has been exclusively within an interdisciplinary model, and I teach students from all disciplines from this perspective. I have specific expertise in autism and have received private and federal funding (as PI) to develop and implement an evidence-based intervention for autistic youth with anxiety. My extensive clinical and academic experiences in autism/IDD have led me to hold the positions of Interim LEND/UCEDD Director and Director of Psychology. I am proud to oversee the LEND program for the foreseeable future and will work to address the unique health needs of individuals and communities as we train the next cohort of leaders.

B. **Positions and Honors.** *Honors*: 1) 2006 Recognition and Contribution to the Autism Community, Organization for Autism Research (OAR), Washington, D.C.; 2) 2007, 2010, 2012, 2016: Dane G. Prugh Award for Outstanding and Inspirational Teaching in Child Psychiatry (Four-time winner); 3) 2019 - Transformative Contribution Award: In recognition of the novelty and broad enduring impact of Dr. Reaven's research on the field of ASDD, presented by ABCT/ASDD Special Interest Group

Positions:

2001 -2004 Senior Instructor, Director of the Autism and Developmental Disabilities Clinic, (A & DD); UC-SOM
2004-2011 Assistant Professor, Director of the A & DD Clinic; UC-SOM
2011- 2019 Associate Professor, Director of the A & DD Clinic, UC-SOM
2019- Professor of Psychiatry and Pediatrics, Director of the A & DD Clinic; UC-SOM
2015- Associate Director; Director of Psychology - JFK Partners
2025- Interim LEND/UCEDD Director – JFK Partners

C**. Other Support.**

Ongoing Research Support

IDD-2022C1-26330 Patient-Centered Outcomes Research Institute (PCORI), **Reaven (PI)** 4/1/2023-2/28/2028
90DDUCO16-01-00 AIDD/ACL, **Reaven (PI)**, Director of JFK Partners 7/01/2022 – 6/30/2027
T73MC11044-18-00 HRSA/MCHB, **Reaven (PI):** LEND Psychology Director 7/1/2021-6/30/2026

Recently Completed Research Support

W81XWH-20-ARP-CTRA Department of Defense (DOD) Blakeley-Smith (PI): **Role – Co-I** 09/1/17 – 12/31/2020

D. **Contributions to Science.** My most significant contribution to science has been the development and implementation of a group cognitive behavior therapy (Facing Your Fears program) for autistic youth with anxiety.

1. **Reaven, J.**, Blakeley-Smith, A., Culhane-Shelburne, K., & Hepburn, S. (2012). A randomized trial: Group cognitive behavior therapy for children with high-functioning autism spectrum disorders and anxiety. Journal of Child Psychology and Psychiatry, 53, 410-419. PMCID: PMC4392045
2. **Reaven, J.,** Moody, E., Grofer Klinger, L., Keefer, A., Duncan, A., O'Kelley, S., Meyer, A., Hepburn, S., & Blakeley-Smith, A. (2018). Training Clinicians to Deliver Group CBT to Manage Anxiety in Youth with ASD: Results of a Multi-Site Trial. Journal of Consulting and Clinical Psychology, 86(3):205-217. DOI: 10.1037/ccp0000285; PMID: 29504790.
3. **Reaven, J**., Meyer, A.T., Pickard, K., Boles, R., Hayutin, L., Middleton, C., Reyes, N., Hepburn, S., Tanda, T., Stahmer, A., & Blakeley-Smith, A. (2022). Increasing Access and Reach: Implementing School-Based CBT for Anxiety in Students with ASD or Suspected ASD. Evidence-based practice in child and adolescent mental health. 7:1, 56-75, http://dx.doi.org/10.1080/23794925.2021.1941430.
4.Reaven, J., Pickard, K., Meyer, A., Hayutin, L., Middleton, C., Reyes, N., Tanda, T., Stahmer, A., Blakeley-Smith, A., & Boles, R. (2024). Implementing school-based CBT for anxiety in students with autism or suspected autism via a train-the-trainer approach: results from a clustered randomized trial. Autism, 28(2), 484-497. https://doi.org/10.1177/13623613231175951PMID: 37208894.
5.Reaven, J., Cosgrove, K., Losh, A., Nickles, S., Kerns, C., Pickard, K., Blakeley-Smith, A., Hayutin, L., Meyer, A., Middleton, C., Reyes, N., & Boles, R. (2025) Facing your fears in schools: Using the ADIS/ASA to characterize anxiety and intervention outcomes in students with autism or suspected autism. Frontiers in Psychiatry, *16,* doi: 10.3389/fpsyt.2025.1569435

NAME Charlifue-Smith, Reneé	POSITION TITLE Assistant Professor of Pediatrics, Interdisciplinary Training Co-Director, and Speech-Language Pathology Discipline Director

EDUCATION/TRAINING

INSTITUTION AND LOCATION	DEGREE *(if applicable)*	Completion Date MM/YYYY	FIELD OF STUDY
Colorado State University Pueblo	B.S.	12/1986	Communication Disorders
Ohio University	M.A.	6/1990	Speech-Language Pathology
University of Colorado School of Medicine	Clinical Fellowship	1990-91	Speech-Language Pathology
Irving Harris Child Development & Infant Mental Health, University of Colorado, Department of Psychiatry	Fellowship	2004-05	Child Development and Infant Mental Health
University of Colorado Denver	Graduate Certificate	2011-12	Applied Behavior Analysis
Early Childhood Intervention Doctoral Consortium, Office of Special Education Programs, U.S. Department of Education	Fellowship		
University of Colorado Denver, School of Education and Human Development	Ph.D. Candidate	2020-present	Inclusive Early Childhood Education

A. **Personal Statement.** I have been a clinician, educator, and researcher for over 35 years working with individuals with intellectual and developmental disabilities including autism at JFK Partners, Colorado's University Center for Excellence in Developmental Disabilities. I am the Co-Director of Interdisciplinary Training and the Speech-Language Pathology (SLP) discipline director on the Leadership Education in Neurodevelopmental Disabilities (LEND), Maternal Child Health Bureau Grant, and the Early Intervention Team Director of the training and services teams for the past 30 years. This work includes extensive assessment and intervention experience with individuals with autism or other IDD. I have had various roles on a variety of federally funded grants including as an evaluator for the Study to Explore Early Development (SEED II). I also provide assessment and intervention services in community settings and at Children's Hospital Colorado. My areas of specialization include assessment, routines-based intervention, teaming, autism, and positive behavior support.

B. **Positions and Honors.**

Positions:

1991-Present Faculty, Department of Pediatrics, School of Medicine, University of Colorado School of Medicine
1991-1994 State Classified Staff
1994-2007 Instructor
2007-2025 Senior Research Instructor
2025-Present Assistant Professor

Honors and Advisory Committee Positions:

2012 Senior Instructor with Distinction Award, Department of Pediatrics, University of Colorado School of Medicine
2015 Leadership Academy for Cultural Diversity and Cultural and Linguistic Competence, Georgetown University National Center for Cultural Competence

Key Professional Positions:

1995-1996 Elected Board Member, Vice-President Elect for Education, Colorado Speech- Language-Hearing Association
- Chair, Annual Conference

2016-2019 Elected Board Member, Council on Academic Accreditation (CAA)in Audiology and Speech-Language Pathology (SLP), American Speech-Language-Hearing Association (ASHA)
2019-2022 Elected Board Member, Vice Chair, Multicultural Council, Association of University Centers on Disabilities
2020-2026 Governor's Appointment, Colorado Traumatic Brain Injury Trust Fund Board, MINDSOURCE Brain Injury Network, Colorado Department of Human Services
2024-Present Selected Co-Chair, Early Childhood Special Interest Group, Association of University Centers on Disabilities (AUCD)

C. Other Support.

90DDUCO16 AIDD/ACL, Reaven (PI), UCE Grant, Interdisciplinary Training Co-Director, Speech-Language Pathology Discipline Director, 7/01/2022 – 6/30/2027

T73MC11044 HRSA/MCHB, Reaven (PI), LEND, Interdisciplinary Training Co-Director, Speech-Language Pathology Discipline Director, 7/1/2021-6/30/2026

NAME Johnson, Dina	POSITION TITLE Senior Instructor of Pediatrics, Interdisciplinary Training Co-Director, and SpEd/Education Discipline Director

EDUCATION/TRAINING

INSTITUTION AND LOCATION	DEGREE *(if applicable)*	Completion Date MM/YYYY	FIELD OF STUDY
Colorado State University Fort Collins, CO	B.S.	08/1991	Human Development and Family Studies, Childhood Concentration
University of Northern Colorado Greeley, CO	teaching certificate	05/1993	Elementary Education, Colorado Teaching Certification
University of Colorado Denver Denver, CO	M.A.	05/2012	School of Education & Human Development Instructional and Learning Technology eLearning Design and Implementation

A. **Personal Statement.** My work is centered on preparing interdisciplinary leaders to improve systems of care for children and youth with autism and other developmental disabilities and their families. In my roles as LEND Training Co-Director, Education/Special Education Discipline Director, and Dissemination Director at JFK Partners, I support the full continuum of trainee development, including recruitment, curriculum design, teaching, mentoring, individualized training plans, dissemination, and program reporting. I am especially committed to training graduate and postgraduate students, practicing professionals, individuals with disabilities, and family members through learning experiences that combine academic instruction, clinical experiences, leadership development, and community engagement. This training is grounded in interdisciplinary collaboration, person- and family-centered practice, and systems improvement for individuals with disabilities across the lifespan. I bring more than 30 years of experience working with interdisciplinary trainees at JFK Partners. I have worked to strengthen accessible and inclusive training pathways that build workforce capacity and prepare trainees to lead meaningful change in partnership with communities.

B. **Positions.**

1998-2015 Training Coordinator, Department of Pediatrics, University of Colorado School of Medicine
2015-2018 Associate Director of Interdisciplinary Training and Dissemination Director, Instructor of Pediatrics
2018-2021 Interdisciplinary Training Co-Director and Dissemination Director, Instructor of Pediatrics
2021-current Interdisciplinary Training Co-Director and Dissemination Director, Senior Instructor of Pediatrics

Committees
2019-current Governor Appointed Member, Colorado Developmental Disabilities Council
2021-2026 Planning & Grants Committee Chair, Colorado Developmental Disabilities Council
2024-present AUCD Neurodevelopmental Disabilities (NDD) Core Curriculum Editorial Board
2018-present AUCD LEND Self-Advocacy Network (LSDN) Workgroup Member
2015-present PacWest Regional LEND Consortium Member

C. Other Support.

University Centers for Excellence in Developmental Disabilities Education, Research, and Service Core. U.S. Department of Health and Human Services, Administration for Children and Families, Administration on Developmental Disabilities. 07/01/2022-06/30/2027, Award 90DDUC0106-01-00 (Interdisciplinary Training Co-Director, Dissemination Director, NIRS Data Coordinator)

Leadership Education in Neurodevelopmental Disabilities (LEND) and Audiology Supplement. U.S. Department of Health and Human Services, Bureau of Maternal Child Health, 7/1/21-6/30/26, Award # 1 T73 MC11044 (Interdisciplinary Training Co-Director)

Collaborative Research Entity, Autism Intervention Research Network on Physical Health (AIR-P) Association of University Centers on Disabilities. 3/1/21-8/31/25, Subaward Agreement #10-21-8840, HRSA Prime Award #6 UT2MC39440-01 (Dissemination)

Completed:

PacWest LEND ITAC Learning Modules, Group Opportunities to Advance Learning (GOAL), Interdisciplinary Technical Assistance Center on Autism and Developmental Disabilities (ITAC), Association of University Centers on Disabilities (AUCD). 2/1/21-6/30/21, MOU (Principal Investigator)

Project/Performance Site Location(s)

Project/Performance Site Primary Location

☐ I am submitting an application as an individual, and not on behalf of a company, state, local or tribal government, academia, or other type of organization.

Organization Name: University of Colorado Denver

UEI: MW8JHK6ZYEX8

* Street1: 12700 East 19th Place

Street2:

* City: Aurora

County: Adams

* State: CO: Colorado

Province:

* Country: USA: UNITED STATES

* ZIP / Postal Code: 80045-2560

* Project/ Performance Site Congressional District: CO-006

Project/Performance Site Location 1

☐ I am submitting an application as an individual, and not on behalf of a company, state, local or tribal government, academia, or other type of organization.

Organization Name:

UEI:

* Street1:

Street2:

* City:

County:

* State:

Province:

* Country: USA: UNITED STATES

* ZIP / Postal Code:

* Project/ Performance Site Congressional District:

Additional Location(s)

Add Attachment | Delete Attachment | View Attachment

RESEARCH & RELATED BUDGET - Budget Period 1

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: MW8JHK6ZYEX8 **Enter name of Organization:** University of Colorado Denver

Budget Type: ☒ Project ☐ Subaward/Consortium **Budget Period:** 1 **Start Date:** 09/30/2026 **End Date:** 06/30/2027

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
Dr.	Judith	A	Reaven	Ph.D.	203,832.00	4.20			71,341.00	19,719.00	91,060.00
Project Role:	PD/PI										
	Teresa	R	Charlifue-Smith		111,500.00	1.20			11,150.00	3,082.00	14,232.00
Project Role:	Discipline Director - Speech / Training Co-Director										
	Dina	D	Johnson		95,731.00	4.80			38,292.00	10,584.00	48,876.00
Project Role:	Training Co-Director - Ed/SpEd										

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 154,168.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
6	Faculty	5.50			68,778.00	19,010.00	87,788.00
1	Training Coordinator	8.00			29,871.00	12,038.00	41,909.00
5	Instructor	23.56			55,150.00	9,354.00	64,504.00
1	Discipline Director - Self-Advocacy	1.20			10,684.00	2,953.00	13,637.00
1	Discipline Director - Occupational Therapy	9.00			5,853.00	844.00	6,697.00
14	**Total Number Other Personnel**					**Total Other Personnel**	214,535.00

Total Salary, Wages and Fringe Benefits (A+B) 368,703.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

		Funds Requested ($)
1.	Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	17,300.00
2.	Foreign Travel Costs	
	Total Travel Cost	17,300.00

E. Participant/Trainee Support Costs

		Funds Requested ($)
1.	Tuition/Fees/Health Insurance	
2.	Stipends	
3.	Travel	
4.	Subsistence	
5.	Other	
	Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	
2.	Publication Costs	
3.	Consultant Services	
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.	Other Costs	207,515.00
9.		
10.		
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	207,515.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	593,518.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
MTDC	8.00	593,518.00	47,482.00
		Total Indirect Costs	47,482.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number): DHHS, Arif Karim, (415) 437-7820

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	641,000.00

J. Fee

Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	641,000.00

L. Budget Justification

(Only attach one file.) UCO.T73MC11044.Budget Narrative -2026- Add Attachment Delete Attachment View Attachment

RESEARCH & RELATED BUDGET - Budget Period 2

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: MW8JHK6ZYEX8 **Enter name of Organization:** University of Colorado Denver

Budget Type: ☒ Project ☐ Subaward/Consortium **Budget Period:** 2 **Start Date:** 07/01/2027 **End Date:** 06/30/2028

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
Dr.	Judith	A	Reaven	Ph.D.	209,947.00	4.20			73,481.00	20,311.00	93,792.00
Project Role:	PD/PI										
	Teresa	R	Charlifue-Smith		114,845.00	1.20			11,485.00	3,174.00	14,659.00
Project Role:	Discipline Director - Speech / Training Co-Director										
	Dina	D	Johnson		98,602.93	4.80			39,441.00	10,902.00	50,343.00
Project Role:	Training Co-Director - Ed/SpEd										

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file:

Total Senior/Key Person 158,794.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
6	Faculty	5.50			69,986.00	19,344.00	89,330.00
1	Training Coordinator	8.00			30,767.00	12,399.00	43,166.00
5	Instructor	26.56			58,143.00	9,829.00	67,972.00
1	Discipline Director - Self-Advocacy	1.20			11,004.00	3,042.00	14,046.00
1	Discipline Director - Occupational Therapy	12.00			8,038.00	1,159.00	9,197.00
14	**Total Number Other Personnel**					**Total Other Personnel**	223,711.00

Total Salary, Wages and Fringe Benefits (A+B) 382,505.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

		Funds Requested ($)
1.	Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	17,300.00
2.	Foreign Travel Costs	
	Total Travel Cost	17,300.00

E. Participant/Trainee Support Costs

		Funds Requested ($)
1.	Tuition/Fees/Health Insurance	
2.	Stipends	
3.	Travel	
4.	Subsistence	
5.	Other	
	Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	
2.	Publication Costs	
3.	Consultant Services	
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.	Other Costs	193,714.00
9.		
10.		
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	193,714.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	593,519.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
MTDC	8.00	593,519.00	47,481.00
		Total Indirect Costs	47,481.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number): DHHS, Arif Karim, (415) 437-7820

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	641,000.00

J. Fee

Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	641,000.00

L. Budget Justification

(Only attach one file.) UCO.T73MC11044.Budget Narrative -2026- Add Attachment Delete Attachment View Attachment

RESEARCH & RELATED BUDGET - Budget Period 3

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: MW8JHK6ZYEX8 **Enter name of Organization:** University of Colorado Denver

Budget Type: ☒ Project ☐ Subaward/Consortium **Budget Period:** 3 **Start Date:** 07/01/2028 **End Date:** 06/30/2029

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
Dr.	Judith	A	Reaven	Ph.D.	222,733.00	4.20			77,957.00	21,547.00	99,504.00
Project Role:	PD/PI										
	Teresa	R	Charlifue-Smith		118,290.35	1.20			11,830.00	3,269.00	15,099.00
Project Role:	Discipline Director - Speech / Training Co-Director										
	Dina	D	Johnson		101,561.02	4.80			40,624.00	11,229.00	51,853.00
Project Role:	Training Co-Director - Ed/SpEd										

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 166,456.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
6	Faculty	5.50			71,231.00	19,688.00	90,919.00
1	Training Coordinator	8.00			31,690.00	12,771.00	44,461.00
5	Instructor	26.56			59,887.00	10,122.00	70,009.00
1	Discipline Director - Self-Advocacy	1.20			11,334.00	3,133.00	14,467.00
1	Discipline Director - Occupational Therapy	12.00			8,279.00	1,194.00	9,473.00
14	**Total Number Other Personnel**					**Total Other Personnel**	229,329.00

Total Salary, Wages and Fringe Benefits (A+B) 395,785.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

		Funds Requested ($)
1.	Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	8,500.00
2.	Foreign Travel Costs	
	Total Travel Cost	8,500.00

E. Participant/Trainee Support Costs

		Funds Requested ($)
1.	Tuition/Fees/Health Insurance	
2.	Stipends	
3.	Travel	
4.	Subsistence	
5.	Other	
	Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	
2.	Publication Costs	
3.	Consultant Services	
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.	Other Costs	189,233.00
9.		
10.		
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	189,233.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	593,518.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
MTDC	8.00	593,518.00	47,482.00
		Total Indirect Costs	47,482.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number): DHHS, Arif Karim, (415) 437-7820

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	641,000.00

J. Fee

Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	641,000.00

L. Budget Justification

(Only attach one file.) UCO.T73MC11044.Budget Narrative -2026- Add Attachment Delete Attachment View Attachment

RESEARCH & RELATED BUDGET - Budget Period 4

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: MW8JHK6ZYEX8 **Enter name of Organization:** University of Colorado Denver

Budget Type: ☒ Project ☐ Subaward/Consortium **Budget Period:** 4 **Start Date:** 07/01/2029 **End Date:** 06/30/2030

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
Dr.	Judith	A	Reaven	Ph.D.	228,000.00	4.20			79,800.00	22,057.00	101,857.00
Project Role:	PD/PI										
	Teresa	R	Charlifue-Smith		121,839.06	1.20			12,185.00	3,367.00	15,552.00
Project Role:	Discipline Director - Speech / Training Co-Director										
	Dina	D	Johnson		104,607.85	4.80			41,843.00	11,566.00	53,409.00
Project Role:	Training Co-Director - Ed/SpEd										

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file:

Total Senior/Key Person: 170,818.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
6	Faculty	5.50			72,513.00	20,043.00	92,556.00
1	Training Coordinator	8.00			32,641.00	13,154.00	45,795.00
5	Instructor	26.56			61,684.00	10,425.00	72,109.00
1	Discipline Director - Self-Advocacy	1.20			11,674.00	3,227.00	14,901.00
1	Discipline Director - Occupational Therapy	12.00			8,528.00	1,229.00	9,757.00
14	**Total Number Other Personnel**					**Total Other Personnel**	235,118.00

Total Salary, Wages and Fringe Benefits (A+B) 405,936.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

		Funds Requested ($)
1.	Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	5,300.00
2.	Foreign Travel Costs	
	Total Travel Cost	5,300.00

E. Participant/Trainee Support Costs

		Funds Requested ($)
1.	Tuition/Fees/Health Insurance	
2.	Stipends	
3.	Travel	
4.	Subsistence	
5.	Other	
	Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	
2.	Publication Costs	
3.	Consultant Services	
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.	Other Costs	182,283.00
9.		
10.		
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	182,283.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	593,519.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
MTDC	8.00	593,519.00	47,481.00
		Total Indirect Costs	47,481.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number): DHHS, Arif Karim, (415) 437-7820

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	641,000.00

J. Fee

Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	641,000.00

L. Budget Justification

(Only attach one file.) UCO.T73MC11044.Budget Narrative -2026- Add Attachment Delete Attachment View Attachment

RESEARCH & RELATED BUDGET - Budget Period 5

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: MW8JHK6ZYEX8 **Enter name of Organization:** University of Colorado Denver

Budget Type: ☒ Project ☐ Subaward/Consortium **Budget Period:** 5 **Start Date:** 07/01/2030 **End Date:** 06/30/2031

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
Dr.	Judith	A	Reaven	Ph.D.	228,000.00	4.20			79,800.00	22,057.00	101,857.00
Project Role:	PD/PI										
	Teresa	R	Charlifue-Smith		125,494.23	1.20			12,550.00	3,468.00	16,018.00
Project Role:	Discipline Director - Speech / Training Co-Director										
	Dina	D	Johnson		107,746.09	3.86			34,694.00	9,590.00	44,284.00
Project Role:	Training Co-Director - Ed/SpEd										

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 162,159.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
6	Faculty	5.50			73,834.00	20,406.00	94,240.00
1	Training Coordinator	8.00			33,620.00	13,548.00	47,168.00
5	Instructor	26.56			63,534.00	10,739.00	74,273.00
1	Discipline Director - Self-Advocacy	1.20			12,025.00	3,323.00	15,348.00
1	Discipline Director - Occupational Therapy	12.00			8,784.00	1,266.00	10,050.00
14	**Total Number Other Personnel**					**Total Other Personnel**	241,079.00

Total Salary, Wages and Fringe Benefits (A+B) 403,238.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

		Funds Requested ($)
1.	Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	5,300.00
2.	Foreign Travel Costs	
	Total Travel Cost	5,300.00

E. Participant/Trainee Support Costs

		Funds Requested ($)
1.	Tuition/Fees/Health Insurance	
2.	Stipends	
3.	Travel	
4.	Subsistence	
5.	Other	
	Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	
2.	Publication Costs	
3.	Consultant Services	
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.	Other Costs	184,982.00
9.		
10.		
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	184,982.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	593,520.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
MTDC	8.00	593,520.00	47,480.00
		Total Indirect Costs	47,480.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number): DHHS, Arif Karim, (415) 437-7820

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	641,000.00

J. Fee

Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	641,000.00

L. Budget Justification

(Only attach one file.) UCO.T73MC11044.Budget Narrative -2026- Add Attachment Delete Attachment View Attachment

RESEARCH & RELATED BUDGET - Cumulative Budget

		Totals ($)
Section A, Senior/Key Person		812,395.00
Section B, Other Personnel		1,143,772.00
Total Number Other Personnel	70	
Total Salary, Wages and Fringe Benefits (A+B)		1,956,167.00
Section C, Equipment		
Section D, Travel		53,700.00
1. Domestic	53,700.00	
2. Foreign		
Section E, Participant/Trainee Support Costs		
1. Tuition/Fees/Health Insurance		
2. Stipends		
3. Travel		
4. Subsistence		
5. Other		
6. Number of Participants/Trainees		
Section F, Other Direct Costs		957,727.00
1. Materials and Supplies		
2. Publication Costs		
3. Consultant Services		
4. ADP/Computer Services		
5. Subawards/Consortium/Contractual Costs		
6. Equipment or Facility Rental/User Fees		
7. Alterations and Renovations		
8. Other 1	957,727.00	
9. Other 2		
10. Other 3		
11. Other 4		
12. Other 5		
13. Other 6		
14. Other 7		
15. Other 8		
16. Other 9		
17. Other 10		

Section G, Direct Costs (A thru F)	2,967,594.00
Section H, Indirect Costs	237,406.00
Section I, Total Direct and Indirect Costs (G + H)	3,205,000.00
Section J, Fee	
Section K, Total Costs and Fee (I + J)	3,205,000.00

RESEARCH & RELATED Other Project Information

OMB Number: 4040-0001
Expiration Date: 01/31/2029

1. Are Human Subjects Involved? ☐ Yes ☒ No

1.a. If YES to Human Subjects

Is the Project Exempt from Federal regulations? ☐ Yes ☐ No

If yes, check appropriate exemption number. ☐ 1 ☐ 2 ☐ 3 ☐ 4 ☐ 5 ☐ 6 ☐ 7 ☐ 8

If no, is the IRB review Pending? ☐ Yes ☐ No

IRB Approval Date:

Human Subject Assurance Number:

2. Are Vertebrate Animals Used? ☐ Yes ☒ No

2.a. If YES to Vertebrate Animals

Is the IACUC review Pending? ☐ Yes ☐ No

IACUC Approval Date:

Animal Welfare Assurance Number:

3. Is proprietary/privileged information included in the application? ☐ Yes ☒ No

4.a. Does this Project Have an Actual or Potential Impact - positive or negative - on the environment? ☐ Yes ☒ No

4.b. If yes, please explain:

4.c. If this project has an actual or potential impact on the environment, has an exemption been authorized or an environmental assessment (EA) or environmental impact statement (EIS) been performed? ☐ Yes ☐ No

4.d. If yes, please explain:

5. Is the research performance site designated, or eligible to be designated, as a historic place? ☐ Yes ☒ No

5.a. If yes, please explain:

6. Does this project involve activities outside of the United States or partnerships with international collaborators? ☐ Yes ☒ No

6.a. If yes, identify countries:

6.b. Optional Explanation:

7. Project Summary/Abstract Blank Page.pdf [Add Attachment] [Delete Attachment] [View Attachment]

8. Project Narrative UCO.T73MC11044.Narrative.final.pdf [Add Attachment] [Delete Attachment] [View Attachment]

9. Bibliography & References Cited [Add Attachment] [Delete Attachment] [View Attachment]

10. Facilities & Other Resources [Add Attachment] [Delete Attachment] [View Attachment]

11. Equipment [Add Attachment] [Delete Attachment] [View Attachment]

12. Other Attachments [Add Attachments] [Delete Attachments] [View Attachments]

UNIVERSITY OF COLORADO JFK PARTNERS LEND PROGRAM

PROJECT NARRATIVE

TABLE OF CONTENTS

INTRODUCTION 1
NEED 2
APPROACH 4
HIGH-LEVEL WORK PLAN 21
RESOLVING CHALLENGES 24
PERFORMANCE REPORTING AND EVALUATION 25
SUSTAINABILITY 30
ORGANIZATIONAL INFORMATION 30
REFERENCES 39

Frequently Used Acronyms (*The following acronyms are used to conserve space.*)

A/DD	Autism and Developmental Disabilities
AUCD	Association of University Centers on Disabilities
CDPHE	Colorado Department of Health and Environment
CHCO	Children's Hospital Colorado
CQI	Continuous Quality Improvement
CU/CU Anschutz	University of Colorado, Anschutz Campus, Aurora, CO
CU SOM	University of Colorado (CU) School of Medicine
DP	Section of Developmental Pediatrics and CHCO clinical unit
ID	Intellectual Disability
IHE	Institution of Higher Education
ITP	Individual Training Plan
IwD	Individuals with Autism & Developmental Disabilities
IwD&F	Individuals with Autism & Developmental Disabilities & their Families
L-/M-/STT	Long-, Medium-, or Short-term Trainees
MCH	Maternal and Child Health
UCEDD	University Center for Excellence in Developmental Disabilities

INTRODUCTION

The University of Colorado School of Medicine (CU SOM) Department of Pediatrics submits this application for continued funding of the JFK Partners Leadership Education in Neurodevelopmental and Related Disabilities (LEND) Training Program in response to Opportunity number: HRSA-26-019. JFK Partners, a program of the CU SOM, Section of Developmental Pediatrics (DP) at the Anschutz Campus in Aurora, Colorado, conducts both Colorado's LEND program and the University Center for Excellence in Developmental

Disabilities Education, Research and Service (UCEDD). JFK Partners LEND program will be implemented consistent with NOFO statement of purpose "to increase the capacity of the workforce trained to meet the complex needs of children and youth with autism/DD by enhancing the expertise and leadership skills of students, practicing professionals, individuals with disabilities, and families dedicated to working with and improving the system of care for this population" (HRSA-26-019 NOFO, pg. 12).

NEED

Colorado is home to more than six million residents,[1] most (82%) living along the Front Range of the Rocky Mountains. Approximately 30% are children and young adults under age 24.[2] The population is 65% non-Hispanic White, 23% Latino, 5% Black, 4% Asian, 4% multiracial, and fewer than 1% American Indian/Alaskan Native; 16% speak a language other than English at home. Most adults (93%) have at least a high school diploma, 47% hold a college or advanced degree. The median household income is $95,000, although about 10% of Coloradans live in poverty.[1] Despite Colorado's natural and population assets, many children and youth with autism and developmental disabilities (A/DD) and their families have unmet needs that limit their health outcomes, wellbeing, and community participation. As an experienced LEND program aligned with HRSA's mission and stewardship expectations, JFK Partners seeks continued funding to expand Colorado's capacity to serve individuals with A/DD and their families. We will prepare the next generation of leaders, strengthen workforce skills and systems, and advance implementation of evidence-based A/DD practices informed by lived experience in collaborative community partnerships.

Despite autism prevalence estimates of 1 in 31 children from the Autism and Developmental Disabilities Monitoring (ADDM) Network and 1 in 25 by parent report,[3,4] service systems continue to fall short in providing timely diagnosis of A/DD, even when symptoms are severe.[5] A well-documented gap persists between the potential and actual age of diagnosis (approximately ages two and four, respectively),[3,4] and during Colorado's ADDM participation, only half of studied children were diagnosed before age four.[6] A shortage of qualified diagnosticians is a major contributor to this gap, which disproportionately limits access for low-income families.[7] Amid a growing "waitlist crisis",[8] families can face delays of a year or more for diagnostic appointments.[9] Such delays increase pre-diagnosis health care utilization and costs, postpone access to specialized services,[10] and may negatively affect long-term quality of life.[11] In addition to delayed diagnosis, misdiagnosis and overdiagnosis can negatively affect child and family wellbeing and contribute to inefficiencies in health care systems.[12] Core autism characteristics are often accompanied by co-occurring physical, mental, and behavioral health conditions that require specialized services and therapies.[13] As a result of their greater and more complex needs, children with autism and other disabilities frequently experience unmet needs that adversely affect family finances, employment, and well-being.[14].

Inadequate access to high-quality transition planning is common among adolescents with A/DD.[15] Families navigating their children's transition to adulthood often face financial and emotional burdens, limited access to services, and challenges coordinating care and supports,[16] which contribute to delays in employment, independent living, and other adult outcomes.[17]

Autism prevalence among adults was estimated at approximately 2% in 2020.[18] Diagnostic rates continue to increase, particularly among individuals under age 35, females, and certain sociodemographic groups.[19] Compared with their peers, autistic adults experience higher

rates of unmet health care needs and disrespectful treatment,[20] as well as greater loneliness.[21] They are also less likely to achieve positive adult outcomes, including employment, postsecondary education, independent living, and social and community participation.[22] By early adulthood, autistic females and adults with other disabilities, including cerebral palsy and intellectual disability, face an elevated risk of mortality.[23]

Colorado ranks 12th nationally for overall child wellbeing but only 24th for child health.[24] Following the post-COVID Medicaid unwinding process, the rate of uninsured children increased significantly, lowering Colorado's ranking to 32nd nationally for child health coverage.[25] Although Colorado scored an impressive 10th nationally for health care quality in the 2025 Scorecard on State Health System Performance,[26] significant gaps remain in access to and delivery of child mental health services, which are frequently needed by individuals with A/DD. The state also has a high prevalence of mental illness and limited access to care, ranking 31st nationally for youth mental health and 48th for adult mental health outcomes.[27]

Autism Speaks reports that nearly 3% of Colorado families have a child with autism, with an average age of diagnosis of 5.8 years. Many have co-occurring ADHD (24%), anxiety (12%), epilepsy (8%), and depression (4%). As adults, anxiety (at 23%) and depression (at 16%) are the most common co-morbidities.[28] In 2025, approximately 6% of Colorado infants and toddlers received early intervention,[29] and autism and developmental delay accounted for 11% and 8%, respectively, of Colorado's public-school special education population (including children enrolled in the state's universal preschool program).[30] Compared with national averages, fewer Colorado autistic children receive special education services (9% vs. 12%), and school suspensions and disciplinary actions are more common (7% vs. 5%). Although Colorado students with A/DD are more likely to complete high school (86% vs. 72%), fewer receive vocational rehabilitation services after graduation (73% vs. 80%). Families report higher rates of unmet health care needs (18% vs. 11%), higher outpatient service costs, and substantial food and housing insecurity, which affects nearly one-quarter of households.[28]

The Colorado Developmental Disabilities Council (CDDC), a partner in the state DD network, sponsored a 2025 needs assessment with input from 288 respondents. It identified key disability needs in the areas of: Systems navigation and continuity of supports (especially during major life transitions); access to health and behavioral health care (e.g., waitlists, competent providers, communication barriers); education and employment (inconsistent job coaching, employer biases); transportation and housing options; social isolation, stigma, and disrespect. The findings also called for more community representation in leadership and policy reform. Central themes stressed the importance of inclusion, respect, and autonomy for individuals with developmental disabilities.[31]

To inform this proposal, JFK Partners conducted needs assessment surveys of the JFK Partners Community Advisory Council (CAC), composed of IwD&F and community agencies, using a long-form version and an optional brief version for individuals who find lengthy surveys challenging); b) our broader community (with versions similar to the CAC formats); and c) JFK Partners faculty and staff. The compiled surveys received 173 responses (83% community responses); 62% were respondents with family or firsthand lived A/DD experience. The findings were consistent with needs reported in the literature and the CDDC results. Respondents identified needs for: 1) a disability-informed workforce equipped with evidence-based knowledge and tools (including leaders with lived A/DD experience), 2) providers and systems that are A/DD-informed, and 3) resources for IwD&F to support informed decision-making and greater autonomy. Open ended question responses also reiterated the importance of inclusion,

respect, and autonomy: *So often…societal pressures and misconceptions become more debilitating than the disability itself…*[since our communities fail to] *see I/DD through a strengths-based, human lens* (respondent quote).

IwD&F experience complex interactions of their disabilities and community health factors with health, education and related systems. Encounters with ill-informed or unresponsive providers and systems result in unmet needs, poor outcomes, dissatisfaction, and distrust of essential services.[31,32] Persistent gaps and system failures underscore the need for professionals trained in respectful, evidence-based assessment, diagnosis, and intervention practices and support. In response, JFK Partners proposes to: 1) increase the number of professionals, individuals with disabilities, and family MCH leaders prepared to serve IwD&F; 2) increase evidence-based A/DD knowledge and skills in the workforce and system capacity to respectfully serve IwD&F; and, 3) demonstrate the impact of a LEND program grounded in accountable, data-informed decision-making, and continuous quality improvement.

APPROACH

This section describes our proposed approaches to Overall Methodology, Goals and Objectives, Trainee Recruitment and Retention, Training and Education, Partnerships, Technical Assistance and Continuing Education for JFK Partners LEND program.

Overall Methodology

JFK Partners developed this proposal to meet LEND program requirements and address gaps and needs identified in the NOFO, current literature, and Colorado community needs assessments to increase the capacity of the workforce prepared to meet the needs of IwD&F by increasing leadership knowledge and skills of students, practicing professionals, individuals with disabilities, and families committed to strengthening the system of A/DD care. Our approach will incorporate ongoing input from and collaboration with community partners and individuals with disabilities and their families throughout program planning, implementation, and evaluation. Consistent messaging and actions will affirm the importance of inclusion, respect, and autonomy for the target population. JFK Partners will align program management and activities with the HRSA Mission and Strategic Priorities as defined in the NOFO (pgs. 61-64) and maintain the protection of educational information, health information, and HHS systems and data consistent with cybersecurity requirements (NOFO, pg.65). Compliance with HRSA MCHB LEND expectations and requirements will be ensured through program monitoring by project leadership, together with the School of Medicine Department of Pediatrics and University oversight, and the technology professionals of CU Denver and CU Anschutz Information Strategy and Services (ISS) and Children's Hospital Colorado (CHCO) Information Services.

Goals and Objectives

JFK Partners has set three broad goals (A – C), addressed through specific, measurable, attainable, relevant, and time-based objectives (e.g., A1, A2, etc.). Objectives will be implemented each year through June 2031 as follows.

> **GOAL A**. Increase access to evidence-based screening, diagnosis, interventions, and supports for individuals with A/DD and their families (IwD&F) by preparing interdisciplinary, person- and family-centered leaders through competency-based training and community partnerships.

- *Objective (Obj.) A1.* Recruit and educate trainees in a competency- & evidence-based LEND curriculum (didactics, clinical, research, & community experiences) with active partner engagement to increase evidence-based A/DD screening, evaluation, diagnosis, & interventions for children who (may) have A/DD. *Outcomes:* At least (≥) 18 long-term trainees (LTTs) in IwD, Family & 8 other disciplines, 10 medium-term trainees (MTTs), & 100 short-term trainees (STTs) trained; ≥ 75% of LTTs and sampled MTTs & STTs acquire new knowledge/skills & are satisfied with training; ≥ 6 active IwD&F or community partners. *Timeframe:* Yrs. 1-5, summer, fall, spring semesters
- *Obj. A2*. Obtain alumni assessments of LEND training value and impact. *Outcomes*: ≥ 75% respond to survey; ≥ 80% recommend LEND; ≥ 70% use skills; 60% report lasting impact *Timeframe:* Yrs. 1- 5, spring

GOAL B. Strengthen the MCH workforce and systems of care, in partnership with individuals with A/DD and their families (IwD&F) and community partners, to improve access to competent, responsive, and evidence-based services and supports.

- *Obj. B1*. Conduct evidence-based CE with IwD&F and agency partners to increase workforce knowledge and skills. *Outcomes*: ≥ 5 CE events (3 for credit); ≥ 200 participants across urban and rural areas; ≥ 75% of participants are satisfied with content; ≥ 75% report increased/refreshed knowledge/skills; ≥ 50% intend to apply recommended practices applicable to role. *Timeframe:* Yrs. 1-5.
- *Obj. B2.* Obj. B2. Promote strong partnerships among IwD&F, state, local, and community disability interests to increase evidence-based systems responsive to community priorities. *Outcomes*: ≥ 6 partner programs; ≥ 75% of sampled TA projects report progress in goals & satisfaction with TA services. *Timeframe:* Yrs. 1-5
- *Obj. B3*. Share evidence-based A/DD information & resources, in accessible & plain language formats, including a research newsletter, annual report, & other products to inform IwD&F and related audiences and maximize LEND impact. *Outcomes*: ≥ 15 project products; product accessibility for lay audiences "substantially achieved." *Timeframe:* Yrs. 1-5

GOAL C. Demonstrate responsible stewardship of LEND resources through accountable grant management to maximize LEND impact for IwD&F.

- *Obj. C1.* Manage data collection to enable accurate performance reporting. *Outcome:* 100% of data reporting requirements met. *Timeframe:* Yrs. 1-5, by defined MCHB deadlines
- *Obj. C2*. Conduct and report comprehensive program evaluation and continuous quality improvement (CQI) findings to assess and strengthen progress towards LEND goals. *Outcomes:* ≥ 80% of overall program measures are positive in all program performance domains; no. of timely data/report submissions: ≥ 1 CQI change package tested. *Timeframe:* Ongoing, Yrs. 1–5
- *Obj. C3*. Manage LEND resources responsibly through fiscal oversight and data-informed resource allocation to support achievement of program objectives. *Outcomes:*

100% required fiscal reports submitted accurately and on time; 0 material audit or compliance findings. *Timeframe:* Yrs 1 – 5

Most goals and objectives represent conservative minimum targets for the five-year project period that we are likely to surpass. Consistent with our past performance, we establish attainable objectives while striving to exceed them during implementation (see Organizational Information, Past Experience).

Brief plan to share project outputs

In addition to required grant reporting on LEND implementation and outcomes (see High-Level Work Plan), project outputs will be disseminated to targeted and public audiences. Targeted dissemination includes peer-reviewed publications, presentations at professional meetings, reports to technical assistance partners, and an annual presentation to the JFK Partners Community Advisory Council. Public dissemination includes an annual report, research newsletter, website postings of faculty and trainee scholarly projects and resources, archived continuing education events with closed captioning, and materials distributed at community events (e.g., print and QR codes). Outputs will also be shared through social media, listservs, email, and the JFK Partners website. All publicly disseminated materials will be accessible, including plain-language versions, and will comply with Colorado House Bill 21-1110, the Digital Accessibility Law for Colorado State and Local Government.

Trainee Recruitment and Retention

We will annually recruit at least 18 long-term trainees (LTTs), 10 medium-term trainees (MTTs), and 100 short-term trainees (STTs). LTTs will be postdoctoral fellows, doctoral candidates, graduate students in professional disciplines, individuals with autism and developmental disabilities (IwD), and family members. We will recruit LTTs representing various populations from at least 10 disciplines: Individuals with Disabilities, Family Members, Developmental Behavioral Pediatrics/Medicine, Education/Special Education, Advanced Practice Providers (Nurse Practitioner and Physician Assistant), Nutrition, Occupational Therapy, Physical Therapy, Psychology, Public Health, Social Work, and Speech-Language Pathology. Each cohort will include at least one Family and one IwD LTT, with no discipline comprising more than 25% of the cohort. If funded, we will seek reapproval of Rehabilitation Counseling to strengthen training in transition to adulthood and employment. Additional disciplines, which participated during 2021-26, may include Dentistry, Family Medicine, Genetic Counseling, and Rehabilitation Counseling. Some trainees may be offered a second LEND year designed to strengthen specific targeted competencies once numerical requirements are met.

LTTs will complete at least 300 hours of training and have Individual Training Plans (ITPs) specifying goals, activities, and training intensity as either full-time (40 hours/week, typically clinical disciplines) or part-time (12 hours/week). Advanced MTTs will complete 150–299 hours of training guided by ITPs, while other MTTs will receive 40–149 hours of didactic, clinical and/or other practical experiences from the LEND training menu. STTs will receive fewer than 40 hours of faculty-delivered or -supervised training.

Recruitment is led by the Project Director, Training Co-Directors, Discipline Directors, and Training Coordinator through the JFK Partners website, listservs, social media, discipline-specific brochures and flyers, professional meetings, partner organizations, and community and campus networks, including those serving different populations. Professional trainees are recruited from Colorado institutions of higher education, statewide, and nationally (Attachment

3: Agreements with Other Entities). Recruitment materials are tailored by discipline and also promote MTT and STT opportunities. Individuals with disabilities (IwD) and Family trainees are recruited primarily within Colorado through community agency referrals, electronic dissemination, word of mouth, LEND alumni, the JFK Partners Community Advisory Council, and community partners. Most MTTs and STTs are CU Anschutz graduate students in health and related disciplines, medical students, residents, fellows, students from partner institutions of higher education (Attachment 3: Letters of Agreement), or trainees with lived experience in A/DD. They are recruited primarily through LEND and Developmental Pediatrics faculty in academic programs and clinical settings, although some are drawn to the program after attending the JFK Partners campus-wide *Disability Dialogues* series (described later).

JFK Partners uses a merit-based selection process that reviews applicants' CVs, transcripts, experience, and recommendations, as well as faculty interviews to identify the most qualified individuals who meet statutory requirements. Selection criteria include leadership potential, interest in serving IwD&F and interdisciplinary practice, commitment to rigorous training, willingness to participate in long-term follow-up evaluations, and statutory eligibility (i.e., US citizens, noncitizen nationals of the US, lawful permanent residents, or other legally qualified aliens). Applicants in clinical disciplines must have sufficient clinical preparation to acquire LEND competencies in A/DD screening, evaluation, diagnosis, and intervention. LTTs and some Advanced MTTs receive salaries/stipends consistent with MCHB guidance, prorated for full- or part-time status.

Trainee retention is supported through mentoring and individualized accommodations. LTTs and Advanced MTTs are assigned a primary same-discipline mentor, with additional mentors as needed. Mentors assess competencies, develop Individual Training Plans (ITPs), foster personal and professional growth, monitor progress, and adjust goals and activities throughout the year. Cohort progress is reviewed in faculty Training Review Meetings (at least eight annually), with responses including revised goals, leadership consultation, mentor reassignment, and referral to campus or community resources.

All incoming LTTs and Advanced MTTs complete a Trainee Needs Assessment to identify accommodations that support retention and goal attainment. Accommodation requests from trainees with fewer than 150 hours of training are typically managed by faculty supervisors and referred to program leadership or the Training Review Committee when needed.

Training and Education

The JFK Partners curriculum delivers LEND content through interdisciplinary mentoring and didactic and practical experiences spanning clinical, research, policy, and community settings. Training is grounded in: a) MCH Leadership Competencies and additional JFK Partners competencies (e.g., clinical practice, consultation); b) person- and family-centered care, systems, policy, and research informed by individuals with lived experience in MCH and A/DD; c) culturally and linguistically appropriate practice; d) gold-standard, evidence-based science; and e) critical appraisal and application of research. Community partners, family members, and individuals with disabilities participate throughout academic, clinical, and field experiences. Because LEND cohorts include trainees with varied academic preparation and lived experience, instruction is individualized through faculty mentoring, competency-based training, interdisciplinary peer learning, accessible educational practices, and tailored supports to promote achievement of LEND competencies.

In recent years, JFK Partners has welcomed an increasing number of neurodivergent trainees across disciplines seeking LEND training. Faculty continue to strengthen their capacity

to support individualized learning through universal design for learning practices, including flexible assignment formats and communication methods, explicit verbal and written expectations, scaffolded learning with intermediate milestones for complex projects, and multiple options for demonstrating knowledge and competencies. We have supported trainees using assistive technologies, pacing and other instructional accommodation (such as providing materials in advance to support preparation and offering plain-language versions of material), and are prepared to adapt sensory aspects of learning environments as needed. Individualized support is guided by the incoming Trainee Needs Assessment, ongoing mentoring, and continuous dialogue with trainees. Consistent with the LEND philosophy of reciprocal learning, we continue to learn with and from our neurodivergent trainees, whose perspectives strengthen our educational practices and our capacity to prepare future leaders.

Curriculum Delivery

The JFK Partners Training Program Review Committee (Director, Training Co-Directors, LEND Discipline Directors, faculty, and Evaluator) meets at least eight times annually to monitor program implementation, evaluation, and trainee progress. Orientation in summer introduces program policies and expectations, followed by didactic and practicum training. By early fall, primary mentors and trainees develop Individual Training Plans (ITPs) that include competency assessments, individualized goals, and course and practicum assignments. LTTs also develop and complete individualized research, quality improvement, or other scholarly projects over the course of the training year, aligned with training goals and supported by faculty mentoring. ITPs are reviewed regularly during mentoring and updated at least once at midyear. Full-time LTTs (primarily in clinical disciplines) complete 24–30 practicum hours weekly, while non-clinical trainees complete 8–16 hours. A June Training Program Retreat evaluates the academic year and identifies priorities for continuous quality improvement and the upcoming training year. (See Attachment 1. Work Plan, Goals A and C, and Attachment 5. Tables, charts, and syllabus/curriculum.)

Didactic content and course descriptions

Didactics are typically delivered by experienced multidiscipline faculty teams in partnership with individuals with disabilities and their families (IwD&F). Public health leaders and community partners serve as guest speakers, discussants, and interviewees. The curriculum addresses evidence-based medical and scientific approaches to A/DD etiology, screening, diagnosis, and intervention, social and other models of disability, and lived experience perspectives to examine the interactions of community health factors, service systems, and life experiences, and their impact on outcomes across the lifespan. Trainees in all LEND disciplines may participate in any course, as well as clinical and other practical experiences that provide opportunities to apply, integrate, and refine knowledge and skills and foster peer-to-peer learning. The LEND program uses Canvas, a cloud-based learning management system, for course delivery, assignments, and faculty access. Additional assigned online modules, workshops and conference opportunities are offered to meet trainee needs.

An overview of didactic treatment of required LEND content (NOFO, pg. 15) is presented below. There are six courses and a research seminar: *Leadership Dialogues 1* and *2* (LD1, LD2), *Key Concepts in Neurodevelopmental Disabilities 1* and *2* (KC1, KC2), *Screening and Assessment in Developmental Disabilities* (S&A), I*nterventions in Developmental Disabilities* (INT), and *Interdisciplinary Research Seminar* (IRS). Course descriptions follow the table (also see Attachment 5).

TABLE 1. Overview of Didactic Treatment of LEND Concepts & Topics							
Concepts & Topics	*LD1*	*LD2*	*KC1*	*KC2*	*S&A*	*INT*	*IRS**
Foundations of Disability and Health							
Various models of disability			✓	✓	✓		
Interaction of disability with other life experiences	✓	✓	✓	✓	✓	✓	✓
Influences of the social environment on A/DD	✓	✓	✓	✓	✓	✓	
Emerging needs & special interests in A/DD		✓	✓	✓	✓	✓	✓
Person- and Family-centered Systems of Care							
Person-/family-centered care practices	✓	✓	✓	✓	✓	✓	✓
Culturally & linguistically appropriate services	✓	✓	✓	✓	✓	✓	✓
Systems of care for children & youth with A/DD & special health care needs		✓	✓	✓	✓	✓	
Application of medical home concepts			✓	✓	✓	✓	
Care coordination			✓	✓			
Supporting care transitions		✓	✓	✓	✓	✓	
Health care/other transitions, youth to adulthood			✓	✓			
Services & supports for A/DD in adulthood			✓	✓	✓	✓	
Innovative health care delivery/telehealth			✓	✓		✓	
Science, Research, and Quality							
Critical thinking & science-based judgment		✓	✓	✓	✓	✓	✓
Research methods		✓	✓	✓	✓	✓	✓
Evidence-based practice	✓	✓	✓	✓	✓	✓	✓
Performance measurement/continuous quality improvement in clinical & community-based practice		✓			✓	✓	✓
Interdisciplinary Leadership and Policy							
Interdisciplinary care	✓		✓	✓	✓	✓	✓
Title V Maternal & Child Health Services Block Grant Program, MCH legislation (& policy processes)		✓	✓	✓			
Evidence-Based A/DD Clinical Practice							
Best practices for autism screening & diagnosis			✓		✓		
Effective tools for A/DD screening & diagnosis			✓		✓		
Evidence-based interventions (lifespan & levels of support)			✓	✓		✓	

** IRS will address additional listed topics, dependent upon faculty and trainee projects.*

- *Leadership Dialogues 1 & 2* (LD 1 & 2; 2 credits and 1 credit, respectively) develop MCH Leadership Competencies (Version 5.0) and their application across public health and health care systems through lectures, readings, self-assessments, assignments, and structured group activities. The summer intensive, *Leadership Dialogues 1 (LD1),* emphasizes: a) Self-Reflection (e.g., leadership styles, influence of experiences); b) Communication (e.g., tailoring communication for specific audiences); c) Interdisciplinary/Interprofessional Team Building (e.g., team roles, dynamics); d) Negotiation and Conflict Resolution (e.g., sources of conflict, negotiation strategies); e) Community Health Factors (e.g., influence on health of individual, family, community circumstances/experiences); and f) Lived Experience in MCH (e.g., engagement in all phases of policy/program processes). Self-assessments and group interactions strengthen leadership skills, self-awareness, and cohort cohesion. Faculty and guest speakers facilitate discussion. *Leadership Dialogues 2* (LD2) builds on LD1 competencies and focus on: a) Policy (e.g., MCH issues, legislative processes, and informational strategies); b) MCH Knowledge Base and Context (e.g., programs, systems, limitations, gaps); c) Critical Thinking (basic statistics, analytical tools, evidence-based communication); and d) Systems Thinking (e.g., partnerships, needs assessment, collaboration). Trainees develop policy products, make presentations, observe the Colorado legislature, and engage with legislators and community agencies to apply competencies.

- *Key Concepts in Neurodevelopmental Disabilities 1 & 2* (KC1 & KC2; 2 credits each) present interdisciplinary, public health, and person- and family-centered perspectives on A/DD, systems of care, and lifespan issues. Individuals with disabilities and their families (IwD&F), Colorado Title V MCH staff, and community partners serve as guest faculty. Course progression addresses: a) A/DD foundations (e.g., epidemiology, ethics, genetics, medical and social models of disability, evidence-based practice, and interdisciplinary roles); b) systems of care (e.g., federal MCH policy and legislation, Colorado Title V MCH priorities, medical home, health care access, care coordination, nutrition, dental care, and early intervention and education); and c) lifespan issues (e.g., adolescence, dual diagnosis, transition to adult health care, higher education, employment, relationships, housing, waivers, guardianship and supported decision-making, caregiver resilience, emergency preparedness, and emerging issues affecting MCH populations). Trainees analyze current legislation, interview Colorado Title V MCH staff, and participate in family-mentored visits. KC1 also introduces the Scholarly Project (research, quality improvement, resource development, technical assistance, or other scholarly work) and related research concepts, including Institutional Review Board requirements and processes.

- *Screening and Assessment in Developmental Disabilities* (S&A) (3 credits, fall) prepares trainees to recognize and differentiate A/DD conditions by addressing foundational knowledge, models of disability, evidence-based screening, assessment, and interdisciplinary evaluation practices. Topics include signs and symptoms across developmental stages; screening and diagnostic tools (e.g., M-CHAT-R, STAT, ADOS-2); differential diagnosis; IQ assessment for youth with IDD; co-occurring conditions; interpretation of behavioral phenotypes; developmental evaluations; parent interviews; communicating results to families; therapeutic listening and coaching; and care

coordination. The course also addresses telehealth, cultural considerations (e.g., bilingual assessment, cultural perspectives on A/DD), controversies in assessment and treatment, and strategies for complex diagnostic presentations. Interactive case studies reinforce application of evidence-based assessment and clinical decision-making.

- *Interventions in Developmental Disabilities* (INT) (3 credits, spring) reviews interdisciplinary evidence-based and complementary, alternative, and integrative medicine (CAM) interventions across a wide range of A/DD concerns. Trainees critically evaluate intervention evidence and learn to apply best-practice recommendations in person- and family-centered, culturally responsive, and telehealth contexts. Content includes interventions for executive functioning, motor, sensory, vision, brain injury, daily living skills, nutrition, social communication, family therapy, and critical MCH topics such as fetal alcohol spectrum disorder. Best-practice reviews (e.g., National Standards Project, Educating Children with Autism, and professional organization guidelines) inform evidence reviews. Instruction includes lectures, critical literature review, case presentations and discussions, and presentation of trainee-developed CAM reviews and intervention tip sheets.

- *Interdisciplinary Research Seminar* (IRS) (not for credit, monthly) enhances understanding of research ethics, methods in project development, design, implementation, community health factors, and engagement of IwD&F in all aspects of research. T/Fs gain skills for critical review of research, crafting research goals and methods, basic data analysis (individual consultation is available for complex analyses), abstract and manuscript preparation, and quality improvement processes. Current A/DD literature is reviewed. Faculty and trainees present works in progress.

- *Disability Dialogues* (non-credit; 8 sessions) introduces STTs and MTTs from campus programs to disability, lived experience, and A/DD across the lifespan. IwD&F, together with service providers, discuss A/DD conditions, health care experiences, interactions with providers, and the intersection of disability with other life experiences. Annual topics are selected by a volunteer interdisciplinary group of STTs under LEND faculty supervision. The series builds awareness of disability-related health issues and fosters familiarity, confidence, and respectful engagement with IwD&F that trainees may carry into future professional practice. Readings and additional resources are provided as appropriate.

All courses are updated annually in response to emerging evidence, competency revisions, HRSA requirements, instructor and partner changes, and evaluation and CQI findings.

Clinical Practicum and Other Practical Experiences

JFK Partners provides practical experiences that prepare trainees for leadership as clinicians, researchers, consultants, and systems leaders. Practica translate classroom learning into MCH Leadership and program-specific competencies, with hours tailored to discipline and prior preparation. Practicum hours include 2+ hours weekly individual or team supervision, during which supervisors use reflective mentoring to foster personal and professional growth.

Clinical practica support trainees to develop expertise in evidence-based A/DD screening, diagnosis, intervention, and transition to adult health care. Throughout the clinical process—from initial interviews to formal assessment, to therapeutic interactions to written documentation—faculty model neurodiversity-affirming practices with individuals with disabilities and their families. Trainees learn to recognize how community health factors and social environments influence clinical interactions and outcomes, and to respond effectively to complex presentations and the cultural and linguistic needs of individuals with disabilities and their families. Practice sites are intentionally interdisciplinary. Trainees from non-clinical disciplines contribute disciplinary perspectives during team discussions and case conferences, while individuals with lived experience provide insight into the patient and family experience of care and how clinician communication and interactions influence trust, engagement, and outcomes. In turn, these trainees strengthen their ability to analyze clinical interactions, communicate their perspectives effectively, contribute to interdisciplinary decision-making, and apply lived expertise in the service of others.

Sites

Most practicum sites use comprehensive, multidisciplinary service models to provide person- and family-centered care to individuals from varied community populations, while single-discipline practica develop specialized skills. Trainees observe culturally and linguistically appropriate care delivered by clinicians who are fluent in patients' preferred languages or skilled in working with interpreters. Selected sites also offer opportunities to participate in quality improvement (QI) projects. All sites maintain HIPAA-compliant electronic health record systems (e.g., Epic) and follow institutional policies to protect patient privacy and confidentiality. Table 2 summarizes sites, trainee experiences, and locations currently planned for 2026–31; additional community sites may be negotiated.

Many clinical practica are offered through our partnership with Children's Hospital Colorado (CHCO), with faculty from the CU Department of Pediatrics, including LEND faculty and alumni. Because of the specialized expertise required, most diagnostic clinics are located at CHCO. Ranked among the U.S. News & World Report *Best Children's Hospitals Honor Roll* and the top children's hospital in the Rocky Mountain region (2026), CHCO provides training at the Anschutz campus and community sites along Colorado's Front Range, with rural outreach clinics throughout the state in Alamosa, Glenwood Springs, Grand Junction, and Durango. Several clinics also provide telehealth services. CHCO serves children and families with varied cultural, linguistic, and socioeconomic backgrounds and a wide range of medical and behavioral health needs. As Colorado's largest provider of care for uninsured, Medicaid-, and CHP+-insured children, CHCO cares for many children with medical complexity and specialty behavioral health needs, while serving families experiencing housing instability, food insecurity, and other social challenges.

TABLE 2. Clinical Sites: Services offered and LTT site experiences				
I = interdisciplinary **Dx** = evaluation, diagnosis & recommendations **T** = therapy/intervention				
Title	*I*	*Dx*	*T*	*Location(s)*
DIAGNOSTIC EVALUATIONS				
JFK Partners Interdisciplinary Evaluation Clinic (12 mos. - 18 yrs.)	✓	✓		CHCO
Adult Evaluation Clinic (18+ yrs.)	✓	✓		CHCO
Infant – Toddler Clinic	✓	✓		CHCO/North Campus/CO Springs
Birth to 3 Clinic	✓	✓		CHCO
Preschool Clinic (3 - 5 yrs.)	✓	✓		CHCO/North Campus/ Wheatridge/CO Springs
School Age Clinic (6 - 10yrs.)	✓	✓		CHCO/CO Springs
Pre-teen/Adolescent Clinic (12- 26 yrs.)	✓	✓		CHCO/CO Springs
Developmental Team (for Child Health Clinic patients referred for evaluation by DBP in Medical Home)	✓	✓		CHCO
BLOSSOM High Risk Neonatal Follow Up Clinic	✓	✓	✓	CHCO/CO Springs
ACES Clinic (Children in foster care and with trauma exposures) (1-7 years)	✓	✓		CHCO
Deaf/Hard of Hearing Clinic	✓	✓		CHCO
Bilingual Clinic (Spanish/English)	✓	✓		CHCO/SE Aurora
One Day Clinic (out-of- state patients)	✓	✓		CHCO
Rural Outreach Sites	✓	✓		Durango/Grand Junction/Alamosa/ Glenwood Springs
Neurogenetics Clinics				
10 Neurogenetics clinics in, e.g., 22Q, Fragile X, Prader Willi Clinic	✓	✓	✓	CHCO
Single Discipline				
MD/NP Single Discipline & Fellows' Clinics		✓		CHCO/North Campus/ Wheatridge/SE Aurora/CO Springs
Child Health Clinic DBP evaluation		✓		CHCO Aurora
Psychology Single Discipline		✓		CHCO/North Campus/South Campus/ CO Springs
THERAPIES/INTERVENTIONS				
Next Steps Clinic		✓		CHCO/Telehealth

Title	*I*	*Dx*	*T*	*Location(s)*
ABC's of Behavior Parent Group			✓	CHCO/Telehealth
CHCO Outpatient Clinic		✓	✓	CHCO/Highlands Ranch
Early Start Denver Model Clinic			✓	CHCO/North Campus/CO Springs
ENRICH (early intervention)	✓		✓	Metro Denver natural environments
Behavioral Therapy Clinic/RUBI			✓	CHCO/Telehealth
Behavior Therapy Group/RUBI			✓	CHCO/Telehealth
Cognitive-Behavior Therapy Clinic			✓	CHCO/CO Springs/Telehealth
Facing Your Fears (8-14 years)	✓		✓	CHCO/CO Springs/Telehealth
Facing Your Fears: IDD (12-18 yrs.)	✓		✓	CHCO
Behavioral Sleep Clinic	✓	✓	✓	CHCO/Telehealth
Pediatric Oral Feeding Clinic	✓	✓	✓	CHCO/Telehealth
Toileting Clinic	✓	✓	✓	CHCO/Telehealth
Birds and Bees Clinic			✓	Telehealth
HOPE Cancer Survivorship Program	✓	✓	✓	CHCO
Transition Clinic	✓	✓		CHCO/CO Springs/Telehealth
Community Partner Sites				
UC Health Behavioral Neurology Adult I/DD Clinic		✓	✓	Central Park Medical Center Denver
PT School Practicum	✓	✓	✓	Aurora Public Schools
Adam's Camp	✓		✓	Metro Denver
Rise School of Denver	✓		✓	Denver
Marion Downs Center		✓	✓	Greenwood Village
KidScreen for Hearing, Speech & Vision	✓	✓	✓	Denver metro area schools
Talking Together	✓		✓	Family homes (Front Range)
CO Public School Districts	✓		✓	Denver, Cherry Creek, Littleton, Colorado Springs

Diagnostic Evaluation Clinics

The diagnostic clinics listed in the table above, include interdisciplinary clinics, several of which are stratified by age, to evaluate and diagnose children with A/DD. Principal among them for the purposes of the LEND training program are the *JFK Partners Interdisciplinary Evaluation Clinic* for children, ages 12 months to 18 years, and the *Adult Evaluation Clinic* (18+ yrs.). Both expose trainees to evidence-based tools and expert interdisciplinary diagnosis and differentiation of A/DD conditions. The *Adult Clinic* serves patients seeking an evaluation for autism, intellectual disability, and co-occurring mental health conditions, or who may need

updated testing to qualify for other services. CHCO also offers several interdisciplinary team evaluation diagnostic clinics stratified by age, including *Infant – Toddler Clinic, Birth to 3 Clinic, Preschool Clinic* (3-5 yrs.), *School Age Clinic* (6-10yrs.), *Pre-teen/Adolescent Clinic* (12-26 yrs.). There is a diagnostic clinic for children referred from the medical home Child Health Clinic for further evaluation by the *Developmental Team*. Trainees may experience diagnostic clinics that address more specific concerns, including *BLOSSOM High Risk Neonatal Follow Up Clinic, ACES* (Adverse Childhood Experiences) *Clinic* (for children ages 1-7 years in foster care and with trauma exposure), and *Deaf/Hard of Hearing_Clinic* (for potential dual diagnosis of hearing impairment and A/DD). The *Bilingual Clinic* offers evaluation and diagnosis of A/DD in Spanish for children ages 1-17 years and their families. To serve families from different regions, *One Day Clinic* serves children ages 4-18 years who reside out-of- state and *Rural Outreach Sites* serves Colorado communities with little access to specialized diagnostic services. In addition, CHCO has the only pediatric clinic for neurogenetic disorders in the Rocky Mountain region, where trainees may work with clinicians who are skilled in highly specialized diagnosis and treatment of complex and rare conditions. In addition to the *Neurogenetics Diagnostic Clinic*, there are 10 clinics for specific disorders: 22Q, Fragile X, Chromosome 15q, eXtrordinarY Kids, Neurogenetics (SLC6A1, STXBP1, SCN2A, SYNGAP), Prader Willi, Rett Syndrome, Tuberous Sclerosis, and Turner Syndrome, Neurofibromatosis. Other diagnostic clinics offer trainees single discipline evaluation and diagnosis experience: *MD/NP Single Discipline & Fellows' Clinics, Child Health Clinic DBP evaluation (*a medical model primary care clinic for Medicaid and low-resources families), *Psychology Single Discipline Clinic, Speech Evaluation Clinic*, and *OT Evaluation Clinic.*

Therapies and Interventions

Therapies cand interventions include practicum experiences with LEND and DP faculty providers. *Next Steps Clinic* provides social work consultation to help families of children of all ages and diagnoses to review developmental evaluation recommendations, process the diagnosis, and assist with community referrals and insurance issues. *ABC's of Behavior*, an 8-week group for parents of children of all ages diagnosed with A/DD and behavioral concerns, supports understanding of strategies for managing difficult behaviors and promoting positive behavior. *CHCO* (Developmental Pediatrics) *Outpatient Clinic* offers comprehensive diagnostic evaluation, medication management, and multidisciplinary treatment for children A/DD, learning disabilities, and behavior challenges. *Early Start Denver Model* teaches autistic children and others with significant developmental delays social, communication, and thinking skills in a play-based therapy. *ENRICH* (Enrichment using Natural Resources in Communities and Homes) is a culturally responsive transdisciplinary early intervention home/community visiting program for Part C eligible children and their families to adapt families' daily routines for development and learning. *Behavioral Therapy Clinic/RUBI* (Research Units in Behavioral Intervention) and *Behavior Therapy Group/RUBI*, individual and group therapies, offer parent-coaching on environmental factors and triggers and their consequences to shape positive behaviors. *Cognitive-Behavior Clinic* provides individual and family therapy (12-20 weeks) to help youth (7-17 yrs.) learn to identify and manage anxiety/depression by discovering new coping strategies. *Facing Your Fears (FYF),* an original JFK therapeutic model with multiple empirical replications and adaptations, is a 14-week multifamily CBT group for autistic children, 8-14 years old, who also experience anxiety. Children and families identify worries, develop coping strategies, and are supported to regularly practice facing their specific fears. *Facing Your Fears: IDD* adapts the FYF model to assist autistic teens ages 12-18 years, with intellectual disability

and anxiety manage their anxiety symptoms. Interventions for other specific issues in A/DD include: *Behavioral Sleep Clinic* (evaluation, medication, desensitization, and sleep hygiene) for children with A/DD and major sleep issues with behavior concerns; *Pediatric Oral Feeding Clinic* (staffed by a physician, occupational therapist, speech therapist, nutritionist, and pediatric dentist) serves children of all ages experiencing feeding and eating difficulties; *Toileting Clinic,* provides collaborative nursing and psychology assessment, and medical and behavioral strategies to encourage positive toileting habits (for children with A/DD or intellectual disabilities of all ages); and *Birds and Bees Clinic* (a parent coaching and sex education model in A/DD). *HOPE (Helping Oncology Patients Excel) Cancer Survivorship Program* is a multidisciplinary clinic that helps pediatric cancer and bone marrow transplant survivors manage long-term side effects. The *Transition Clinic* serves youth with intellectual disability or A/DD (ages 12–26) and their families plan for the transition to adult health care. A developmental pediatrician and social worker assess transition readiness and adaptive functioning to develop an individualized transition plan with recommendations and resources for patients, families, and adult providers. Trainees and faculty from the JFK Partners Colorado Office of Employment First contribute vocational recommendations to support successful transition to adulthood.

Community Partner Sites

Placements are also available at community partner sites and numerous Colorado schools (see Attachment 3: Agreements with other entities). *UC Health Behavioral Neurology Adult I/DD Clinic* in Denver offers a consultative model for adults with intellectual disabilities and A/DD emphasizing care coordination. *Talking Together, Inc*., offering OT experiences, serves the Front Range of Colorado with in-home early intervention services that use a transdisciplinary primary provider and parent coaching model. *PT School Practicum* is a discipline-specific clinical experience for PT trainees in Aurora Public Schools. *Adam's Camp*, a Denver area nonprofit, delivers adventure programs and intensive, multidisciplinary therapies for children and young adults with intellectual disabilities and A/DD. OT & PT practicum placements occur at urban and rural camp locations. *Rise School of Denver* hosts clinical practicum, including staff and family consultations, delivered by DBP and PT trainees in an inclusive, therapeutic preschool for children, ages 1-5 years, with autism, and genetic, seizure, and other disorders. At *Marion Downs Center,* in suburban Greenwood Village, Speech Language and Audiology trainees can provide diagnostic, specialized testing, and treatment services for hearing-related, auditory processing, communication, and swallowing challenges. These trainees may also observe developmental hearing, vision, and speech/language screenings at the Marion Downs *KidScreen* program for children aged birth to 18 on-site in schools, preschools, and Head Start programs in metropolitan Denver.

Other Practical Experiences

Non-clinical practicum experiences include community, policy, systems improvement, and research experiences tailored to trainees' Individual Training Plan (ITP) goals and often aligned with Scholarly Projects. The JFK Partners Director, Training Co-Directors, and Discipline Directors connect trainees with state, local, and community partners whose needs match trainee interests and expertise. We have letters of agreement with major Colorado school districts including the large urban systems of Denver Public School District, Cherry Creek School District, Littleton Public School District, and Colorado Springs District 11 for research and technical assistance that may create opportunities for trainee participation with faculty as well. Other experiences well suited to trainees in either clinical or non-clinical and lived

experience disciplines are exemplified by recent examples. An OT trainee partnered with Assuring Better Child Health & Development, a Title V MCH contractor, to design and deliver Spanish-language family training on developmental screening, early intervention, and reducing stigma surrounding referrals. A Family trainee collaborated with a Denver Public School in an autism awareness event for staff, students, and families, while another partnered with Parent-to-Parent of Colorado to update a statewide family resource on transition planning.

Research Competency

Most trainees in professional disciplines enter LEND with foundational or advanced research skills, whereas Family and IwD trainees often require introductory instruction and individualized mentoring in the critical appraisal and application of research. Research competencies are developed through didactics, research practica, and scholarly projects. Instruction begins in *Key Concepts 1* and is supplemented, as appropriate, by CITI training in human subjects research, health information privacy and security, and social and behavioral research, University training modules, and Interdisciplinary Research Seminar sessions. Critical appraisal and application of evidence-based literature are reinforced throughout the curriculum.

Each LTT completes an individualized Scholarly Project, guided by one-to-one mentoring from faculty throughout project conceptualization, implementation, analysis, and dissemination. Projects may include independent studies, faculty-led research, quality improvement (QI), resource development, technical assistance, or secondary analyses of existing datasets. Research practica and scholarly projects provide experience in literature review, IRB applications, data collection, analysis and interpretation, and dissemination through manuscripts, posters, or podium presentations. Each project undergoes a fall review via an oversight committee, with the trainee's mentor and two faculty members from other disciplines.

Scholarly projects reflect trainees' backgrounds and career goals. For example, in 2025–26, an IwD trainee with no prior research experience designed and conducted a statewide survey of travel-training methods for individuals with disabilities. Other trainees participate in faculty research, including replications and adaptations of *Facing Your Fears (FYF)*, or analyze multiyear FYF and *Study to Explore Early Development* datasets to address new research questions. LTTs present their research, QI, and other scholarly projects at the annual JFK Partners Scholarly Project Symposium.

Supplementary Experiences

An intensive four-hour *Facing Your Fears* summer workshop introduces trainees to JFK Partners' signature cognitive behavioral therapy for co-occurring autism and anxiety. As appropriate to their training goals, trainees complete required University modules (e.g., HIPAA, data security, and sexual harassment prevention). LTTs also participate in conferences, including the JFK Partners Annual Autism Conference and professional meetings (e.g., International Society for Autism Research). Trainees attend Colorado IDD Awareness Day and Disability Rights Advocacy Day. As funding permits, 2–4 trainees attend the AUCD Conference and the Washington, DC Disability Policy Seminar.

Partnerships

JFK Partners maintains partnerships with state, local, national, and international agencies and organizations that support recruitment, training, continuing education, technical assistance, research, dissemination, and systems improvement (see Attachment 3: Agreements with other entities). Example collaborations include the CU School of Medicine Department of Psychiatry

and the Autism Society of Colorado (recruitment and training); the Colorado Title V MCH Program, Assuring Better Child Health and Development (ABCD), Alliance Colorado, Colorado school districts, and the School-Based Health Center Program (CE); and the Colorado Developmental Disabilities Council and Disability Justice Colorado (TA and systems improvement). Collectively, these partnerships strengthen access to evidence-based services and coordinated systems of care for individuals with disabilities and their families (IwD&F).

JFK Partners also collaborates extensively with federally funded training and research programs, including Autism CARES initiatives. Through the Pacwest Regional LEND Consortium, our 12 western programs collaborate on trainee exchanges, the annual Pacwest Regional LEND Consortium Trainee Conference, and joint presentations at the Association of University Centers on Disabilities (AUCD) and other national meetings. We have also partnered on HRSA-funded AIR-B and AIR-P projects and welcome future collaborations with Autism CARES-funded programs. The Birds and Bees Clinic, now a LEND training site (Table 2), originated through AIR-P funding, awarded to a JFK Partners LEND graduate. JFK Partners also participated in the LEND Program Quality Improvement (LPQI) Network and will continue participation should the network be renewed, incorporating network measures and findings into our internal CQI program.

Partnerships also strengthen dissemination of LEND products. Goal B, Objective B3, commits JFK Partners to sharing reports, resources, and other products statewide and nationally. A recent example was an Interdisciplinary Technical Assistance Center (ITAC)-funded collaboration in which JFK Partners led the development of nine disability e-learning modules on autism, co-occurring conditions (e.g., hearing loss and mental health), and transition to adulthood. Principal Investigator Dina Johnson, MA, coordinated module development with Pacwest Regional LEND Consortium faculty and individuals with disabilities and their families, managed implementation and technology for the consortium, developed the user guide, presented the project at the AUCD annual conference, and oversaw transfer of the modules to AUCD for national dissemination.

As Colorado's LEND program, partnerships with state agencies, community organizations, and individuals with disabilities and their families are essential to ensuring that our work remains responsive to community priorities and local decision-making. During development of this proposal, JFK Partners Director Dr. Judy Reaven proposed to our Developmental Disabilities Network partners—the Colorado Developmental Disabilities Council and Disability Justice Colorado— that we collaborate to conduct series of statewide listening sessions as a platform to give voice to Colorado IwD&F and to inform our collaborative priorities. Individuals with autism, intellectual and developmental disabilities, and family members around the state are invited to share their needs, concerns, priorities, and recommendations in small group meetings without interruption. Service providers may participate in the listening session with IwD&F as silent listeners, a role for which they are earlier prepared by the facilitator. At the end of the listening portion, providers convene in facilitated follow-up provider discussion to reflect on what they learned. Two sessions were conducted to date in 2026. Dr. Reaven will prepare a listening session series summary to share with participants and the public, consistent with participant requests. The series findings will continue to guide program planning, implementation, and evaluation throughout the project period.

Additional examples of collaborative partnerships are described below in the Continuing Education and Technical Assistance sections.

Continuing Education and Technical Assistance

Continuing Education (CE)

JFK Partners will deliver at least five continuing education (CE) events annually for MCH professionals and others serving individuals with A/DD, with at least three offering CE credit and a cumulative annual audience of at least 200 participants. Most events are free and open to the public, expanding access to evidence-based education for professionals, individuals with A/DD and their families (IwD&F), and community partners.

CE priorities will be guided by the 2026 JFK Partners and community needs assessments and refined annually with community partners to address emerging workforce and public health needs and promote interdisciplinary collaboration across health, education, and community systems. At least three events each year will be developed in collaboration with the Colorado Department of Public Health and Environment Title V MCH Program (Attachment 3) to address CYSHCN priorities and strengthen the MCH workforce, including rural and frontier care coordinators and nurses. Additional topics and presenters will be selected with IwD&F, state agencies, community organizations, provider groups, and partners such as the Colorado Developmental Disabilities Council and Disability Justice Colorado. Potential topics, identified in needs assessments, may include developmental disabilities, mental health in A/DD, neurodiversity-affirming care, community resources and benefits, caregiver resilience, transition to adulthood, and aging caregivers. From topic identification through planning, presentation, and evaluation of CEs (and all other goals and objectives), partnerships with individuals with disabilities and their family members is a core principle of the JFK Partners mission that is replicated throughout JFK Partners program.

CE will be delivered through conferences, webinars, and other virtual or in-person formats. to expand access to rural, frontier, and low-resource areas and communities. JFK Partners will also collaborate with the western region Pacwest Regional LEND Consortium and other Autism CARES-funded programs by contributing content, speakers, moderators, and discussants. Faculty and trainees will also participate in and contribute to educational programs hosted by partner organizations locally and nationally. Using its established registration, evaluation, marketing, and accessibility systems, JFK Partners will provide CE credit, closed captioning, Spanish interpretation as appropriate for our hosted events, and will disseminate announcements broadly through our own and partner listservs, social media, the program website, and our JFK Partners YouTube channel. Participant evaluations will inform required reporting, annual topic selection, and continuous quality improvement. Selected webinars will be archived on the JFK Partners website to extend their reach throughout the project period.

Technical Assistance (TA)

JFK Partners provides technical assistance (TA) to local, state, and national agencies and organizations to strengthen services and systems for individuals with disabilities and their families (IwD&F). Projects are identified though responsive engagement and proactive outreach. Requests for TA arise through faculty scholarship, professional meetings, alumni networks, and outreach through JFK Partners listserv and Community Advisory Council, and longstanding community partnerships. Services range from consultation on specific clinical, programmatic, or systems issues to sustained collaborations supporting implementation of evidence-based

practices, evaluation, policy development, and systems improvement. JFK Partners faculty are recognized for expertise in evidence-based A/DD clinical practice, research, evaluation, and policy, serving as consultants, collaborators, advisors, and reviewers. TA engagements are typically collaborative, advancing evidence-based and respectful neurodiversity-affirming practices while strengthening relationships with community partners and informing ongoing program improvement through reciprocal learning. Examples of continuing or representative TA activities during the 2021–26 LEND cycle are summarized below.

Evidence-Based Practice Implementation

A notable example of JFK Partners' technical assistance is dissemination and implementation of Facing Your Fears (FYF), a gold-standard, evidence-based intervention for anxiety in autism. Under the leadership of Director Judy Reaven, PhD, FYF training and implementation support have been provided to clinicians, agencies, and service systems locally, nationally, and internationally. Over time, technical assistance has supported implementation across Colorado; university and care networks in Alabama, Georgia, Maryland, New Mexico, New York, North Carolina, Ohio, Pennsylvania and Rhode Island; multiple Canadian provinces (Alberta, British Columbia, Nova Scotia, Ontario, and Saskatchewan); and programs in Argentina, Ecuador, Ireland, Japan, Kuwait, New Zealand, and Singapore. During the current LEND cycle, additional technical assistance was provided to the Maine Behavioral Health System, Oregon providers, and a collaborator adapting the FYF treatment manual for Spanish-speaking populations.

Systems Consultation

JFK Partners faculty and staff provide technical assistance to strengthen organizations and systems serving individuals with disabilities in Colorado and nationally. The Colorado Office of Employment First (COEF) advances equitable, competitive, integrated employment through systems change, professional training, policy development, and removal of barriers to employment for youth and adults with disabilities. Within Colorado, COEF has strengthened the work of six state agencies, including the Colorado Department of Health Care Policy and Financing and the Division of Vocational Rehabilitation; led the statewide Benefits Counseling Collaborative; trained employment professionals; provided leadership to the Colorado Association of People Supporting Employment First (Colorado APSE) in communications, policy, and outcomes-based practice; and convened the statewide community of practice for certified benefits planners. Beyond Colorado, COEF has supported implementation of Employment First principles in Arizona, Hawaii, and North Carolina.

JFK Partners faculty and staff also provide technical assistance to state agencies, advisory boards, and community organizations. During the current cycle, they served as advisors to the Colorado Office of Behavioral Health, the Colorado Traumatic Brain Injury Trust Fund Board, the University of New Hampshire START Advisory Committee, and local early intervention programs. Faculty and staff, including LEND graduates, also supported initiatives promoting inclusion, quality improvement, disability research, and public policy through organizations such as Special Olympics, a Wyoming disability research collaborative, and California partners California partners working to strengthen legislation and service quality.

Professional Leadership

JFK Partners faculty technical assistance helps shape the A/DD evidence base through editorial leadership and peer review. 2021–26, faculty served on the editorial boards of eight professional journals and reviewed manuscripts for twelve additional journals spanning autism, developmental disabilities, pediatrics, psychology, early intervention, and public health. Representative journals include *Focus on Autism and Other Developmental Disabilities*, *Journal of Autism and Developmental Disorders*, *Autism Research*, and the *American Journal on Intellectual and Developmental Disabilities*. Additional editorial and reviewer service included journals in psychiatry, nutrition, speech-language pathology, and preventive medicine.

HIGH-LEVEL WORK PLAN

The overview below details the three LEND goals and objectives, activities to achieve the objectives, timing, and key JFK Partners personnel responsible for implementation. Most objectives include **community partners** in one or more aspects of delivery, from planning through evaluation (see Organization Information, Table 6, and Attachment1. Work Plan presents more details on specific activities and measurement).

TABLE 3. High-Level Work Plan	
JFK Partner Leads, abbreviated PD = Project Director CDT = Co-Director(s) of Training E = Evaluator TC = Training Coordinator RM = Research Manager	
GOAL A. Increase access to evidence-based screening, diagnosis, interventions, and supports for individuals with A/DD and their families (IwD&F) by preparing interdisciplinary, person- and family-centered leaders through competency-based training and community partnerships.	
Objectives: Outcome & Timeframe	***Key Activities & JFK Leads*** *(Activity labels (a, b…) match Attachment 1 labels.)*
Obj. A1. Recruit and educate trainees in a competency- & evidence-based LEND curriculum (didactics, clinical, research, & community experiences) with active partner engagement to increase evidence-based A/DD screening, evaluation, diagnosis, & interventions for children who (may) have A/DD. *Outcomes:* At least (≥) 18 LTTs in IwD, Family & 8 other disciplines, 10 MTTs, & 100 STTs trained; ≥ 75% of LTTs and sampled MTTs & STTs acquire new knowledge/skills & are satisfied with training; ≥ 6 active IwD&F or community partners. Yrs 1-5, summer, fall, spring semesters	a. Update & implement recruitment plan & selection procedures; b. Train an interdisciplinary LTT cohort in the comprehensive LEND curriculum; c. Train MTTs in selected LEND components; d. Train STTs in selected LEND components; e. Update & offer a comprehensive didactic LEND curriculum with partners; f. Deliver faculty & trainee clinical services/practica to increase evidence-based A/DD screening, evaluation, diagnosis, & interventions; g. Deliver research & non-clinical practical experiences that address trainee goals & learning needs in research and scholarship; h. Administer curriculum QI measures/focus group/ evaluations; i. Review syllabi for community engagement in didactic courses

	Leads: PD, CDTs, Discipline Directors; also, for specific selected activities, TC, E, other Faculty, Clinical Supervisors
Obj. A2. Obtain alumni assessments of LEND training value and impact. *Outcomes*: ≥ 75% respond to survey; ≥ 80% recommend LEND; ≥ 70% use skills; 60% report lasting impact. Yrs 1-5, spring (a); Yrs 2-4 (b)	a. Administer 2, 5 & 10 NIRS graduate LTT survey; b. Explore strategies for alumni networking to encourage survey response *Leads:* TC, CDT, E; also PD for b
GOAL B. Strengthen the MCH workforce and systems of care, in partnership with individuals with A/DD and their families (IwD&F) and community partners, to improve access to competent, responsive, and evidence-based services and supports.	
Objectives: Outcome & Timeframe	***Key Activities & JFK Leads***
Obj. B1. Conduct evidence-based CE with IwD&F and agency partners to increase workforce knowledge and skills. *Outcomes*: ≥ 5 CE events (3 for credit); ≥ 200 participants across urban and rural areas; ≥ 75% of participants are satisfied with content; ≥ 75% report increased/refreshed knowledge/skills; ≥ 50% intend to apply recommended practices applicable to role. Yrs 1-5, ongoing	a. Conduct partner outreach to define CE agenda/OEM: Annual CE agenda & partner list: b. Host CEs with IwD&F and agency partners; c. Administer CE participant evaluations *Leads:* PD, CTD, DD, Faculty; also TC & E for c
Obj. B2. Promote strong partnerships among IwD&F, state, local, and community disability interests to increase evidence-based systems responsive to community priorities. *Outcomes:* ≥ 6 partner programs; ≥ 75% of sampled TA projects report progress in goals/satisfaction. Yrs 1-5, ongoing	a. Conduct outreach to state agencies, Title V MCH programs, schools, other community/ national partners to identify collaborative projects & partners; b. Deliver collaborative TA/ consultation with IwD&F and partner agencies to infuse evidence-based practices in systems & services; c. Administer TA/consultation evaluations *Leads*: PD, CDT, Faculty; also E for item c
Obj. B3. Share evidence-based A/DD information & resources, in accessible & plain language formats, including a research newsletter, annual report, & other products to inform IwD&F and related audiences and maximize LEND impact. *Outcomes:* ≥ 15 project products; product accessibility for lay audiences "substantially achieved." Yrs 1 - 5, newsletter (fall), assorted project products (as available), annual report (Yrs 2–5, fall); CAC rates accessibility of sampled products (May)	a. Create consumer summaries of LEND project accomplishments; b. Disseminate faculty & trainee projects & resources; c. Archive selected CEs for on-demand delivery; d. Acquire sampled program product accessibility ratings (from JFK Partners CAC members). *Leads:* For various activities a-d: PD, CDT, RM, E, Faculty, Trainees

GOAL C. Demonstrate responsible stewardship of LEND resources through accountable grant management to maximize LEND impact for IwD&F.	
Objectives: Outcome & Timeframe	***Key Activities & JFK Leads***
Obj. C1. Manage data collection to enable accurate performance reporting. *Outcome*: 100% of data reporting requirements met. Yrs 1-5, by defined MCHB deadlines	a. Administer data collection system; b. Manage data input for reporting *Leads:* CTD, E, TC
Obj. C2. Conduct and report comprehensive program evaluation and continuous quality improvement (CQI) findings to assess and strengthen progress towards LEND goals. *Outcomes:* At least 80% of overall program measures are positive in all program performance domains; no. of timely data/report submissions: ≥ 1 CQI change package tested. Yrs 1–5; Implement, fall, spring, summer; review; Report to PD, TD, DD - ongoing; & Retreat (June)& CAC report/eval (May); federal reporting, as MCHB determined	a. Analyze evaluation data (recruitment, training program, NIRS LTT survey, CE, TA, JFK Partners CAC feedback); b. Summarize program evaluation findings; c. Report to and seek JFK Partners CAC community comprehensive program performance evaluation each May; d. Collaborate in LEND Program Quality Improvement (LPQI) data submission; e. Implement CQI change package and data analysis; f. Update leadership on evaluation findings, CQI modifications/ adaptations, and recommendations; g. Conduct data-driven program review in JFK Partners Training Program Retreat to improve LEND program *Leads:* E, PD, TC, CDT
Obj. C3. Manage LEND resources responsibly through fiscal oversight and data-informed resource allocation to support achievement of program objectives. *Outcomes:* 100% required fiscal reports submitted accurately and on time; 0 material audit or compliance findings. Yrs. 1 -5, ongoing	a. Monitor budgets & expenditures for fiscal oversight quarterly in collaboration with the Department of Pediatrics Office of Grants & Contracts; b. Apply fiscal data (and C2 evaluation findings) to planning & allocation of program resources *Leads:* PD, CDT
Note. *See Attachment 3, Agreements with other entities for agency partners & committed roles in planning, design, and implementation of activities.*	

How activities will be implemented and objective achieved

JFK Partners will implement all program activities listed above through an evidence-informed, partnership-driven, and continuously improving approach. We are committed to translating the best available science into practice while recognizing that individuals with autism and developmental disabilities and their families are our essential partners in planning, implementation, evaluation, and systems improvement. Their lived expertise, together with the perspectives of community agencies, public systems, and trainees, strengthens our opportunity to create meaningful outcomes for our State.

Our implementation philosophy emphasizes reciprocal learning. We are motivated not only to share knowledge and build workforce capacity, but also to learn from our partners,

trainees, evaluation findings, and continuous quality improvement processes. The challenges we'll encounter offer opportunities to refine educational practices, strengthen partnerships, and improve systems of care.

Across training, continuing education, technical assistance, research, and dissemination, we are guided by scientific rigor, responsible stewardship, accountability, responsiveness to evolving workforce and community needs, and a commitment to neurodiversity-affirming, person- and family-centered practice. These principles provide a consistent framework for implementing program objectives and activities while ensuring that LEND resources produce meaningful and sustainable improvements in the lives of individuals with autism and developmental disabilities and their families.

RESOLVING CHALLENGES

JFK Partners designed this proposal to address anticipated challenges; however, additional issues will inevitably arise during the five-year project period. We will use continuous quality improvement (CQI) processes to identify, monitor, and respond to emerging needs.

Trainee accommodation

JFK Partners has successfully addressed diverse accessibility needs among trainees and faculty (e.g., screen readers, communication devices, reading-level adaptations) since 2015, and we will continue to leverage CQI data, CU Anschutz accessibility expertise, AUCD technical assistance, and emerging technologies to strengthen supports. To support trainee success, we will modify our incoming Trainee Needs Assessments and other tools to identify and accommodate needs earlier in training. Recruitment targets may be increased in years 2–5 to ensure training commitments are met, although retention is the priority.

Differences in trainee preparation.

LEND cohorts include trainees with varied academic preparation, professional experience, and lived A/DD expertise. Leadership Dialogues 1 promotes early cohort cohesion through interactive learning, while subsequent courses use interdisciplinary peer learning, faculty mentoring, tailored supports, and assignment modifications to address differing needs. Clinical practica and scholarly projects provide individualized supervision matched to trainee competencies, with Scholarly Oversight Committees refining projects through interdisciplinary faculty review.

Evolving partnerships

Leadership changes among community partners shifted priorities during the 2021-26 cycle. We will strengthen continuity through formal partnership agreements, regular partner check-ins, shared advisory activities, and continued responsiveness to the evolving needs of individuals with disabilities and their families (IwD&F) in Colorado.

As new challenges emerge, the JFK Partners Director will consult the Training Review Committee, LEND faculty, Developmental Pediatrics leadership, university colleagues, the MCHB Project Officer, AUCD technical assistance, the Pacwest Regional LEND Consortium, and other LEND Directors to identify effective solutions.

PERFORMANCE REPORTING AND EVALUATION

JFK Partners implements a comprehensive performance reporting and evaluation system to fulfill support Performance measurement and reporting, Program Evaluation and Continuous Quality Improvement as described below.

Performance Measurement and Reporting

To *monitor implementation, milestones, and emerging challenges*, Project Director Judy Reaven, PhD, convenes the Training Review Committee at least eight times annually. Members include the Training Co-Directors, Discipline Directors, faculty, Training Coordinator, and Evaluator. The committee monitors trainee recruitment and retention, courses, practica, and scholarly projects, and continuing education (CE) and technical assistance (TA), for progress toward workplan objectives. Quarterly meetings of all Developmental Pediatrics faculty and staff—including JFK Partners core and contributing faculty, staff, and selected trainees—provide additional review of program progress within the broader departmental context. Routine performance monitoring supports timely course corrections and informs continuous quality improvement (CQI). Table 4 (below) summarizes measures used to monitor program implementation and reporting.

To meet *HRSA reporting requirements*, including Non-Competing Continuation Progress Reports and Project Period End Performance Reports, JFK Partners maintains an internal activity database for faculty, staff, and trainees. Data collection forms mirror current LEND performance measures and workforce reporting requirements. Activity data are collected twice annually (January and June), with Training Co-Director Dina Johnson, MA, overseeing data management, internal Excel tracking, and submission to the National Information Reporting System (NIRS). Where NIRS reports do not fully address HRSA reporting needs, supplementary internal reports are generated. Internal data collection procedures are updated as reporting requirements evolve.

Ms. Johnson and Training Coordinator Christine Cook maintain the current and long-term Trainee database, supporting HRSA reporting and annual administration of the two- and five-year graduate surveys. Trainee agreements require maintenance of current contact information, while faculty mentoring relationships, social media, and reminder contacts support follow-up. A new workplan activity (Goal A, Objective A2, Activity A2b) will further strengthen alumni networking and reduce loss to follow-up.

Autism CARES evaluation data are drawn from the internal activity database and, where required, aggregated reports requested from Children's Hospital Colorado clinical programs using secure electronic health record systems. Table 4 presents core performance indicators and data sources.

Table 4. Core Indicators & Data Sources

Indicator	*Data Source(s)*
No. LTTs, LTT disciplines/Cohort composition by discipline	Trainee database
No. Faculty disciplines	Faculty roster
Nos. MTTs & STTs and disciplines	Internal Activity Database: faculty reports

Indicator	*Data Source(s)*
Nos. Faculty & trainees conducting A/DD diagnostic evaluations & therapies	CHCO/Department of Pediatrics Report; Internal Activity Database: faculty/trainee reports
Former LTTs lost to follow-up/response rate	Trainee database; Former LTT Survey report
Nos. CE events, participants & topics (including A/DD focus)	Internal Activity Database: faculty reports; NIRS reports
No. TA activities & participants & topics/issues (including A/DD focus)	Internal Activity Database: faculty/trainee reports; NIRS reports
Similar indicators: LEND Audiology Supplement (*if funded;* excluding cohort & faculty composition by discipline)	Internal Activity Database: faculty & trainee reports; Trainee, CE, TA data

Annual report of measures aligned with program goals

Collection of these data enable *annual reporting on measures that align with program goals and objectives*, such as training an LTT cohort of appropriate size and composition to meet funding recruitment commitments; breadth and reach of the JFK Partners CE program for the MCH workforce; scope and intensity of opportunities and demand for LEND subject matter expertise and TA, technical assistance provided to the community. These data also facilitate CARES Act reporting on preparation of the A/DD workforce and A/DD screening, diagnose, evidence-based therapies delivered in the context of the program, as well as documentation of LEND Audiology Supplement productivity (if funded).

Data management and secure data storage

Trainee records and other program data are maintained in CU Anschutz institutionally approved, access-controlled electronic systems with encryption, role-based permissions, regular backups, and compliance with applicable University, FERPA, HIPAA, and information security policies, overseen by the CU Anschutz Office of Information Technology. The CU Anschutz Office of Digital Learning supports Canvas, the program's learning management system, to maintain security of trainee educational data.

Report completion and submission

DGIS reports, prepared by Ms. Johnson and Evaluator Geraldine Steinke, PhD, with oversight by Dr. Reaven. Financial reporting is completed in collaboration with the Department of Pediatrics Office of Grants and Contracts by Dr. Reaven and Ms. Johnson.

Program Evaluation

JFK Partners is committed to rigorous program evaluation, reporting, and Autism CARES Act evaluation. The LEND evaluation framework assesses program implementation of training, continuing education (CE), technical assistance (TA), and project outcomes. It supports HRSA-required Discretionary Grants Information System (DGIS) and Autism CARES, using NIRS output and JFK Partners program-specific measures to monitor progress toward goals, objectives, and performance measures and to inform continuous quality improvement (CQI).

Evaluation findings are used to refine curricula, CE, partnerships, and program strategies to strengthen the MCH workforce and improve access to evidence-based systems of care for IwD&F.

Program evaluation is directed by Geraldine Steinke, PhD, a psychologist with more than 40 years of experience in program development and evaluation in clinical and educational settings. Since 2008, she has served as evaluator for JFK Partners LEND and several other HRSA-funded programs. Her previous experience includes evaluation of a large New York City community mental health center, three CU Anschutz College of Nursing programs, and a 12-year national study of quality improvement in state childcare regulations funded sequentially by MCHB and CDC. Dr. Steinke reports to Project Director Judy Reaven, PhD, and collaborates with Training Co-Director Dina Johnson, MA, and Training Coordinator Christine Cook to implement the evaluation plan and reporting.

Evaluation is integrated throughout the project workplan (Attachment 1), with data management and use addressed under Goal C, Objective C2. All program goals include SMART (Specific, Measurable, Achievable, Relevant, and Time-bound) objectives, with activities linked to expected outcomes, outcomes, evidence, or measures, and the timeline for implementation. Surveys and questionnaires assess outcomes, satisfaction, and program quality while soliciting comments, suggestions, and targeted feedback to inform continuous quality improvement, including the strengths and opportunities for improvement in training, continuing education, technical assistance program components. Evaluation incorporates the perspectives of trainees, alumni, faculty, and partners to assess the quality, effectiveness, and impact of LEND training, continuing education, and technical assistance. Table 5 summarizes the major evaluation domains, timing, and data sources.

TABLE 5. Program Evaluation Domains, Evaluation Timing & Data Sources

Abbreviations Used
Timing: S = summer F = fall Spr - spring
Data source: T = trainees A = alumni F = faculty/mentors C = community/partners

Program Domain	*S*	*F*	*Spr*	*Data Source/Instrument(s)*
Trainee MCH Leadership	✓		✓	T/F: MCH & JFK Competency Ratings; LPQI
Trainee JFK Core Competencies	✓		✓	T/F: JFK Competency Ratings/Reflections
Tr/F A/DD skills	✓	✓	✓	T/F: Course assignments/preceptor ratings; Exit Eval; JFK Competency Ratings/Reflections
Trainee Patient- & Family-centered skills			✓	T/F: Mentor/Preceptor ratings; LPQI; Competency Reflections
Appropriate Cultural/Linguistic knowledge & skills			✓	T/F: Mentor/Preceptor ratings; Competency Reflections; Exit Eval.
Didactic quality & contribution	✓	✓	✓	T: Course & Exit Evals; Focus group

Program Domain	*S*	*F*	*Spr*	*Data Source/Instrument(s)*
Clinical supervision quality; objectives met	✓	✓	✓	T/F: Practicum supervision ratings; Competency Reflections; Focus group; Exit Eval
Research/Scholarly Experience			✓	T/F: Focus group; Scholarly Project
Community-based projects			✓	T/F: Faculty review; Focus group; Exit Eval; Scholarly Project
Policy Experiences		✓	✓	T: LD2 and KC course ratings; Focus group; some Scholarly Projects
Cumulative Trainee feedback			✓	T: Focus group; Competency Reflections; Year-End Evals
STT experience		✓	✓	T: Disability Dialogues ratings
MTT experience/skill acquisition	✓	✓	✓	T/F: Assorted/sample, depends on experience
Continuing Education	✓	✓	✓	C: JFK Partners CE Eval Form; Internal Activity Database (registrations)
TA/Consultation		✓	✓	C: JFK Partners TA Eval Form (intensive/ extended projects); Internal Activity Database (reviewer/advisory roles)
Research/Scholarly experience	✓	✓	✓	F/T: JFK Research Table; Scholarly Projects
Project product dissemination nos. & quality/accessibility	✓	✓	✓	C/F: JFK Partners CAC evals; JFK Research Table, website product views/downloads
Long-tern Graduate Outcomes			✓	2 – 5 Yr. Former LTT Surveys
Comprehensive program review			✓	C: JFK Partners CAC eval
Overall LEND goal attainment			✓	Cumulative evaluation data

Evaluation timeline

To give context to the table above, the phases of evaluation implementation in brief are 1) Summer-fall - initial trainee competency assessment; 2) Course evaluation administration - upon course completion (summer, fall and spring); 3) January - Initial data sweep for faculty/trainee activity reports (Internal Activity Database); 4) February – initiation of Former LTT survey prompts; CE evaluation - as sessions conclude; 5) Early spring - focus group; 6) Spring - trainee competency reassessment/reflections, and Exit evaluation administration; 7) May - JFK Partners comprehensive program evaluation; 8) Late spring - TA evaluation administration (some arrive earlier), and 2nd data sweep for faculty/trainee activity reports. (Attachment 5. Tables, charts, and curriculum/syllabus, includes a summary of the schedule of training program delivery and evaluation in that context.)

Tracking attainment of goals and objectives and program impact

Near the end of the training year, the LTT cohort evaluate their immediate training outcomes in rated and qualitative tools. These include the focus group, competency reassessment

and reflections, and an Exit Evaluation. Compiled rating and open-ended data from CE and from TA evaluations are summarized for outcome and impact data, A comprehensive community assessment of JFK Partners annual performance occurs in May with the JFK Partners Community Advisory Council, whose members are representative of Colorado IwD, families, and A/DD agencies. In the new cycle, CAC members will also evaluate the quality and accessibility of sampled faculty and trainee products.

Evaluation barriers

The primary barrier we have encountered to program evaluation has been occasionally low or tardy response rate, which can occur for various reasons with our different instruments (e.g., technological glitches, competing demands for responsiveness in peak period of activity). Outreach reminders to encourage response, whether electronic or personal, has been our most effective tool to encourage response. We have also used alternative formats, short form surveys or personal interviews for different circumstances.

Sharing project results

In addition to required HRSA reporting, JFK Partners disseminates project findings and products to professional, community, and public audiences through multiple channels. Professional dissemination includes peer-reviewed publications; podium, poster, and workshop presentations at scientific and professional meetings; AUCD network communications; webinars with continuing education credit; and presentations to partner organizations, advisory bodies, and professional workgroups, meeting with other grantees (e.g., Pacwest Regional LEND Consortium). Community dissemination includes informal updates to collaborating agencies (e.g., Colorado Developmental Disabilities Council), annual presentations to the JFK Partners Community Advisory Council, and distribution of trainee scholarly projects, implementation tools, practice guides, resource materials, and technical assistance products. Public dissemination includes an annual program report, research newsletter, website and social media postings, archived webinars and educational resources, university communications, and other web-based outreach. These activities promote translation, dissemination, and implementation of evidence-based practices to strengthen systems of care and extend the impact of LEND.

Continuous Quality Improvement

JFK Partners uses continuous quality improvement (CQI) to strengthen all aspects of program implementation, from preparing future leaders to improving the MCH workforce and systems of care for individuals with autism and developmental disabilities and their families. CQI draws on performance measures, program evaluation, stakeholder feedback, and partner input to assess the quality and effectiveness of training, continuing education, technical assistance, and partnerships, and to guide ongoing refinement of program activities.

CQI findings are translated into targeted improvement strategies and monitored for their effect on program implementation and outcomes. During the current project period, for example, mentor communication was identified as an opportunity for improvement. A CQI change package emphasizing timely, clear communication and support for trainee goals, including career planning, resulted in incremental improvements in trainee ratings of mentor effectiveness. Expansion of scholarly project opportunities to selected Advanced MTTs also strengthened collaboration between the LEND Audiology program and a state agency, creating additional opportunities for partnership and training.

Looking forward, JFK Partners will continue using CQI to respond to evolving workforce and community needs. A priority for the upcoming project period is incorporating findings from facilitated community listening sessions (described under Partnerships) to strengthen relationships with Developmental Disabilities Network partners and ensure that the perspectives, priorities, and recommendations of individuals with disabilities and their families directly inform program planning, implementation, and improvement.

If our proposal is successful, JFK Partners will strengthen its culture of continuous quality improvement by establishing regular instructor learning meetings, a small community of practice, throughout the academic year. Building upon recommendations from the June 2026 Faculty Retreat, course instructors will meet during each semester to share observations across courses, identify emerging trainee needs, exchange effective teaching strategies, and coordinate timely improvements in support of each LEND cohort. These discussions will complement formal evaluation activities by promoting ongoing reflection, shared learning, and rapid implementation of evidence-informed improvements.

SUSTAINABILITY

The JFK Partners proposal is designed to sustain LEND's impact on Colorado's system of care for children and youth with A/DD and their families by integrating program activities into existing institutional, community, and systems-level infrastructure. We will pursue diversified funding through state, federal, foundation, and, when appropriate, fee-based support for activities such as continuing education and technical assistance while minimizing barriers to access and promoting evidence-based practice.

Ongoing evaluation will document workforce and systems-level outcomes to demonstrate impact, support continued investment, and guide replication and systems improvement. If funding is reduced, activities will be strategically prioritized based on evaluation findings and partner needs. We will preserve core curricula, training infrastructure, and community partnerships within JFK Partners or, when appropriate, institutionalize key components within the CU School of Medicine, particularly Developmental Pediatrics and Psychiatry. Training materials, evidence-based practices, and faculty- and trainee-developed resources will be sustained through adaptation, transfer, and shared implementation with state and community partners.

JFK Partners has a strong record of translating evidence-based practices into clinical care and service systems. Examples include Director Judy Reaven's PCORI-funded multi-state comparative effectiveness study informing individualized program selection for autistic students with anxiety (*Facing Your Fears* and *Zones of Regulation*), adaptation of *Facing Your Fears* for autistic youth with intellectual disability and anxiety, and dissemination of the ENRICH transdisciplinary early intervention model to private practice. Continuing education, technical assistance, the annual program report, and the research newsletter will further extend evidence-based A/DD practices beyond the grant period.

ORGANIZATIONAL INFORMATION

JFK Partners Mission, Structure, and Scope of Activities

Mission

Mission alignment and positioning within the University of Colorado Anschutz School of Medicine (CU SOM) Department of Pediatrics (PEDS) supports JFK Partners current and proposed LEND program, from planning and execution to evaluation and accountability. *Mission.* The mission of CU SOM PEDS is to improve children's health through high-quality, coordinated programs of patient care, education, research, and advocacy. JFK Partners' more targeted mission is to lead the way in evidence-based interdisciplinary clinical care, education, research, and community partnerships to improve the lives of children and youth with A/DD and their families. Fundamental to our mission is a commitment to person-centered, family- and community-centered care that is culturally and linguistically responsive, while promoting access to services for all populations, including the medically underserved, also consistent with the Departmental focus.

Structure

The University of Colorado (CU) is a state postsecondary institution accredited by the Higher Learning Commission, with campuses in Boulder, Colorado Springs, Denver, and the CU Anschutz Medical Campus in Aurora, which together enroll more than 67,800 students and employ 7,600 full-time faculty. The 227-acre Anschutz campus educates approximately 4,500 students annually in more than 40 degree programs on campus. They include the Colorado School of Public Health (a collaboration of CU, Colorado State University, and the University of Northern Colorado), and the CU Colleges and Schools of Nursing, Dental Medicine, Biomedical and Clinical Sciences, Pharmacy, and Medicine, and are a rich recruiting environment.

The CU School of Medicine (SOM) provides an exceptional academic and clinical home for LEND, with more than 30 multidisciplinary centers and 24 departments, including Pediatrics (PEDS) and Psychiatry (PSYCH), where most LEND faculty hold primary appointments. PEDS is ranked first nationally in NIH funding (Blue Ridge Institute for Medical Research), and the SOM receives more than $516 million annually in research funding. Campus clinical training sites are integrated with Children's Hospital Colorado (CHCO), UCHealth, and the Rocky Mountain Regional VA Medical Center, also located on the Anschutz campus.

JFK Partners is housed within the Developmental Pediatrics Section, one of 22 Pediatrics subspecialties. Director Judy Reaven, PhD, reports to Section Head Ann Reynolds, MD, while LEND faculty and staff report to Dr. Reaven for their assigned LEND effort (see Attachment 4. Organization Chart for the location of our program in the CU Anschutz academic hierarchy).

Current scope of activities

JFK Partners is funded through HRSA MCHB LEND, a LEND Audiology Supplement, ACL (UCEDD), a Department of Pediatrics UCEDD match, and grants, contracts, and collaborative agreements that support research and specific evaluation and technical assistance projects. We provide interdisciplinary training for LTTs, MTTs, and STTs through didactic, clinical, and community-based experiences led by qualified faculty. We also offer accessible continuing education for professionals, individuals with A/DD, and their families. Faculty and trainees advance evidence-based practice and policy, informed by lived experience, through clinical care, research, scholarship, and collaborative technical assistance. Many clinical experiences occur in JFK Partners clinics and other Developmental Pediatrics programs at Children's Hospital Colorado, serving individuals and families from varied racial, ethnic, cultural, linguistic, and socioeconomic backgrounds.

Infrastructure Support and Financial Oversight

The CU School of Medicine provides the infrastructure, policies, and administrative resources necessary for effective and accountable LEND implementation. Key supports include access to CU Anschutz disciplines for trainee and faculty recruitment; Information Strategy and Services; and campus offices (e.g., Disability, Access & Inclusion, Digital Accessibility, and Facilities Management) that ensure accessible technologies, facilities, and learning environments. These resources complement JFK Partners' internal supports by addressing individualized faculty and trainee needs identified through the annual Trainee Needs Assessment (e.g., assistive technologies, accessible materials, technology support, and other accommodations).

The Departments of Pediatrics and Psychiatry Offices of Grants and Contracts and Human Resources provide essential fiscal and administrative oversight. In partnership with LEND leadership, the Pediatrics Office of Grants and Contracts monitors adherence to the approved workplan, allowable expenditures, accurate financial records, and timely reporting to ensure responsible stewardship of federal and other funds. Internally, JFK Partners maintains an Excel-based management system overseen by Training Co-Director Dina Johnson, MA, to track faculty effort, trainee activities, products, and project assignments. These data support program monitoring, evaluation, fiscal oversight, and required LEND reporting.

Physical and Technology Resources

JFK Partners is housed on the fifth floor of the ADA-compliant CU Anschutz Education 2 South Building, which includes trainee workstations, faculty and administrative offices, and conference space sufficient for LEND programming. Courses are delivered in person and, as needed, via Zoom; mentoring occurs in person, by Zoom, or by phone. A glass-enclosed corridor connecting Education 2 North and South provides informal space for interdisciplinary interaction, and trainees have individual Zoom accounts to support collaboration.

The nearby Strauss Health Sciences Library provides extensive electronic journals, books, research databases (e.g., MEDLINE, Web of Science, Cochrane Library), reserve materials, interlibrary loan, instructional services, and meeting and study space. The CU Anschutz Office of Digital Learning supports Canvas, the program's learning management system, while the Office of Information Technology manages Zoom for meetings, instruction and continuing education.

In response to Colorado House Bill 21-1110 (Digital Accessibility Law for Colorado State and Local Government), CU Anschutz implemented a digital accessibility compliance program requiring accessible digital content, communication tools, information systems, and reporting platforms. JFK Partners provides accessibility training for faculty and incoming trainees, while individuals with disabilities, faculty, trainees, and community partners guide continuous quality improvement of accessibility across LEND activities.

Clinical practicum occurs in hospital, clinic, community, and telehealth settings (See Attachment 3 and also Table 2). Children's Hospital Colorado (CHCO) and Network of Care sites provide multidisciplinary clinical space, including examination and observation rooms with audio/video capability, evaluation and motor rooms, family conference rooms, team rooms, and work areas. Faculty and trainees also provide services to adults with A/DD through the UCHealth Behavioral Neurology Clinic in Denver.

Partner Organizations

To fulfill program goals and meet our training objectives, JFK Partners collaborates with campus and external programs. LEND faculty members teach in CU SOM residency and

fellowship programs in Psychiatry, Child Psychiatry, Rehabilitation Medicine, and Family Medicine, as well as the Colorado School of Public Health (LEND PH Discipline Director) and the University of Northern Colorado (2021-26 Rehabilitation Counseling Discipline Director). A LEND faculty member directs the APA-accredited Clinical Psychology predoctoral internship, and the DP Section administers the ACGME-accredited Developmental Behavioral Pediatrics fellowship. We recruit Speech-Language, Social Work, and Psychology trainees from other institutions of higher education regionally. JFK Partners also maintains agreements with organizations that serve IwD&F, state agencies, community organizations, CU Anschutz programs, regional institutions of higher education, and other MCHB-funded programs to support training, continuing education, technical assistance, research, and collaborative goals.

Table 6, Partner Organizations, lists agencies and programs with which we have established agreements or contracts that support LEND program goals (also see Attachment 3: Agreements with other entities). Table 6 identifies the organization (first column) and the program commitments for specific collaboration, including: Trainee Recruitment (abbreviated "TR" below), Training Support (TS), Continuing Education (CE), Technical Assistance (TA), and Research Collaboration (RC).

TABLE 6. Partner Organizations

Organization	TR	TS	CE	TA	RC
Adams Camp		✓			
Assuring Better Child Health & Development (ABCD: Title V MCH contractor focused on early identification of developmental needs and access to behavioral health support)		✓	✓	✓	
Alliance of Colorado (membership association for I/DD policy change and promotion of best practices)		✓	✓	✓	
Autism Society of Colorado (advocacy, resources, support groups/events)	✓	✓	✓	✓	
CO Developmental Disabilities Council (supports I/DD system change for independence, self-determination, inclusion)	✓	✓			✓
CO Department of Health Care Policy & Financing	✓	✓	✓	✓	
CO Division of Vocational Rehabilitation (IwD employment resource)				✓	
Colorado K-12 Public Schools (Denver Public Schools, Cherry Creek, D11-Colo Springs School District, Littleton Public Schools)			✓	✓	✓
Colorado K-12 Public Schools (Aurora Public Schools) PT Residency	✓	✓			
CU Boulder Speech, Language, and Hearing Services	✓	✓	✓	✓	
CU SOM Dept. of Pediatrics & Children's Hospital Colorado		✓		✓	
CU SOM Dept. of Psychiatry	✓	✓	✓	✓	
CU SOM, APTA-accredited Physical Therapy Residency	✓	✓			
CO School of Public Health, MCH program	✓		✓		

Organization	TR	TS	CE	TA	RC
CO Title V, MCH		✓	✓	✓	
Disability Justice Colorado (IwD & aging legal representation, investigation, education, & legislative analysis)		✓	✓	✓	
Family Consortium of Colorado members (PEAK Parent Center, El Grupo Vida, Parent to Parent CO, & Family Voices CO)	✓	✓	✓	✓	✓
JFK Partners Advisory Council (IwD&F, state & community agencies)	✓			✓	
Marion Downs Center (clinical sites)		✓	✓	✓	
Pac West Regional LEND Consortium Agreement		✓	✓	✓	✓
Rise School of Denver	✓	✓	✓	✓	
School-Based Health Center Program (including Title V-funded centers)	✓	✓	✓	✓	
Talking Together, Inc. (clinical agency)		✓			
The Arc of Colorado		✓	✓	✓	
UC Health Behavioral Neurology Adult I/DD Clinic		✓			
University of Denver – Social Work/Psychology	✓				

The nature of commitments and number of partners may increase over time as community needs and relationships evolve and new partners are cultivated. The table above does not include DP research programs in which our trainees may participate nor professional associations, journals, etc., with which individual faculty have invited roles (e.g., editors, reviewers, advisors), as letters were not sought, although these relationships also serve to advance LEND goals (e.g., dissemination of evidence-based/-informed practices).

LEND Project Director and Faculty

Among JFK Partners faculty members listed below are 11 Discipline Directors fully qualified by knowledge and experience to meet LEND responsibilities. All have served in their roles for at least a year (10 for five or more years). LEND support is requested for the Director, Training Co-Directors, Discipline Directors (all of whom have expertise in A/DD and their individual disciplines), and five other faculty and staff needed to deliver the program The FTEs (full time equivalent) below refer to LEND grant support for the role. Most faculty are CU SOM employees (Departments of Pediatrics, Psychiatry, Rehabilitation Medicine, and Family Medicine), although our Public Health Discipline Director is now employed by the Colorado School of Public Health, compensated by LEND funds for her LEND FTE. The LEND administrative structure includes the positions with administrative responsibilities: the Director, Co-Directors of Training, JFK Discipline Directors, Evaluator, and Training Coordinator. (See uploaded Research & Related Senior/Key Person Profile biographical sketches for key staff and Attachment 2. Staffing Plan and Job Descriptions).

Project Leadership

Project Director, Psychology Discipline Director: Judy Reaven, PhD, (.30 FTE with LEND), is the CU SOM Professor of Psychiatry and Pediatrics, Director of JFK Partners, and

Psychology Discipline Director. She is responsible for overall, day-to-day direction of LEND grant activities, including budget oversight and supervision of all faculty and staff for their work on the project. She ensures proper use of grant funds, attends LEND director and related meetings, works directly with our federal project officer, and is the steward of all funds under the LEND program. She lectures in core courses, mentors LEND trainees and fellows, and child psychiatry residents, and directs the life span JFK Partners/Department of Pediatrics interdisciplinary diagnostic Autism and Developmental Disabilities Clinic. Dr. Reaven joined the CU SOM faculty in 2002, and was appointed the JFK Director on October 1, 2025, after ten years as Associate Director. During her career, she has mentored over 100 trainees in clinical and scholarly work. She is a nationally and internationally invited speaker and trainer on the evidence-based intervention model, *Facing Your Fears,* developed with significant private foundation and federal research funding. Dr. Reaven published two *Facing Your Fears* manuals, along with numerous journal articles, abstracts and book chapters.

Training Co-Director; Education/Special Education Discipline Director: Dina Johnson, MA, (.40 FTE) has degrees in early childhood development (BS) and eLearning Development and Implementation (MA). She has been with JFK Partners for more than 25 years in administrative and faculty roles. As Training Co-Director, she provides leadership in recruitment and curriculum design, teaches didactic courses *(Leadership 1 & 2* and *Key Concepts 1 & 2*) and mentors Education/Special Education and other LTTs. Ms. Johnson supports the Director in managing daily program operations and coordination with the Department of Pediatrics Office of Grants and Contracts. She oversees the internal data management system and the import of data for NIRS reporting. She collaborates with the Evaluator in federal reporting. She is an invited member of the AUCD Neurodevelopmental Disabilities Core Curriculum Editorial Board and represents JFK Partners on the Colorado Developmental Disabilities Council.

Training Co-Director; Speech Language Pathology (SLP) Discipline Director: Renee Charlifue-Smith, MA-CCC-SLP (.10 FTE), Assistant Professor of Pediatrics, has been a clinician, educator, and researcher in A/DD for over 35 years. Her areas of specialization are assessment, routines-based intervention, teaming, autism, and positive behavior support. She co-chairs the AUCD Early Childhood Special Interest Group and leads the JFK Partners and Developmental Pediatrics professional development series. She is the ENRICH Early Intervention Team Director and coordinates the JFK Partners *Interventions* course.

Lived Experience Discipline Directors

Discipline Directors support the Project Director in program planning, implementation, coordination, and evaluation, supervise components related to their discipline, including recruitment criteria, identification and selection of candidates, mentoring, and coordination of the discipline with related programs.

Family: Kristen Kaiser, MA, MPH, (.10 FTE), an Adjoint Instructor of Pediatrics, is a JFK Partners LEND Family graduate and parent of young adult triplets with A/DD. She mentors Family LTTs, oversees other LTTs in family-mentored field experiences, teaches family perspectives and lived experience in the Key Concepts 1 & 2 courses, and assists other families as guest lecturers. Ms. Kaiser is a co-investigator and Family Lead for a HRSA-funded demonstration project focused on transitions to adulthood and family navigation of systems of care. She consults with Autism Elevated, an agency that supports young autistic adults and their families exploring job/career planning. She is the past Executive Director of The Joshua School, a non-profit agency providing specialized education and therapy to promote independence in students with A/DD.

Individuals with Disabilities (IwD): Robin Ennis, DSW, LCSW, CPC, (.10 FTE), Adjoint Instructor of Pediatrics and a JFK Partners LEND graduate who lost all vision in early adolescence. She prepares and supports IwD trainees for courses and practica, and mentors their Scholarly Projects. She lectures in classes, including *Leadership Dialogues 1* on lived experience in MCH. Dr. Ennis is AUCD Co-Chair of SAIL (Self-Advocates In LEND) and serves on the LEND Self-Advocacy Discipline Network, as well as several State of CO technology accessibility committees, as an expert user of screen readers.

Additional Discipline Directors

Advanced Practice Providers-Nursing: Ronda Franke DNP, CPNP-PC, PMHNP-BC, a CU SOM Sr. Instructor and Advanced Practice Provider Mentor with 30 years' experience in clinical practice development serving children, adolescents, and young adults with A/DD. At CHCO, she provides evaluation, treatment, education, and support, working on diagnostic and transition-to-adulthood teams, manages medications, and consults with families. Dr. Franke also specializes in toilet training for children with disabilities, providing care and professional education. She is a member of the CHCO Fragile X research team and a sub-investigator for clinical trials for targeted treatments. She will mentor NP and PA mid-level practitioner trainees.

Developmental Behavioral Pediatrics: Maria Novoa Uriarte, MD, (.10 FTE), Visiting Associate Professor, is a board certified DBP. Previously, she was LEND pediatric discipline director at Stony Brook University and Division Chief of DBP/Medical Director of the Early Childhood Clinic, Stony Brook Children' Hospital, Long Island, NY. Dr. Novoa Uriarte specializes in early detection and evaluation of A/DD, especially among children of bilingual Spanish speaking families, co-founding a bilingual clinic for the assessment of children and teens with A/DD.

Nutrition: Liliane Diab, MD (0.05 FTE), Assistant Professor of Pediatrics, Section of Nutrition, is board-certified in Pediatrics and completed fellowship training in Pediatric Clinical Nutrition. She is Medical Director of the CHCO Growth and Medical Nutrition Clinic and specializes in evaluation and management of pediatric nutrition disorders. Dr. Diab will recruit an mentor Nutrition trainees.

Occupational Therapy (OT): To be announced: Position vacant as of July 1, 2026 and to be filled if funded.

Physical Therapy (PT): Meghan TB Hernandez, PT, DPT, EdD (.10 FTE), a LEND graduate, is Assistant Professor in the Departments of Physical Medicine, Rehabilitation, and Pediatrics and directs the CU Pediatric Physical Therapy Residency Program. She mentors LTTs in PT and interdisciplinary trainees, and lectures in LEND courses.

Public Health: Kristina Hightshoe, MSPH, a Sr. Research Scientist at the Colorado School of Public Health (CSPH), and experienced researcher/coordinator for many large scale, collaborative CU and CSPH public health epidemiology studies. She coordinates the of the CDC-funded Study to Explore Early Development (SEED), Colorado site, and coordinated the SAMHSA-funded SUCCEED: Supporting Colorado Children's Early Emotional Development (a mental health awareness training). She mentors LEND Public Health trainees and is co-instructor of the *Leadership 1* course.

Social Work: Sarah McSwegin, LCSW (.10 FTE), a MA LEND graduate, is a Senior Instructor of Pediatrics. She provides follow-up and support to children and families with new developmental diagnoses throughout evaluation and intervention at CHCO. She offers service coordination, crisis management, support groups, and is a lead in the DP Transition Clinic. Ms. McSwegin is also the DP Social Worker at the CHCO Outpatient Care in Colorado Springs. She

is a member of the CHCO workgroup for streamlining CHCO autism services and lead faculty for the DP Parent Advisory Committee.

Other Supported Roles

Evaluator: Gerri Steinke, PhD (.20 FTE), Adjoint Senior Instructor in Pediatrics, is psychologist who specializes in program development, evaluation, and reporting. She evaluated numerous HRSA-funded programs (e.g., Bureau of Health Professions; State Autism Implementation, LEND, and UCEDD projects; and other state and federal grants, contracts, and cooperative agreements. She also led a national study tracking obesity prevention regulations and quality improvement in childcare, funded sequentially by MCHB and CDC. She is responsible for JFK Partners federal reporting in collaboration with Dr. Reaven and Ms. Johnson, program evaluation, and quality improvement.

Research Manager: Valentina Postorino, PhD (.10 FTE), Clinical Instructor of Pediatrics, mentors trainees in clinical, research and professional communication skills (e.g., writing for publication, IRB support). She is an experienced interdisciplinary clinician, teacher, and DD researcher, and an internationally invited trainer for the evidenced-based RUBI PT model in autism.

Training Coordinator: Christine Cook (.40 FTE), mother of an autistic young adult and experienced administrative professional. She onboards new trainees, manages trainee recruitment, trainee and graduate evaluations, and supports distance education/CE delivery.

Screening & Assessment Co-Instructor: Audrey Blakeley-Smith, PhD (.05 FTE), JFK Partner LEND graduate, is a Professor of Psychiatry and Pediatrics. She is the training director of the CU SOM Family Medicine APA-accredited psychology internship. She co-authored the *Facing Your Fears* (FYF) treatment manuals and was PI of a randomized clinical trial to adapt FYF for children who have autism and intellectual disabilities. Dr. Blakely-Smith teaches the *Screening and Assessment* course (with Dr. Reynolds, below), mentors LEND psychology interns and supervises interdisciplinary trainees in the JFK Partners Autism and Developmental Disabilities Clinic.

Screening & Assessment Co-Instructor: Ann Reynolds, MD (.025 FTE), Professor of Pediatrics and DP Section Head, co-teaches the *Screening and Assessment* course. Her research interests are sleep and nutritional issues in children, especially with autism and Fragile X syndrome. Dr. Reynolds is a past member of the Autism Speaks Scientific Review Committee, the Autism Treatment Network Scientific Review Committee, and the Autism Intervention Research Network for Physical Health Steering Committee.

If funded, we will seek continued MCHB approval of a 13th discipline, Rehabilitation Counseling, re-appointing current Discipline Director (Brian Dean, MA, CRC) a Certified Rehabilitation Counselor who specializes in support of individuals with A/DD who face significant barriers to employment. CU Anschutz faculty across most potential LEND disciplines may be engaged to support training. Recently, faculty in Dentistry, Genetic Counseling, and Family Medicine mentored LEND trainees and may be re-invited as new candidates emerge. Audiology mentors are also available as we pursue refunding of the LEND Audiology Supplement.

Faculty with ≥ 0.5 FTEs have annual performance reviews within their primary schools/ colleges and departments. All faculty can access campus-wide and departmental training opportunities. Deans provide professional development funds for meetings and workshops, and DP Section offerings are available to all JFK Partners faculty, staff, and trainees. Vacancies are filled by best-qualified applicants committed to better care of individuals with A/DD.

Past Experience

JFK Partners, an original University Affiliated Facility and LEND program founded in 1968, has consistently provided interdisciplinary training, continuing education (CE), research and dissemination, technical assistance (TA), and community collaboration to improve the lives of individuals with A/DD and their families (IwD&F). We implemented approved workplans with responsible programmatic and fiscal stewardship, meeting all reporting requirements. Supported by an accomplished faculty and staff (as described above), LEND faculty, trainees, and graduates collaborated with national and community partners (e.g., NDD Curriculum Editorial Board, Pacwest Regional LEND Consortium, CDDC, Assuring Better Child Development Colorado, and state and local family and ID support organizations).

Faculty annually delivered an updated, accessible curriculum of didactics, clinical practica, and leadership experiences addressing MCH Leadership Competencies, core A/DD knowledge, policy, research, and evidence-based screening, assessment, intervention, and supports. We annually trained LTTs from 10–13 disciplines recruited across CU Anschutz, partner institutions (Attachment 3), and match programs, including trainees with lived experience in A/DD. During the 2021–26 cycle, we trained nearly 90 LTTs, plus 11 LEND Audiology Supplement LTTs, and met or exceeded commitments for MTTs, STTs, continuing education, technical assistance, and dissemination products, as documented in DGIS.

Clinical trainees developed evidence-based competencies in A/DD screening, diagnosis, intervention, and person- and family-centered care, enriched by the perspectives of Family and IwD trainees. In turn, Family and IwD trainees strengthened their leadership skills and ability to contribute lived expertise in service to others. Program evaluation consistently confirmed that graduates are prepared to support individuals with A/DD and their families across the lifespan. Exit evaluations showed that 97% of respondents reported increased A/DD knowledge and skills, 94% were satisfied with the training received, and 99% of respondents to the NIRS LTT graduate survey would recommend the program. As trainees and graduate reflected: “Before this fellowship I had a surface understanding of autism. I now know how to explain autism to families, evaluate for autism, recognize autistic traits, and better understand the experiences of people living with autism.” (Exit Evaluation, 2025); “Because of the lived experience trainees, guest speakers, and family mentorship, I heard perspectives I may never have encountered in the provider-patient relationship.” (Exit Evaluation, 2023); “LEND shaped my career trajectory and, perhaps even more importantly, the way I move through this world. I am a more valuable member of clinical teams because of my LEND experiences.” (Five-year graduate, NIRS LTT Survey, 2025)

A notable example of the enduring legacy of MCHB LEND funding and JFK Partners' systems-level impact is the dissemination and implementation of *Facing Your Fears* (FYF), an evidence-based therapy for autistic individuals with co-occurring anxiety. Through adoption, adaptation, and evaluation in clinics, training programs, behavioral health systems, schools, and international settings, FYF has expanded access to evidence-based care while creating training and research opportunities for LEND faculty and trainees. This work continues through a PCORI-funded comparative effectiveness study of FYF and *Zones of Regulation* to inform individualized program selection for autistic students with anxiety, reflecting the growing evidence base and JFK Partners' commitment to advancing gold-standard science.

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Attachment 1: Work Plan

The workplan below lists JFK Partners three LEND program goals, with objectives and planned activities to be carried out by the assigned JFK staff through June 2031.

Abbreviations & symbol used below	
CDT = Co-director(s) of Training **DP** = Development Pediatrics Section **DD** = Discipline Directors **E** = Evaluator **ITP** = Individual Training Plan **IwD&F** = Individuals with autism/developmental disabilities & families	**L/M/STTs** = Long-, Medium-, Short-Term Trainees **OEM** = Outcome, Evidence, Measure **PD** = Project Director **RM** = Research Manager **TC** = Training Coordinator **Symbol:** ≥ greater than or equal to, at least

JFK PARTNERS 2026-31 GOALS, OBJECTIVES, AND ACTIVITIES						
GOAL A. Increase access to evidence-based screening, diagnosis, interventions, and supports for individuals with A/DD and their families (IwD&F) by preparing interdisciplinary, person- and family-centered leaders through competency-based training and community partnerships.						
Objective (Obj) A1. Recruit and educate LEND trainees in a competency- & evidence-based LEND curriculum (didactics, clinical, research, & community experiences) with active partner engagement to increase evidence-based A/DD screening, evaluation, diagnosis, & interventions for children who (may) have A/DD. *Outcomes*: At least (≥) 18 LTTs in IwD, Family & 8 other disciplines, 10 MTTs, & 100 STTs trained; ≥ 75% of LTTs and sampled MTTs & STTs acquire new knowledge/skills & are satisfied with training; ≥ 6 active IwD&F or community partners. *Timeframe*: Yrs. 1-5, summer, fall, spring						
Activity/OEM (**O**utcome, **E**vidence or **M**easure)	y1	y2	y3	y4	y5	**Staff**
A1a. Update & implement recruitment plan & selection procedures/**OEM:** Dissemination records; Training Review Committee minutes; Offer letters	✓	✓	✓	✓	✓	PD, CDTs, TC, DD, Faculty
A1b. Train an interdisciplinary LTT cohort in the comprehensive LEND curriculum/**OEM:** 18 LTTs in IwD, Family, ≥ 8 other disciplines	✓	✓	✓	✓	✓	PD, CDTs, DD, Faculty, TC

A1c. Train MTTs in selected LEND components/**OEM:** ≥ 10 MTTs	✓	✓	✓	✓	✓	PD, CDTs, DD, Faculty, Clinical Supervisors
A1d. Train STTs in selected LEND components/**OEM:** ≥ 100 STTs trained	✓	✓	✓	✓	✓	PD, CDTs, DD, Faculty, Clinical Supervisors
A1e. Update & offer a comprehensive didactic LEND curriculum with partners/**OEM:** 6 didactic courses, 1 research seminar, Disability Dialogues delivered	✓	✓	✓	✓	✓	PD, CDTs, Faculty
A1f. Deliver faculty & trainee clinical services/practica to increase evidence-based A/DD screening, evaluation, diagnosis, & interventions for children who (may) have A/DD/**OEM:** No. trainees prepared in evidence-based A/DD practices; No. diagnostic evaluations conducted	✓	✓	✓	✓	✓	PD, CDT, Faculty, Clinical supervisors, Trainees
A1g. Deliver research & non-clinical practical experiences that address trainee goals & learning needs in research and scholarship/**OEM:** ≥75% positive supervisor ratings	✓	✓	✓	✓	✓	PD, CDT, DD, Clinical supervisors
A1h. Administer curriculum QI measures/focus group/evaluations/**OEM:** positive QI progress metrics; ≥ 75% positive course & program evaluations; ≥ 90% LTT cohort increased competencies (ratings/reflections)	✓	✓	✓	✓	✓	E, TC
A1i. Review syllabi for community engagement in didactic courses/**OEM:** ≥ 6 community partners/yr.	✓	✓	✓	✓	✓	E
Obj. A2. Obtain alumni assessments of LEND training value and impact. *Outcomes*: ≥ 75% respond to survey; ≥ 80% recommend LEND; ≥ 70% use skills; 60% report lasting impact *Timeframe:* Yrs. 1- 5, spring						
Activity/OEM (Outcome, Evidence or Measure)	y1	y2	y3	y4	y5	**Staff**
A2a. Administer 2, 5 & 10 NIRS graduate LTT survey (prompts as needed)/**OEM**: ≥ 80% recommend LEND; ≥ 75% use skills; ≥ 60% report lasting impact	✓	✓	✓	✓	✓	TC, CDT, E
A2b. Explore strategies for alumni networking to encourage survey response/**OEM:** ≥ 75% respond to survey		✓	✓	✓		PD, CDT, TC, E

GOAL B. Strengthen the MCH workforce and systems of care, in partnership with individuals with A/DD and their families (IwD&F) and community partners, to improve access to competent, responsive, and evidence-based services and supports.						
Obj. B1. Conduct evidence-based CE with IwD&F and agency partners to increase workforce knowledge and skills. *Outcome:* ≥ 5 CE events (3 for credit); ≥ 200 participants across urban and rural areas; ≥ 75% of participants are satisfied with content; ≥ 75% report increased/refreshed knowledge/skills; ≥ 50% intend to apply recommended practices applicable to role. *Timeframe:* Yrs. 1-5						
Activity/OEM (**O**utcome, **E**vidence or **M**easure)	y1	y2	y3	y4	y5	**Staff**
B1a. Conduct partner outreach to define CE agenda/**OEM:** Annual CE agenda & partner list	✓	✓	✓	✓	✓	PD, CTDs, DD
B1b. Host CEs with IwD&F and agency partners/**OEM:** ≥ 5 CEs (3 for credit); ≥ 200 participants; No. active partners	✓	✓	✓	✓	✓	CTD, TC, DD, Faculty
B1c. Administer CE participant evaluations/**OEM:** 75% of participants rate CE positively, 60% report new/refreshed knowledge, 50% intend to apply to practice	✓	✓	✓	✓	✓	TC, E
Obj. B2. Promote strong partnerships among IwD&F, state, local, and community disability interests to increase evidence-based systems responsive to community priorities. *Outcomes:* ≥ 6 partner programs; ≥ 75% of sampled TA projects report progress in goals & satisfaction with TA services. *Timeframe:* Yrs. 1-5						
Activity/OEM (**O**utcome, **E**vidence or **M**easure)	y1	y2	y3	y4	y5	**Staff**
B2a. Conduct outreach to state agencies, Title V programs, schools, other community/ national partners to identify collaborative projects & partners/**OEM:** Nos. agencies, partners, topics identified	✓	✓	✓	✓	✓	PD, CDT, Faculty
B2b. Deliver collaborative TA/consultation with IwD&F and partner agencies to infuse evidence-based practices in systems & services/**OEM:** ≥ 6 active partner programs; ≥ 75% sampled TA projects report progress towards targeted goals	✓	✓	✓	✓	✓	PD, CDT, Faculty
B2c. Administer TA/consultation evaluations/**OEM**: ≥ 75% sampled TA recipients satisfied	✓	✓	✓	✓	✓	PD, CDT, Faculty, E
Obj. B3. Share evidence-based A/DD information & resources, in accessible & plain language formats, including a research newsletter, annual report, & other products to inform IwD&F and related audiences and maximize LEND impact. *Outcomes*: 1 research newsletter, no. disseminated; annual report, no. disseminated; ≥ 15 project products/Yr.; accessibility for lay audiences "substantially achieved". *Timeframe:* Yrs. 1-5, newsletter (spring), assorted project products (as available), annual report (Yrs. 2–5)						

Activity/OEM (**O**utcome, **E**vidence or **M**easure)	y1	y2	y3	y4	y5	**Staff**
B3a. Create consumer summaries of LEND project accomplishments/**OEM:** 1 annual fall research newsletter, nos. disseminated; (Yrs. 2-5)1 annual fall report, nos. disseminated	✓	✓	✓	✓	✓	CDT, Faculty, Trainees, RM, E
B3b. Disseminate faculty & trainee projects/resources/**OEM:** ≥ 15 presentations, posters, assorted products created & published or posted	✓	✓	✓	✓	✓	TDC, Faculty, Trainees
B3c**.** Archive selected CEs for on-demand delivery/**OEM**: ≥ 3/yr. archived on JFK Partners website	✓	✓	✓	✓	✓	CDT
B3d. Acquire JFK Partners CAC ratings of accessibility of sampled products/**OEM**: ≥ 75% of ratings = "substantially achieved" or higher	✓	✓	✓	✓	✓	CTD, E
GOAL C. Demonstrate responsible stewardship of LEND resources through accountable grant management to maximize LEND impact for IwD&F.						
Obj. C1. Manage data collection for accurate performance reporting. *Outcome:* 100% of data reporting requirements met. *Timeframe:* Yrs. 1-5, by defined MCHB deadlines						
Activity/OEM (**O**utcome, **E**vidence or **M**easure)	y1	y2	y3	y4	y5	**Staff**
C1a. Administer internal data collection system/**OEM:** Bi-annual fall & winter faculty & trainee data collection	✓	✓	✓	✓	✓	CTD, E
C1b. Manage data input for reporting/**OEM:** timely NIRS, DGIS Performance Measures, CARES Evaluation, non-competing LEND continuation, & other reports as required	✓	✓	✓	✓	✓	CTD, E, PD
Obj. C2. Conduct and report comprehensive program evaluation and continuous quality improvement (CQI) findings to assess and strengthen progress towards LEND goals. *Outcomes:* At least 80% of overall program measures are positive in all program performance domains; no. of timely data/report submissions: ≥ 1 CQI change package tested. *Timeframe:* Ongoing, Yrs. 1–5						
Activity/OEM (**O**utcome, **E**vidence or **M**easure)	y1	y2	y3	y4	y5	**Staff**
C2a. Analyze evaluation data (recruitment, training program, NIRS LTT survey, CE, TA, JFK Partners CAC feedback)/**OEM:** Up-to-date Excel database for program assessment & recommendations	✓	✓	✓	✓	✓	E
C2b. Summarize program evaluation findings/**OEM**: JFK Partners CAC (May) & Training Program Retreat (June) PowerPoints; Narrative synopsis (July)	✓	✓	✓	✓	✓	E

C2c. Report to and seek JFK Partners CAC community comprehensive program performance evaluation each May/**OEM**: PowerPoint (see C2b), CAC evaluation findings (July)	✓	✓	✓	✓	✓	PD, E
C2d. Collaborate in LEND Program Quality Improvement (LPQI) data submission/**OEM:** LPQI-defined trainee & faculty data points (Yr. 2 or later, when applicable)		✓	✓	✓	✓	TC, Faculty, Trainees
C2e. Implement CQI change package and data analysis/**OEM:** TBD using internal CQI quantitative & qualitative data, & LPQI data as available	✓	✓	✓	✓	✓	PD, CDT, E, Faculty
C2f. Update leadership on evaluation & CQI findings and recommendations/**OEM:** Verbal/written report in Training Review Committee meetings/email documentation	✓	✓	✓	✓	✓	E
C2g. Conduct data-driven program review in JFK Partners Training Program Retreat (June)/**OEM:** Agenda; Annual academic plan; Revised syllabi, recruitment/mentoring strategies as needed; new or ongoing CQI targets	✓	✓	✓	✓	✓	PD, Training Co-Directors, E
Obj. C3. Manage LEND resources responsibly through fiscal oversight and data-informed resource allocation to support achievement of program objectives. *Outcomes:* 100% required fiscal reports submitted accurately and on time; 0 material audit or compliance findings. *Timeframe:* Yrs 1 – 5.						
Activity/OEM (**O**utcome, **E**vidence or **M**easure)	y1	y2	y3	y4	y5	**Staff**
C3a. Monitor budgets & expenditures for fiscal oversight quarterly in collaboration with the Department of Pediatrics Office of Grants & Contracts (OGC)/**OEM**: 100% of submitted LEND budgets, expenditures & modifications achieve OGC approval	✓	✓	✓	✓	✓	PD, CDT
C3b. Apply fiscal data (and evaluation findings in C2) to planning & allocation of program resources/**OEM:** ≥ 95% of funds expended in accordance with approved program objectives & budget allocations	✓	✓	✓	✓	✓	PD, CDT

Attachment 2 – Staffing Plan and Job Descriptions

JFK Partners maintains an interdisciplinary faculty team with expertise in maternal and child health, autism and developmental disabilities, leadership education, research, evaluation, clinical care, systems change, technical assistance, and family- and person-centered practice. Faculty and staff collaboratively plan, implement, and contribute to evaluation and continuous quality improvement in all aspects of the LEND program to meet HRSA program requirements.

PROJECT DIRECTOR

Administrative Direction:
The Project Director reports to the Chair of the Department of Pediatrics, University of Colorado School of Medicine, and receives administrative guidance from the University of Colorado Office of Grants and Contracts for grant administration. The Project Director also works closely with the HRSA Project Officer regarding implementation of all programmatic requirements.

Duties and Responsibilities:
The Project Director provides overall leadership, administration, and management of the JFK Partners Leadership Education in Neurodevelopmental and Other Related Disabilities (LEND) Program. The Project Director is responsible for:

a. Providing day-to-day leadership and oversight of all LEND training, continuing education, technical assistance, research, evaluation, and community engagement activities.
b. Directing implementation of graduate, postgraduate, continuing education, and interdisciplinary leadership training consistent with HRSA LEND program requirements.
c. Supervising faculty and staff and ensuring coordination of interdisciplinary curricula, clinical training, practicum experiences, mentoring, and leadership development.
d. Overseeing project budget, fiscal stewardship, grant compliance, and federal reporting.
e. Leading strategic planning, program evaluation, continuous quality improvement, performance measurement, and implementation of data-informed program improvements.
f. Ensuring accurate and timely reporting of HRSA performance measures, DGIS reporting requirements, and participation in Autism CARES evaluation activities.
g. Overseeing recruitment and retention of interdisciplinary faculty and trainees, including family members and individuals with developmental disabilities in faculty and trainee roles.
h. Ensuring that all program activities promote person- and family-centered, culturally and linguistically appropriate, and accessible services.
i. Developing & maintaining partnerships with Title V MCH and state agencies, community organizations, academic programs, clinical partners, & Autism CARES-funded programs.
j. Representing JFK Partners within the AUCD network, HRSA LEND meetings, and other regional and national initiatives.
k. Supporting dissemination of scholarship, publications, presentations, technical assistance products, and other activities that advance systems change and improve care for children and youth with autism and developmental disabilities.

Minimum Qualifications:
A doctoral or medical degree is required. The Project Director shall have at least ten years of experience in maternal and child health and at least five years of experience in interdisciplinary leadership education at the graduate level. Demonstrated expertise in autism and developmental disabilities, curriculum development, mentoring, systems collaboration, leadership development, quality improvement, program evaluation, and administration of federally funded programs is required. Experience in budget management, HRSA reporting, partnership development, and person- and family-centered systems of care is preferred.

JFK PARTNERS LEND FACULTY

Administrative Direction:
LEND Faculty report to the Project Director and collaborate in implementation of assigned activities.

Duties and Responsibilities:
LEND faculty share responsibility for planning, implementing, coordinating, mentoring, and contributing to the evaluation of all aspects of the interdisciplinary LEND Program. Faculty responsibilities include:

a. Planning, implementing, coordinating, and supervising all discipline-specific training and service activities with emphasis on autism and developmental disabilities.
b. Collaborating across disciplines to develop, refine, deliver, and evaluate interdisciplinary curricula that incorporate MCH Leadership Competencies and evidence-based practice.
c. Coordinating accessibility accommodations and ensuring training activities are accessible to all participants.
d. Recruiting, selecting, mentoring, supervising, and evaluating trainees.
e. Providing interdisciplinary clinical, research, community, and leadership mentoring.
f. Identifying, coordinating, and supervising clinical, community-based, and systems-level practicum experiences.
g. Collaborating with faculty across disciplines to ensure person- and family-centered, culturally and linguistically appropriate, and accessible educational experiences.
h. Serving as liaison with professional organizations, academic departments, community agencies, Title V MCH programs, and Autism CARES-funded partners.
i. Participating in program evaluation, performance measurement, continuous quality improvement, and implementation of program improvements.
j. Conducting scholarship related to maternal and child health, autism, developmental disabilities, systems improvement, and interdisciplinary practice.
k. Participating in AUCD and national LEND workgroups, committees, and collaborative initiatives.
l. Developing manuscripts, presentations, continuing education activities, technical assistance products, and other dissemination materials.
m. Supporting recruitment and retention of trainees and faculty representing diverse disciplines and lived experience, including individuals with disabilities and family members.
n. Providing technical assistance and consultation to community organizations, state agencies, healthcare systems, and other partners to improve access to evidence-based services.

Minimum Qualifications:
Training Co-Directors and Discipline Directors shall possess a minimum of five years of professional experience following completion of their terminal degree. Faculty shall demonstrate expertise in autism/developmental disabilities, interdisciplinary education, mentoring, leadership development, and person- and family-centered care. Experience with health systems, community partnerships, technical assistance, and scholarship is preferred. A master's degree is required for applicable disciplines; doctoral preparation is preferred. Family and Self-Advocacy Discipline Directors shall possess lived experience consistent with their discipline. Faculty serving exclusively as clinical mentors shall have a minimum of two years of professional experience following completion of their terminal degree.

PROJECT EVALUATOR

Administrative Direction:
The Project Evaluator reports to the Project Director.
Duties and Responsibilities:
The Project Evaluator is responsible for planning, implementing, and overseeing program evaluation, quality improvement, and performance measurement systems. Responsibilities include:

a. Developing and implementing the comprehensive evaluation plan.
b. Designing and maintaining systems for performance measurement, outcome tracking, and data collection.
c. Conducting formative, process, outcome, and impact evaluations.
d. Supporting HRSA DGIS reporting requirements and Autism CARES evaluation activities.
e. Monitoring trainee outcomes, including longitudinal tracking of graduates.
f. Collaborating with faculty and leadership to implement continuous quality improvement activities.
g. Preparing evaluation reports and assisting with dissemination of findings through presentations and publications.
h. Supporting data-informed decision-making across all project activities.

Minimum Qualifications:
Doctoral degree required. Minimum of five years of experience in program evaluation, implementation science, quality improvement, performance measurement, mixed-methods research, and data analysis. Experience with federally funded training programs and HRSA reporting is preferred.

TRAINING COORDINATOR

Administrative Direction:
The Training Coordinator reports to the Training Co-Director and works collaboratively with the Project Director, Evaluator, and faculty.
Duties and Responsibilities:
The Training Coordinator provides administrative and operational support for implementation of the LEND Program by:

a. Coordinating trainee recruitment, application, onboarding, orientation, and tracking.
b. Maintaining trainee records, databases, documentation, and program schedules.
c. Coordinating faculty meetings, advisory committee meetings, continuing education activities, and technical assistance events.
d. Assisting with collection of performance measures and documentation required for HRSA reporting.
e. Supporting communication among faculty, trainees, community partners, and university programs.
f. Providing administrative support for program evaluation, recruitment activities, and dissemination events.

Minimum Qualifications:
Bachelor's degree preferred. Minimum of two years of experience coordinating interdisciplinary education or training programs. Experience with higher education administration, project coordination, event planning, database management, and communication with multiple stakeholders is preferred.

Attachment 3: Letters of Partnership Agreement

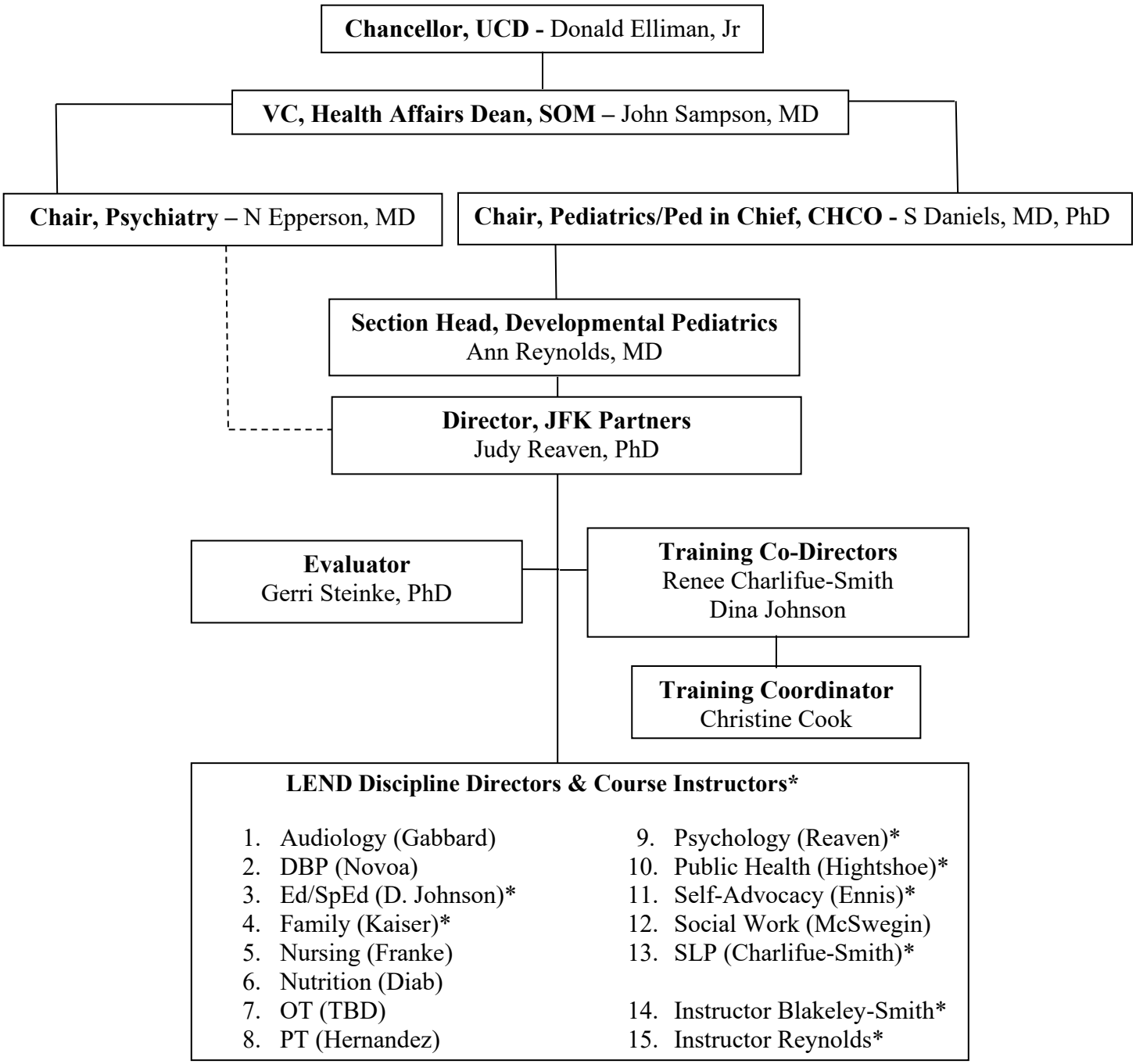
Organization	Partnership Commitment Letter
Adams Camp – Early Start Therapy Camp is a half-day intensive therapy program for children 6 months to 4 years old.	Early Start Therapy Camp provides a practicum for OT/PT trainees providing intensive, multidisciplinary therapies for children with A/DD.
Assuring Better Child Health & Development (ABCD-Colorado) is a locally known and nationally recognized leader in championing progressive early childhood policies and equitable access to high quality, comprehensive early childhood developmental services.	ABCD Colorado helps identify emerging needs, provides expert speakers for classes and webinars, and collaborates on screening projects of mutual interest that support young children and their families.
Alliance of Colorado is a nonprofit, statewide association comprised of Intellectual and Developmental Disability (IDD) Provider Agencies, Case Management Agencies, Early Intervention Brokers, businesses serving the IDD community, and disability advocacy organizations.	Alliance of Colorado helps identify emerging priorities, provide expert speakers for classes and webinars, and collaborate on systems and workforce projects of mutual interest that support children and their families.
Autism Society of Colorado (ASC) is a statewide nonprofit organization dedicated to improving the lives of autistic individuals and their families through advocacy, education, information and referral, community connections, and support across the lifespan.	ASC helps with recruitment of self-advocacy and family trainees, panelists for courses and webinars, share training opportunities, and collaborate on projects of mutual interest to support autistic individuals and their families.
Colorado Developmental Disabilities Council (CDDC) is the federally authorized, governor-appointed state council that promotes systems change, capacity building, and advocacy to improve the independence, productivity, inclusion, and self-determination of people with intellectual and developmental disabilities and their families.	CDDC helps with recruitment of self-advocacy and family trainees, panelists for courses and webinars, share training opportunities, serve on the JFK Partners Advisory Council, and collaborate on projects of mutual interest to support individuals with I/DD and their families.
Colorado Department of Health Care Policy & Financing (HCPF) is the agency responsible for administering Colorado's Medicaid program and driving health care policy.	HCPF partners to identify emerging policy and systemic priorities, sharing resources to support trainee recruitment and professional development, and engaging in collaborative projects of mutual interest.
Colorado Division of Vocational Rehabilitation (DVR) is the state agency dedicated to assisting individuals with disabilities prepare for, obtain, and maintain meaningful employment, including competitive integrated employment.	DVR is committed to collaborating with JFK Partners, via the Colorado Office of Employment First (COEF), to support and enhance Individual Placement and Support (IPS) fidelity reviews across the state.

Organization	Partnership Commitment Letter
Colorado K-12 public schools (Denver, Cherry Creek, D11 Colorado Springs, Littleton Public Schools)	Colorado K-12 Schools partner to apprise of needs they encounter to address in CE, share research results and implications for their students, scale interventions in districts through training, TA to schools for problem-solving.
Colorado K-12 public schools (Aurora Public Schools)	Aurora Public Schools partner to provide a PT Pediatric Resident school practicum The PT Resident participates on a team with related service providers who support a student's ability to access their educational environment. As specialists in movement, they provide services for students' physical participation in a variety of settings throughout the school day, in the primary role as a school PT, to help students benefit from their educational program within the educational environment.
CU Boulder Speech, Language, and Hearing Services (CU-B, SLHS) offers educational programs for the preparation of professional clinical practitioners in Speech Language Pathology and Audiology.	CU-B, SLHS partners to prepare professional clinical practitioners with high quality clinical learning experiences, and includes trainee recruitment, continuing education opportunities sharing, and collaborating on projects of mutual interest.
CU SOM, Dept of Peds & Children's Hospital Colorado (CHCO) The Dept of Peds clinical experiences take place at CHCO and is the region's only pediatric academic medical center. The Dept of Pediatrics prepares interdisciplinary clinical leaders.	CU SOM, Dept of Peds & CHCO commit to providing JFK Partners trainees with opportunities to observe and participate in CHCO, DevPeds and JFK Partners clinical practices under the supervision of faculty from their respective disciplines.
CU SOM, Dept of Psychiatry provides nationally recognized expertise in child and adolescent mental health, psychology, research, and interdisciplinary education.	CU SOM, Dept of Psych partners to recruit psychology trainees and fellows, support JFK Partners psychology faculty, share the announcements of JFK Partners community education events within the department
CU SOM, APTA-accredited Physical Therapy Residency prepares PTs for advanced pediatric clinical practice through interdisciplinary education, evidence-based care, leadership development, and family-centered services.	CU SOM, APTA-accredited PT Residency partners to provide clinical mentorship and interdisciplinary training for future leaders in pediatric rehabilitation.
Colorado Title V MCH Program, within the Colorado Department of Public Health & Environment (CDPHE), leads statewide efforts to improve the health and well-being	Title V MCH partners to educate JFK Partners trainees as lecturers & host trainee MCH staff interviews; partner in webinars on CSHCN & A/DD population, systems, & emerging issues

Organization	Partnership Commitment Letter
of mothers, infants, children, adolescents, and children and youth with special health care needs through public health leadership, data-driven planning, and systems development.	to educate the MCH workforce, including rural & frontier care coordinators & nurses.
CO School of Public Health (CSPH) MCH Program prepares future public health leaders through interdisciplinary education, research, and community partnerships focused on improving the health and well-being of women, children, youth, families, and communities.	CSPH-MCH partners to support trainee recruitment and share continuing education opportunities.
Disability Justice Colorado (DJC) is Colorado's federally designated Protection and Advocacy (P&A) organization, providing legal advocacy, education, and systems change efforts to protect the rights, promote self-determination, and advance the inclusion of people with disabilities across the state.	DJC partners to identify emerging priorities, trainee recruitment, provide expert speakers for courses and continuing education, serve on JFK Community Advisory Council, review selected products for alignment with their mission, and collaborate on projects of mutual interest.
Family Consortium of Colorado (FCC) is a statewide family-led organization that empowers families of children/youth with SHCN and disabilities through leadership development, peer support, education, and advocacy to improve services, systems, and outcomes across Colorado.	FCC partners to help recruit family members, siblings, and self-advocates as interdisciplinary trainees; shares continuing education opportunities; serves on the JFK Partners Advisory Council; and provides lived-experience perspectives that strengthen JFK Partners activities.
JFK Partners Advisory Council (JFK-CAC) provides strategic guidance and diverse perspectives to support program planning, evaluation, and continuous quality improvement, ensuring that activities remain responsive to the needs of individuals with developmental disabilities, families, communities, and state partners.	JFK-CAC partners to identify priorities, support trainee recruitment, provide community feedback on annual performance, share lived experiences, share continuing education opportunities, promote meaningful engagement with a variety of partners, and ensure that goals remain responsive to the needs of individuals with developmental disabilities and their families
Marion Downs Center (MDC) is a nonprofit organization that provides comprehensive hearing health services, early intervention, education, and community programs for children and adults who are Deaf or Hard of Hearing, partnering to promote communication access and improve outcomes across the lifespan.	MDC partnership provides a community clinic training site for AuD and SLP trainees, identify community needs, provide expert speakers for classes and webinars, and collaborate on system and workforce projects. Trainees may also experience developmental hearing, vision, and speech/language screenings at the KidScreen program for children aged birth to 18 on-site in schools, preschools, and Head Start programs in metropolitan Denver.

Organization	Partnership Commitment Letter
Pacwest Regional LEND Consortium is a collaborative network of HRSA-funded LEND programs that advances interdisciplinary leadership education, faculty development, scholarly collaboration, and regional systems improvement to enhance services and outcomes for children/youth with A/DD, and their families in the western U.S.	The Pacwest Regional LEND Consortium agrees to advance a common agenda of leadership, training, service, and research; disseminate research in the field; improve mentorship opportunities; conference/meeting presentations/posters reflective collaborative work.
Rise School of Denver is an inclusive early childhood education program providing high-quality preschool education for children with and without developmental disabilities, fostering language, social-emotional development, and school readiness through evidence-based, inclusive practices and family partnerships.	Rise School of Denver partners to host clinical practicum, including staff and family consultations, delivered by DBP and PT trainees in an inclusive, therapeutic preschool for children, ages 1-5 years, with autism, and genetic, seizure, and other disorders.
School-Based Health Center (SBHC) Program operates in 58 school sites across Colorado, providing medical, mental health, dental, and behavioral health care for students.	SBHC partners to receive specialized training to more effectively support neurodivergent students to include screening, referral, co-occurring mental health conditions, and resources for teachers and parents.
Talking Together, Inc. (TTI) provides early intervention services to support young children's communication, development, and participation in inclusive environments.	TTI partners in providing rotation for OT trainees seeking clinical experience in early intervention using Transdisciplinary Primary Provider Model, including Parent Coaching.
The Arc of Colorado is a nonprofit advocacy organization for people with I/DD, providing advocacy, education, leadership development, and systems change to promote inclusion, self-determination, and equitable access to community services and supports.	The Arc-CO partners to engage trainees in policy learning, identify emerging priorities, provide legislative updates, support trainee recruitment, identify speakers for classes/webinars, & collaborate on system & workforce projects.
UC Health Behavioral Neurology Adult I/DD Clinic provides a consultative model for adults with intellectual disabilities and A/DD emphasizing care coordination	This clinical partnership provides opportunities to observe patient evaluations, participation in interdisciplinary case discussions, and coordinated care for adults with complex neurodevelopmental conditions.
University of Denver, Social Work/ Psychology (DU) prepare future behavioral health professionals through evidence-based education, clinical training, research, and community partnerships, contributing expertise in mental health, family systems, trauma-informed care, and interdisciplinary services for children, youth, and families.	DU partners to prepare clinical practitioners with high quality clinical learning experiences, including trainee recruitment, continuing education opportunities sharing, and collaborating on projects of mutual interest.

JFK Partners LEND Organizational Chart



Attachment 5: Tables, charts, and curriculum/syllabus

CONTENTS

Didactic Course Descriptions 1
Clinical Site Descriptions 5
Schedule for Training Program Implementation 8

Didactic Course Descriptions

Most JFK Partners didactic courses are taught by interdisciplinary teams, and all promote interdisciplinary teamwork and interaction. The table below is an overview of content by course. Abridged course descriptions follow, including Instructors, Methods, Outcome competencies, Sample readings/assignments, and a summary of Key content Area.

Table A. Didactic Coverage of LEND Content at a Glance
(Italicized Didactic column headers are course title abbreviations included in descriptions below.)

Key content areas	Course					
	LD1/2	*KC1/2*	*S&A*	*INT*	*IRS*	*DD*
DD: Autism/developmental disabilities	✓	✓	✓	✓	✓	✓
PH: Public health perspectives & systems	✓	✓		✓	✓	✓
CL: Cultural/linguistic responsiveness	✓	✓	✓	✓	✓	✓
CH: Community Health Factors	✓	✓	✓	✓	✓	✓
EQ: Evidence-based/Quality improvement	✓		✓	✓	✓	
PF: Person-/family-centered care	✓	✓	✓	✓	✓	✓
CE: Current/emerging areas	✓	✓		✓	✓	
TL: Transition/lifespan issues		✓		✓		✓
Res: Research	✓	✓	✓	✓	✓	
Tech: Technology/Telehealth		✓	✓	✓	✓	

***CLSC 6661/6662 Leadership Dialogues (LD*) 1 & 2** (Summer, 2 cr./Spring, 1 cr.).
Instructors: *LD1* - Johnson D., Hightshoe; *LD2* - Johnson D.
LD1 & 2 cultivate Maternal and Child Health Leadership Competencies (version 5.0). *LD1* emphasizes Self-Reflection, Communication, Interdisciplinary/Interprofessional Team Building, Negotiation and Conflict Resolution, Community Health Factors, and Lived Experience in MCH. Sessions address application of leadership skills to consultation and teamwork, both including people with disabilities and families, to facilitate change in the contexts of early childhood, education, health care, mental health, public health, and other systems. *LD2* clarifies leadership practices in policy to support individuals with disabilities and their families and addresses the application of evidence to policy processes. Focal leadership competencies are Policy, Systems Thinking, MCH Knowledge Base/Context, Critical Thinking, Communication, Community Health Factors, Lived Experience in MCH, and Teaching, Coaching, and Mentoring. *Methods*: (*LD1*) Lectures, self-assessment, and highly interactive group processing; (*LD2*) Expert guest lectures, interactive work groups, observation of a legislative session and meetings with community agencies, using the Colorado example. *Outcome competencies:* Emerging leadership skills such as: Understanding and use of strategies for managing team dynamics (inclusive of

professional and lived experiences) (in *LD1*); Understanding of lived experience and professional roles and community collaboration in legislative policy processes; Skills for evidence-based education of policy makers (in *LD2*).
Sample readings:
- Autistic Self-Advocacy Network (ASAN). (ND). Identity-First Language. https://autisticadvocacy.org/about-asan/identity-first-language/
- Heen, S., Stone D. Find the coaching in criticism. *Harvard Business Review.* 2014 Apr; 92(4):18-19. https://hbr.org/2014/01/find-the-coaching-in-criticism
- Colorado Legislative Council Staff Memorandum – Public Participation in the Legislative Process – January 17, 2017 (in CANVAS)

Assignments: Completion of the Thomas Kilmann Conflict Styles Instrument (Thomas K, 2001, 2007. CCP) & Leadership Practices Inventory, Self, 5th ed. (Kouzes JM, Posner BZ, 2017. Wiley), development of personal leadership profiles, interviews with a leader; development of a - policy brief, letter to the editor, or testimony.
Key content areas: **DD, PH, CL, CF, EQ, PF, EA, Res**

CLSC 6653/6654 Key Concepts in Developmental Disabilities (*KC*) 1 & 2 (Fall/Spring, 2 cr. ea.). *Instructors*: Reaven, Charlifue-Smith, Johnson D., Kaiser, Price-Johnson, Ennis
KC 1 & 2 examine central concepts in supporting individuals with A/DD and their families following a lifespan trajectory from public health policy and person-/family-centered perspectives. *KC1* provides: overviews of progress in evidence-based practice in the field of A/DD, healthcare access, federal and state Title V early childhood policies and systems (e.g., EPSDT, EHDI, DD waivers), medical home, accessing behavioral therapy, child abuse (prevention, treatment, and reporting), vicarious trauma and compassion fatigue, discipline roles, and scholarly project instruction (e.g., IRB). KC2 examines federal policies (Olmstead Act, IDEA, DD Act and Bill of Rights, Section 504 of the Rehabilitation Act, HCBS waivers), and state disability partners (Colorado Disability Law and Developmental Disabilities Council), ethics and genetics in disability, dual diagnosis in children and adults, accessible and assistive technologies, dental care, sexual health, emergency preparedness, healthcare and other adult transitions for youth with A/DD (guardianship/supported decision-making), adult services and employment. *Methods*: System expert/family/self-advocate presentations and panel discussions, trainee individual and team presentations/discussions, family/self-advocate mentored home visit. *Outcome competencies*: Understanding of interdisciplinary roles and contributions in A/DD condition, major federal legislation history/impact, and CO service systems and resources; Abilities to integrate community health factors, self-advocate, and family input into program planning, delivery, and evaluation; ability to conceptualize and implement a scholarly project.
Sample readings/resources:
- Colorado MCH Priorities/Measures/Strategies/Strategy Leads 2026-30: MCH Priorities/ Measures/Strategies/Strategy Leads FY2026-2023. https://docs.google.com/document/d/1NrIVo7VoNcZWyv7p-lksR3J1PDXTeeQIkym5lE0TDVc/edit?tab=t.0
- Association of University Centers on Disabilities. Fabric not fringe: Weaving family involvement throughout training and practice for professionals and advocates working with individuals with disabilities and special health care needs. https://systems.aucd.org/docs/publications/2019_family_fabric.pdf

- Berg H. Is evidence-based practice justified? A philosophical critique. *J Eval Clin Pract*. 2024;30(5):855-859. doi:10.1111/jep.14059
  Flannery KA, Wisner-Carlson R. Autism and education. *Child Adolesc Psychiatr Clin N Am*. 2020;29(2):319-343. doi:10.1016/j.chc.2019.12.005

Assignments: Discipline presentations; Colorado Title V/MCH interviews, Federal Legislation summaries, Family-Mentored interview; Scholarly Projects (i.e., research, leadership, quality improvement, scholarly, or community/systems change project).
Key content areas: **DD, PH, CL, CF, PF, EA, TL, Res, Tech**

CLSC 6668 Screening and Assessment for Children/Youth with A/DD (**S&A**) (Fall, 3cr.)
Instructors: Reynolds, Blakely-Smith
CLSC 6668 addresses evidence-based practices in screening and assessment for children who may have A/DD. Topics include: Signs and symptoms of autism in young childhood through adulthood, screening tools (e.g., M-CHAT-R, STAT), differential diagnosis (e.g., Autism Diagnostic Observation Schedule), IQ testing in A/DD, co-morbidities, tools for detecting symptoms, professionals and family collaboration and feedback, cultural considerations (e.g., bilingual assessment), behavioral phenotype variations, testing controversies, therapeutic listening and coaching, developmental evaluations, strategies for "hard to test kids," parent interviews, adult evaluations, non-speaking/minimally verbal profiles, telehealth approaches to care coordination, and testing approaches for differential diagnosis. *Methods:* Lectures, discussions, and case presentations. *Outcome competencies*: Abilities to describe: Heterogeneity in autism and characteristics of autism across the lifespan, screening tools, developmental evaluations, standardized testing challenges/benefits, co-occurring medical and mental health conditions, cultural aspects of valid assessments, understanding of the family perspective on assessment, and a working knowledge of an interdisciplinary evaluation. (Practicum, for additional credits, applies clinical skills.)
Sample readings:

- Lord, C, Rutter, M, DiLavore, PC, Risi, S, Gotham, K, & Bishop, LS. (2012). *Autism Diagnostic Observation Schedule, Second Edition (ADOS-2) Manual (Part I): Modules 1-4.* Torrance, CA: Western Psychological Services.
- Bal VH, Wilkinson E, White LC, et al. The Adapted ADOS: a new module set for the assessment of minimally verbal adolescents and adults. *J Autism Dev Disord.* 2020;50(3):719-729. doi:10.1007/s10803-019-04300-3.

Assignments: Presentation on a screening or assessment topic (e.g., an assessment case, evaluation of toddlers, tips and tricks for a successful evaluation, frequently encountered challenges, working with parents) & associated reading list
Key content areas: **DD, CL, CF, EQ, PF, Res, Tech**

CLSC 6663 Intervention for Children/Youth with Autism/DD (INT) (Spring, 3 cr.).
Instructor: Charlifue-Smith
INT reviews evidence-based practice interventions for children and youth with A/DD, human research ethics, skills to appraise literature (e.g., commonly used statistics) and to select and apply evidence-based principles. A wide range of interventions are examined, e.g., family therapy; social interventions for autistic individuals; interventions for executive function difficulties, challenging behaviors, sleep disorders; assistive technology; daily living skills; complementary and alternative treatments; transition support; emerging issues/public health

threats (e.g., neonatal abstinence syndrome). Content delivery is informed by best practice reviews (e.g., National Standards Project, the Academy of Sciences Educating Children with Autism report, recommendations of professional organizations). *Methods:* Lectures, readings, case presentations and discussions. *Outcome competencies*: Understanding of evidence-based practices in A/DD interventions and abilities to a) describe common areas of difficulty in A/DD and interdisciplinary best practices; b) critically review current models of A/DD therapies and complementary and alternative treatments, and apply published evidence in their discussion.
Sample readings:

- Carnett, A., McLay, L., Hansen, S. *et al.* Sleep Problems in Children and Adolescents with Autism: Type, Severity and Impact. *J Dev Phys Disabil* **33**, 977–991 (2021). https://doi.org/10.1007/s10882-020-09783-5
- About Fetal Alcohol Spectrum Disorders. CDC. https://www.cdc.gov/fasd/about/index.html

Assignments: Development of a Family Tip Sheet on a specific skill or activity; Complementary and alternative intervention for A/DD model & evidence review.
Key content areas: **DD, PH, CL, CH, EQ, PF, EA, TL, Res**, **Tech**

Interdisciplinary Research Seminar (**IRS**) (monthly, September – June; not for cr.).
Instructors: Boles (DP faculty), Postorino (JFK faculty), Losh (DP faculty).
IRS addresses topics to inform research and scholarly project development, design, implementation, and engagement of self-advocates, families, and underserved populations in all phases. Sessions are flexibly designed to meet the needs of current trainees. A typical sequence includes: IRB for newbies/institutional regulatory information; creating a quality improvement project; conducting a systematic review; meaningful community partnerships (e.g., authorship, research team members roles); creating useful surveys; faculty panel on success, failures, & rejections; authorship and conducting research responsibly; manuscript writing & other dissemination methods; manuscript critiques and peer review; and, wrap-up and debrief.
Outcome competencies: Understanding of steps and responsibilities entailed in
Assignments: A/DD journal articles; presentation of current projects
Key content areas: **DD, PH, CL, CH, EQ, PF, EA, Res, Tech**

Disability Dialogues (DD) (8 monthly sessions, not for cr.) *Instructor:* Price-Johnson
Lectures are offered for AMC students. A/DD topics vary, as selected by faculty and 3-5 short-term trainee team. Sample topics include: physical exams for patients with disabilities, communicating with patients who use assistive technology, and understanding experiences with individuals who are Deaf/Hard of Hearing. Lectures are presented principally by individuals with A/DD and their family members, with ample time for Q & A. *Outcome competencies*: Improved confidence in communicating with a person with A/DD. Awareness of one or more A/DD conditions and of varying care needs of neurodivergent people that will require additional or specialized knowledge and skills.
Sample readings/assignments: Not applicable. Suggested reading may be offered.
Key content areas: **DD, PH, CL, CH, EQ** (importance of EBP), **PF, TL**

Accommodations & Atmosphere: Syllabi instruct trainees to contact Training Co-Directors D. Johnson or Charlifue-Smith for support of accessibility or accommodation needs. Syllabi also establish commitments to a respectful learning community (value different perspectives, active listening, thoughtful and respectful communication, constructive dialogue, and accountability).

Clinical Site Descriptions

Table B. Clinical Site Descriptions, Services, and Locations					
I = interdisciplinary **Dx** = evaluation, diagnosis & recommendations **T** = therapy/intervention					
Title	*Description*	*I*	*Dx*	*T*	*Location(s)*
DIAGNOSTIC EVALUATIONS					
JFK Partners Interdisciplinary Evaluation Clinic	Serves ages 12 months to 18 yrs.; use evidence-based tools and expert interdisciplinary diagnosis and differentiation of A/DD conditions.	✓	✓		CHCO
Adult Evaluation Clinic	Serves adults seeking A/DD/ID evaluation , and/or co-occurring mental health condition, or needing updated testing for other services	✓	✓		CHCO
Infant – Toddler Clinic	Serves youngest children	✓	✓		CHCO/North Campus/CO Springs
Birth to 3 Clinic	Serves youngest children	✓	✓		CHCO
Preschool Clinic	Serves children ages 3 - 5 yrs	✓	✓		CHCO/North Campus/ Wheatridge/CO Springs
School Age Clinic	Serves children ages 6 - 10yrs.	✓	✓		CHCO/CO Springs
Pre-teen/Adolescent Clinic	Serves youth ages 12- 26 yrs.	✓	✓		CHCO/CO Springs
Developmental Team	Serves Child Health Clinic referrals for DBP evaluation	✓	✓		CHCO
BLOSSOM High Risk Neonatal Follow Up Clinic	Serves high-risk newborns	✓	✓	✓	CHCO/CO Springs
ACES Clinic	Serves children ages 1-7 yrs in foster care & trauma exposed	✓	✓		CHCO
Deaf/Hard of Hearing Clinic	Serves children with hearing loss	✓	✓		CHCO
Bilingual Clinic	Serves children & families needing Spanish/English care	✓	✓		CHCO/SE Aurora
One Day Clinic	Serves out-of- state patients	✓	✓		CHCO
Rural Outreach Sites	Serves communities with little/no access A/DD specialists	✓	✓		Durango/Grand Junction/Alamosa/ Glenwood Springs

Title	*Description*	*I*	*Dx*	*T*	*Location(s)*
Neurogenetics Clinics					
10 Neurogenetics clinics in, e.g., 22Q, Fragile X, Prader Willi Clinic	Serves children with genetic and rare disorders	✓	✓	✓	CHCO
Single Discipline					
MD/NP Single Discipline & Fellows' Clinics	For discipline specific skills and services		✓		CHCO/North Campus/ Wheatridge/SE Aurora/CO Springs
Child Health Clinic DBP evaluation	For discipline specific skills and services		✓		CHCO Aurora
Psychology Single Discipline	For discipline specific skills and services		✓		CHCO/NO, SO, & CO Springs Campuses
THERAPIES/INTERVENTIONS					
Next Steps Clinic	Social work support for new diagnosis		✓		CHCO/Telehealth
ABC's of Behavior Parent Group	Offers strategies for difficult behaviors/positive behaviors			✓	CHCO/Telehealth
CHCO Outpatient Clinic	Comprehensive care A/DD, learning disability & behavior		✓	✓	CHCO/Highlands Ranch
Early Start Denver Model Clinic	A/DD play-based support of development in A/DD			✓	CHCO/North Campus/CO Springs
ENRICH	Early intervention in natural environments	✓		✓	Metro Denver natural environments
Behavioral Therapy Clinic/RUBI	Individual parent-coaching to shape positive behaviors			✓	CHCO/Telehealth
Behavior Therapy Group/RUBI	Group parent-coaching to shape positive behaviors			✓	CHCO/Telehealth
Cognitive-Behavior Therapy Clinic	Serves ages 7-17 yrs.) in coping strategies for anxiety, depression			✓	CHCO/CO Springs/Telehealth
Facing Your Fears	Group family CBT for autism/anxiety in ages 8-14 yrs	✓		✓	CHCO/CO Springs/Telehealth
Facing Your Fears: IDD	Serves ages 12-18 yrs with CBT for autism + ID & anxiety	✓		✓	CHCO
Behavioral Sleep Clinic	Evaluation, medication, desensitization &sleep hygiene	✓	✓	✓	CHCO/Telehealth

Title	*Description*	*I*	*Dx*	*T*	*Location(s)*
Pediatric Oral Feeding Clinic	Interdisciplinary care in feeding/eating difficulties	✓	✓	✓	CHCO/Telehealth
Toileting Clinic	Nursing and psychology assessment, medical/behavioral strategies in toileting	✓	✓	✓	CHCO/Telehealth
Birds and Bees Clinic	Parent coaching for A/DD sex education			✓	Telehealth
HOPE Cancer Survivorship Program	Management of long-term side effects in pediatric cancer & bone marrow transplantation	✓	✓	✓	CHCO
Transition Clinic	A/DD, ID transition planning with vocational options	✓	✓		CHCO/CO Springs/Telehealth
Community Partner Sites					
UC Health Behavioral Neurology Adult I/DD Clinic	ID & A/DD adult consultations emphasizing care coordination		✓	✓	Central Park Medical Center Denver
PT School Practicum	School-based PT practicum	✓	✓	✓	Aurora Public Schools
Adam's Camp	OT & PT practicum in therapies for children & youth with ID/A/DD	✓		✓	Metro Denver
Rise School of Denver	DBP & PT practicum in inclusive therapeutic preschool	✓		✓	Denver
Marion Downs Center	SLP practicum in communication disorders		✓	✓	Greenwood Village
KidScreen for Hearing, Speech & Vision	School- & Head Start-based evaluations & treatment	✓	✓	✓	Denver metro area schools
Talking Together	OT practicum in Early Intervention private practice	✓		✓	Family homes (Front Range)
CO Public School Districts	Consultation & research practica plus other opportunities	✓		✓	Denver, Cherry Creek, Littleton, Colorado Springs

Practica: *See the JFK Partners Project Narrative* for descriptions of **additional practical experiences** in research, policy/advocacy, and community agencies, and required and optional **workshops** (e.g., *Facing Your Fears Intervention*) and **conferences,** and **required online modules,** including CITI research and University HIPAA and Data Security modules.

Schedule for Training Program Implementation

Table C. Schedule for Training Program Implementation	Full time	Part time
Summer semester		
Orientation Session	✓	✓
Leadership Dialogues 1 (2 cr.) (4 hr. daily/2 wk., summer intensive)	✓	✓
Interdisciplinary Research Seminar (Th. PM)	*	*
Practicum (hrs./wk.)	24-30	8-16
Fall semester (late Aug.-Dec.)		
Screening & Assessment in Developmental Disabilities (3 cr.) (Tu. PM)	✓	*
Key Concepts in Neurodevelopmental Disabilities 1 (2 cr.) (Tu. AM)	✓	✓
Interdisciplinary Research Seminar (Th. PM)	*	*
Practicum (hrs./wk.)	24-30	8-16
ITP Review/Revision (individually scheduled)	✓	✓
Spring Semester (Jan-May)		
Interventions in Developmental Disabilities (3 credits) (Tu.)	✓	*
Key Concepts in Neurodevelopmental Disabilities 2 (2 credits) (Tu. AM)	✓	✓
Interdisciplinary Research Seminar (Th. PM)	*	*
Leadership Dialogues 2 (1 cr., spring), 6 sessions (Tu. midday)	✓	✓
Practicum (hrs./wk.)	24-30	8-16
Focus group/Competency reassessment/reflections/ Exit evaluation/Annual Scholarly Project Symposium/LEND Graduation	✓	✓

** Participation varies with discipline and ITP goals.*

Attachment 6: Funding Tier Calculation

LEND Funding Year	Number of long-term trainees reported in HRSA performance reports excluding AuD trainees funded on the supplement
July 1, 2021-June 30, 2022	**17**
July 1, 2022-June 30, 2023	**17**
July 1, 2023-June 30, 2024	**17**
July 1, 2024-June 30, 2025	**18**

Average number of LTTs for the first four years of this funding cycle = **17.25**
Tier Funding Level = Tier 3

Justification for Tier 3 Funding
JFK Partners respectfully requests continued Tier 3 funding to support 18 long-term trainees annually. During the previous project period, we admitted an average of 18.25 long-term trainees per year, demonstrating our capacity to recruit a Tier 3 cohort. However, our four-year average of trainees completing the required 300 hours was 17.25, falling two trainees short of the Tier 3 threshold minimum of 18. This shortfall resulted from a combination of persistent COVID-19-related disruptions that affected recruitment during the first year of the project period and unforeseen withdrawals after admission of two trainees, one due to a personal crisis and the second due to a serious illness. The loss of the ill trainee complicated the reportable number of trainees trained, since we were no longer able to report on an additional second-year trainee who did complete training. Lastly, one committed trainee withdrew at the start of the training year due to an unexpected employment-related emergency. While we experienced unforeseen challenges in retention, we have demonstrated both the commitment and the ability to recruit at the requested Tier level. Our plan for improving retention and responding to unanticipated challenges is described below.

Plan to achieve Recruitment and Retention Requirements
JFK Partners has implemented a comprehensive recruitment and retention strategy designed to consistently achieve the Tier 3 trainee target. Recruitment will begin earlier and include expanded outreach across participating disciplines and community partners, with strategic over-recruitment of qualified applicants, when needed, to account for potential attrition. At our most recent retreat (June 23, 2026), we targeted strategies to strengthen trainee support and retention and refine our incoming Trainee Needs Assessment to identify accommodation needs earlier. Universal Design for Learning principles, intentional strategies for cohort bonding, faculty and peer mentoring, and structured alumni engagement will further support trainee success and positive training experiences. While retention remains our highest priority, recruitment targets will be increased in Years 2–5, if necessary, to ensure we meet the Tier 3 commitment of 18 long-term trainees annually.

Attachment 7: LEND Pediatric Audiology Supplement

BACKGROUND 1
PROBLEM 2
GOALS AND OBJECTIVES. 2
METHODOLOGY. 3
RECRUITMENT 6
COLLABORATION 7
EVALUATION 7
INNOVATION 8
Budget Narrative: July 1, 2026– June 30, 2031 10

Frequently Used Acronyms (*The following acronyms are used to conserve space.*)

A/DD	Autism and Developmental Disabilities
AuD	Audiology
CHCO	Children's Hospital Colorado
CQI	Continuous Quality Improvement
DevPeds	Section of Developmental Pediatrics and CHCO clinical unit
DHH	Deaf/Hard of Hearing
EHDI	Early Hearing Detection & Intervention
ITP	Individual Training Plan
IwD&F	Individuals with Autism/Developmental Disabilities & their Families
L-/M-/STT	Long-, Medium-, or Short-term Trainees
UCEDD	University Center for Excellence in Developmental Disabilities

BACKGROUND

This application for the Pediatric Audiology (AuD) Supplement to the Leadership Education in Neurodevelopmental and Related Disabilities (LEND) Program is submitted by the University of Colorado (CU) School of Medicine, JFK Partners, in response to HRSA-26-019. JFK Partners, within the CU Anschutz Department of Pediatrics, has served as Colorado's University Center for Excellence in Developmental Disabilities (UCEDD) for more than 60 years, the Colorado LEND Program since 1995, and the LEND Pediatric AuD program since 2010.

Children who are deaf or hard of hearing (DHH) with or at increased likelihood of autism and other developmental disabilities (A/DD) require coordinated, interdisciplinary care because developmental disabilities are common among children receiving hearing health care, and pediatric audiologists frequently evaluate these children before a developmental disability has been formally identified.[1,3] Pediatric audiologists are essential members of the Early Hearing Detection and Intervention (EHDI) system, providing hearing screening, diagnostic evaluation,

developmental surveillance, and timely referral to early intervention and other services that support language acquisition and developmental outcomes for children who are DHH.[2,3]

Colorado's EHDI system continues to experience challenges with timely diagnosis, early intervention enrollment, workforce shortages, care coordination, and access to pediatric AuD services, particularly in rural communities. The proposed LEND AuD supplement will prepare pediatric audiologists as clinical and systems leaders who strengthen EHDI partnerships, expand workforce capacity, and improve timely diagnosis, family engagement, language acquisition, and developmental outcomes for children who are DHH who (may) have A/DD.

PROBLEM

The need for graduate-level pediatric AuD trainees and practicing pediatric audiologists is driven by persistent workforce shortages and gaps in systems of care for DHH children who (may) have A/DD. National EHDI data and recent federal review findings continue to identify delays in diagnosis, gaps in timely access to early intervention, and shortages of experienced pediatric audiologists, highlighting the need for advanced competencies to serve children who are DHH with A/DD.[4]

The estimated prevalence of hearing loss among U.S. newborns is 1.7 per 1,000 infants screened.[4] In Colorado, approximately 1.2% of newborns were identified with confirmed hearing loss in 2024.[5] Although Colorado exceeded the newborn hearing screening benchmark (96.4%), only 42.2% of infants who did not pass screening received a diagnostic evaluation by 3 months and only 32.6% of infants with hearing loss were enrolled in early intervention by 6 months.[6] Loss to Follow-up/Loss to Documentation (LFU/LTD) remains a significant systems challenge, with rates of 1% for newborn hearing screening, 58.4% for diagnostic evaluation, and 23.4% for early intervention enrollment.[7] These findings demonstrate the need for pediatric AuD leaders who can strengthen EHDI systems, improve care coordination, and increase timely diagnosis and intervention.

The need for advanced pediatric AuD training is reinforced by the high prevalence of co-occurring developmental disabilities. Approximately 40% of DHH children have a co-occurring disability,[8] and autism prevalence among children who are DHH is estimated at 6%–9%.[1,9] Recent electronic health record studies demonstrate that pediatric audiologists frequently evaluate children before a developmental disability has been formally identified, placing them in a critical position to recognize developmental concerns and facilitate timely referral for developmental evaluation, and early intervention.[2,10]

Although developmental monitoring is within the pediatric AuD scope of practice, a Colorado survey found that more than half of audiologists reported discomfort with autism screening and believed it fell outside their scope of practice.[11] Families likewise report fragmented systems of care that complicate coordination and navigation across hearing health, developmental, educational, and early intervention services.[12] These challenges underscore the need for advanced training, leadership, and stronger EHDI partnerships.

GOALS AND OBJECTIVES

Faculty Leadership. The supplement project will be led by Sandra Gabbard, PhD (Project Director), Deborah Mood, PhD (developmentally informed pediatric AuD and interdisciplinary clinical training), Tammy Fredrickson, PhD (EHDI systems and workforce development), Dina Johnson, MA (curriculum and evaluation), Lauren Pontis, PhD (CU Boulder liaison and trainee

mentorship), under the overall leadership of Judy Reaven, PhD, Principal Investigator. Collectively, faculty oversee trainee recruitment, curriculum, clinical supervision, mentoring, continuing education, technical assistance, evaluation, and dissemination.

Goals & Objectives. JFK Partners AuD LEND has two program goals with objectives.

GOAL A. Increase access to evidence-based screening, diagnosis, interventions, and supports for individuals who are DHH with increased likelihood of A/DD and their families (IwD&F) by preparing Audiologists through competency-based training and community partnerships.

Objective (Obj.) A1. Recruit and train pediatric AuD trainees through competency-based didactic, clinical, leadership, research, and community experiences that increase knowledge and skills in evidence-based A/DD screening, diagnosis, intervention, and follow-up for children who are DHH. *Outcomes:* At least (≥) 2 long-term trainees (LTTs) in AuD, 1 medium-term trainee (MTT), & 4 short-term trainees (STTs) trained annually; ≥ 75% of LTTs and sampled MTTs & STTs acquire new knowledge/skills & are satisfied with training; ≥ 2 active IwD&F or community partners. *Timeframe:* Yrs. 1-5, summer, fall, spring

Obj. A2. Obtain assessments of LEND training value and impact. Outcomes: ≥80% recommend LEND; ≥70% used knowledge/skills; 60% report lasting impact. *Timeframe:* Yrs. 1-5, spring.

GOAL B. Strengthen Colorado's workforce and EHDI systems capacity, in partnership with IwD&F and community partners, to expand pediatric AuD capacity and improve access to competent, responsive, evidence-based assessment and follow-up services for DHH infants and young children who (may) have A/DD and their families.

Obj. B1. Deliver evidence-based continuing education (CE) with family and agency partners to strengthen pediatric AuD workforce knowledge and implementation of best practices. *Outcomes*: 2 CE events; ≥ 50 participants across the state; ≥ 75% of participants are satisfied with content; ≥ 75% report increased/refreshed knowledge/skills; ≥ 50% intend to apply recommended practices applicable to role. *Timeframe:* Yrs. 1-5.

Obj. B2. Promote strong partnerships among IwD&F, state, local, and community disability interests to increase coordinated evidence-based systems that respond to community needs. *Outcomes*: ≥ 2 partner programs/year; ≥ 75% of sampled technical assistance (TA) projects report progress in goals & satisfaction with TA services. *Timeframe:* Yrs. 1-5

Obj. B3. Share evidence-based information & resources about D/HH infants and children who (may) have A/DD, in accessible formats, to maximize LEND impact and inform the pediatric AuD workforce. *Outcomes*: ≥ 4 new resources/yr; *Timeframe:* Yrs. 1-5

Excluding the fixed number of grant-supported trainees, most goals and objectives represent conservative minimum targets likely to be exceeded during the five-year project period.

METHODOLOGY.

The JFK Partners AuD LEND program will implement an integrated, competency-based training model that combines interdisciplinary didactics, pediatric clinical experiences, leadership development, scholarly activity, family partnership, and systems collaboration. Training activities are designed to prepare pediatric audiologists to improve developmental and language outcomes for DHH children, including those with co-occurring A/DD, while strengthening Colorado's EHDI system. Activities align with program goals to expand the pediatric AuD workforce and build EHDI systems capacity through evidence-based practice, family-centered care, and leadership.

Curriculum and Training Content. The LEND AuD program will provide competency-based, evidence-based training that complements traditional AuD education by emphasizing pediatric AuD within interdisciplinary, family-centered, and systems-based care. Training activities are aligned with MCH Leadership Competencies and pediatric AuD competencies, with trainee progress assessed through Individualized Training Plans (ITPs), faculty mentoring, reflective activities, and competency-based evaluations. Individuals with lived experience and family partners will serve as educators, mentors, advisors, and partners in program planning and evaluation throughout the program.

The curriculum addresses audiologic screening, diagnostic assessment, amplification fitting, and developmental monitoring for children who are DHH with co-occurring A/DD. Training includes the Joint Committee on Infant Hearing (JCIH) 1-3-6 EHDI benchmarks; hearing screening and surveillance through age 3; family-centered medical home principles; family engagement and peer support; communication of accurate, comprehensive, current, evidence-based information to support shared decision-making; and the pediatric audiologist's role in promoting language acquisition and developmental outcomes. Clinical and community experiences address community factors that influence health and access to care, with emphasis on improving services for rural populations and others who experience barriers to timely pediatric AuD and EHDI services.

LEND AuD LTTs will develop competencies in evidence-based pediatric AuD, interdisciplinary practice, family-centered care, leadership, and systems improvement through LEND courses, monthly seminars, clinical practica, mentoring, leadership experiences, and a Scholarly Project. Leadership experiences emphasize quality improvement (QI), policy, advocacy, and systems change within local, state, and national EHDI systems. Guided by faculty mentors, Scholarly Projects strengthen critical thinking and MCH Leadership Competencies through research, QI, policy analysis, community engagement, systems change, or other scholarly activities. Projects may include using EHDI data to improve referral pathways, reduce loss to follow-up, strengthen family engagement, or enhance coordination across systems. MTTs and STTs will participate in selected curriculum components based on their training goals.

LEND and LEND AuD Didactics. LEND AuD LTTs complete four interdisciplinary parent LEND courses: *Leadership Dialogues 1 & 2* (LD 1 & 2; 2 credits and 1 credit) develop MCH Leadership Competencies (V5) and their application across public health and health care systems through lectures, readings, self-assessments, assignments, and structured group activities. *Key Concepts in Neurodevelopmental Disabilities 1 & 2* (KC1 & KC2; 2 credits each) present interdisciplinary, public health, and person- and family-centered perspectives on A/DD, systems of care, and lifespan issues. Individuals with disabilities and their families (IwD&F), Colorado Title V MCH staff, and community partners serve as guest faculty.

Monthly LEND AuD Presentations will focus on evidence-based pediatric AuD competencies, EHDI systems, developmentally informed practice, language acquisition, family-centered care, and interdisciplinary management of children who are DHH with A/DD. Sessions support trainees' ITPs, Scholarly Projects, leadership development, and collaboration with EHDI partners on QI initiatives.

AuD LEND Clinical Practica provide hands-on experience, including newborn and pediatric hearing screening, diagnostic assessment, amplification management, developmental

surveillance, language acquisition, interdisciplinary care, and family-centered counseling. **DevPeds DHH Clinic (CHCO).** Trainees participate in interdisciplinary evaluation of children with hearing loss and increased likelihood A/DD, developing competencies in developmentally informed hearing assessment, differential diagnosis, family feedback, and care coordination. **Help for Hearing Device Users Clinic.** Trainees learn behavioral and family-centered strategies to improve hearing technology use among children with complex developmental needs while maximizing auditory access and language outcomes. **Marion Downs Center (MDC).** Trainees gain experience in pediatric diagnostic AuD, hearing technology, cochlear implant services, school hearing screening, and aural rehabilitation while monitoring hearing access and communication development. **Rural Pediatric Audiology Partnership Rotation.** LEND AuD LTTs will complete an immersive rural pediatric AuD rotation, participating in hearing screening, diagnostic assessment, amplification management, family counseling, referral coordination, and follow-up planning. Trainees will gain experience identifying and addressing barriers to timely diagnosis and intervention in rural communities, including limited specialty access and reliance on tele-audiology and regional partnerships. LEND AuD faculty, who accompany trainees, will provide technical assistance to host practices on developmentally informed audiologic assessment, care coordination for children who are DHH with A/DD, EHDI referral processes, tele-audiology, and strategies to strengthen local pediatric hearing services. This rotation prepares trainees to improve access to evidence-based pediatric AuD services while supporting rural EHDI capacity. Collectively, practicum experiences expose trainees to EHDI systems of care, including screening, diagnosis, amplification, early intervention, developmental monitoring, and transitions across systems of care.

Professional Development integrates ITPs, mentoring, scholarly dissemination, national LEND AuD workshops, EHDI conference presentations, Grand Rounds, and participation in the Colorado EHDI Alliance and Colorado Infant Hearing Advisory Committee to strengthen leadership, systems thinking, QI, advocacy, and professional networks.

Training Outcomes. Through didactic, clinical, leadership, and community-based experiences, trainees will demonstrate competency in evidence-based pediatric hearing screening, diagnostic assessment, hearing technology management, developmental monitoring, family-centered counseling, interprofessional collaboration, and EHDI systems leadership. Graduates will be prepared to improve language and developmental outcomes for children who are DHH with A/DD and to lead QI and systems-change initiatives that strengthen pediatric AuD services and Colorado's EHDI system.

Strengthening Pediatric Audiology Workforce and EHDI Systems

Continuing Education (CE). LEND AuD will conduct at least two interdisciplinary CE programs annually that disseminate evidence-based pediatric AuD and EHDI practices to audiologists, physicians, early intervention providers, educators, and families. CE needs will be identified collaboratively with CO EHDI partners. Most CEs also will be archived on the JFK Partners website and announced on the website and through listservs and social media to support a more informed pediatric workforce and the public. Sample topics include the presentation of A/DD in children who are DHH, building clinician confidence and competence in developmental screening, and developmentally informed pediatric AuD.

Technical Assistance (TA). Faculty and trainees will provide TA through consultation, QI, and collaborative problem-solving with EHDI partners. Participation in the Colorado EHDI Alliance and Colorado Infant Hearing Advisory Committee (CIHAC) will identify opportunities for TA and trainee Scholarly Projects. Based on strong interest expressed by audiologists participating in the 2026 JFK Partners LEND AuD webinar *How to Use Developmentally Informed Strategies During a Hearing Evaluation*, LEND AuD will establish a statewide Pediatric Audiology Community of Practice to disseminate developmentally informed assessment strategies, evidence-based pediatric AuD practices, case-based learning, EHDI resources, and QI strategies through quarterly case consultation, continuing education, and peer collaboration.

Dissemination. LEND AuD will disseminate developmentally informed pediatric AuD practices, educational resources, clinical tools, QI products, and research findings to MCHB- and Autism CARES-funded programs, EHDI partners, and the broader pediatric AuD community. Products will include evidence-based resources for Colorado stakeholders (e.g., screening and evaluation algorithms for children who are DHH with A/DD) and nationally relevant scholarly products. Dissemination will occur through peer-reviewed publications, professional meetings, AUCD networks, webinars, and the JFK Partners website, where archived presentations, toolkits, tip sheets, and other faculty- and trainee-developed resources will be publicly available.

Collectively, these training, leadership, TA and dissemination activities will strengthen Colorado's pediatric AuD workforce and EHDI system while improving timely identification, family-centered care, language acquisition, and developmental outcomes for children who are DHH with A/DD.

RECRUITMENT

The LEND AuD program will recruit an average of two long-term pediatric AuD trainees annually from fourth-year Doctor of AuD externship applicants interested in pediatric AuD and EHDI. Primary recruitment occurs through established partnerships with CHCO and the MDC, supplemented by outreach through the CU-Boulder, Speech, Language, and Hearing Sciences clinical education network, the American Academy of Audiology HEARCareers platform, professional meetings, and the JFK Partners Pediatric Audiology Portal, which highlights evidence-based resources and scholarly products developed by LEND AuD trainees and faculty.

Since 2010, LEND AuD has trained 40 long-term trainees, with 68% currently working in pediatric settings. National Information Reporting System (NIRS) follow-up surveys at 2, 5, and 10 years indicate sustained program impact, with 94% of LEND AuD alumni recommending LEND training and reporting enhanced competencies in interdisciplinary teamwork, family-centered care, developmental evaluation of children with A/DD, pediatric AuD for children with complex needs, systems of care, and community collaboration.

Pediatric AuD Competencies. The competency-based curriculum prepares trainees to provide evidence-based, family-centered care in EHDI systems of care. Trainees develop competencies in: (1) **Clinical Pediatric AuD**—newborn and early childhood hearing screening, pediatric diagnostic evaluation, amplification fitting and management, hearing technology, and assessment adaptations for children with A/DD; (2) **Developmental Care and Early Intervention**—developmental monitoring, language acquisition, timely referral, and implementation of JCIH 1-3-6 recommendations; (3) **Family-Centered Care**—shared decision-

making, culturally responsive counseling, communication of accurate, comprehensive, evidence-based information, and partnership within the family-centered medical home; (4) **Interprofessional Collaboration and EHDI Systems**—care coordination, QI, and collaboration with health care, early intervention, education, and community partners; and (5) **Leadership and Professional Practice**—evidence-based practice, systems thinking, advocacy, scholarly inquiry, and continuous QI.[13,14]

To strengthen Colorado's pediatric AuD workforce, the program will expand trainees' exposure to rural pediatric practice through the *Rural Pediatric Audiology Partnership Rotation,* establish a statewide *Community of Practice* for peer consultation and continuing education, facilitate networking with pediatric employers, and use alumni feedback to continuously refine training in response to workforce needs.

COLLABORATION

The LEND AuD program will collaborate with Colorado's EHDI system and national EHDI partners to strengthen coordinated, family-centered systems of care that improve language acquisition and developmental outcomes for children who are DHH with co-occurring A/DD. Trainees will participate in interdisciplinary leadership, QI, TA, and workforce development activities that strengthen Colorado's EHDI system.

State and National Partnerships. LEND AuD collaborates with: (1) Pediatric AuD clinical practicum sites at CHCO and MDC and (2) CO-EHDI partners including the Colorado Department of Public Health and Environment, Colorado Department of Human Services, Colorado Department of Early Childhood, Colorado Department of Education, CoHears Coordinators, the Colorado Home Intervention Program (CHIP), Colorado Hands & Voices, pediatric providers, family organizations, the HRSA-funded EHDI National Network, AUCD, and other national EHDI organizations to improve screening, diagnosis, early intervention, family engagement, and coordinated care across the EHDI system.

Systems-Level Projects. LEND AuD and partners will improve or increase the number of referrals for diagnosis and early intervention, reduce loss to follow-up, strengthen developmental monitoring and earlier identification of A/DD, expand developmentally informed practice, tele-AuD, rural service delivery, support a statewide Pediatric AuD Community of Practice, use EHDI data for QI, and disseminate evidence-based resources. Faculty and trainees will participate in the Colorado EHDI Alliance, Colorado Infant Hearing Advisory Committee, TA, and partner-led QI projects to develop leadership and system-thinking competencies.

Impact. These collaborations will strengthen Colorado's pediatric AuD workforce and EHDI system by improving timely identification, diagnosis, early intervention, family engagement, care coordination, language acquisition, and developmental outcomes for children who are DHH with A/DD.

EVALUATION

The JFK Partners AuD LEND program will use the established JFK Partners LEND evaluation framework to assess program implementation, trainee outcomes, and project impact. Evaluation integrates HRSA-required reporting through the Discretionary Grants Information System (DGIS) and National Information Reporting System (NIRS) with program-specific measures to monitor progress toward project goals, objectives, and performance measures and support CQI. Evaluation findings will be used to refine curriculum, strengthen partnerships,

respond to emerging workforce needs, and improve trainee, workforce, and EHDI system outcomes. The evaluation is led by the JFK Partners Evaluator (40+ years of evaluation experience), in collaboration with the Training Coordinator, and the LEND Training Co-Director.

Trainee Progress will be assessed through ITPs, competency-based evaluations, faculty observation, Scholarly Projects, reflective activities, and trainee satisfaction. Measures include attainment of MCH Leadership Competencies, pediatric AuD competencies, and participation in interdisciplinary, clinical, leadership, and community-based experiences. LEND AuD also contributes data to the national LEND Program Quality Improvement (LPQI) network to benchmark outcomes and guide program improvement.

Graduate Outcomes and CQI. NIRS follow-up surveys conducted two and five years after training assess employment in pediatric AuD and the EHDI system, leadership roles within EHDI and professional organizations, implementation of evidence-based practices, and scholarly productivity, including publications, presentations, and educational resources. Program implementation, CE, TA, scholarly productivity, and faculty, family partner, and EHDI partner feedback are reviewed regularly to monitor progress toward project goals, objectives, and performance measures, identify emerging workforce needs, guide CQI, and inform annual DGIS reporting.

INNOVATION

An emerging need in pediatric AuD is improving access to accurate, timely hearing assessment for DHH children with co-occurring A/DD. Standard audiologic procedures often do not adequately address differences in cognition, communication, sensory processing, and behavior, contributing to inconclusive evaluations, delayed diagnosis, delayed intervention, and poorer language outcomes. Current JCIH guidance provides limited recommendations for adapting hearing assessments for children with developmental disabilities, highlighting an important gap in pediatric AuD practice.

The LEND AuD program will address this need by piloting a Developmentally Informed Pediatric Audiology Clinical Implementation Model within the DevPeds DHH Clinic at CHCO. Rather than learning developmentally informed assessment through didactic instruction alone, trainees will gain hands-on experience implementing developmentally informed hearing screening and assessment strategies in a pediatric clinic. The model aligns assessment with each child's developmental profile through caregiver interviews, adapted behavioral tasks, individualized reinforcement, visual supports, sensory accommodations, communication supports, and tailored assessment protocols that improve engagement, test accuracy, timely diagnosis, and access to intervention.

To build pediatric AuD workforce capacity, pilot practices will be disseminated through a statewide Pediatric Audiology Community of Practice and collaboration with Vanderbilt University, another leader in DHH clinical care. Faculty, trainees, psychologists, early intervention providers, and pediatric audiologists will refine and share developmentally informed practices, strengthen interdisciplinary referrals, support rural providers through consultation and tele-education. This innovation combines hands-on clinical implementation, workforce development, and systems change to improve pediatric AuD practice, strengthen the EHDI workforce, and improve language and developmental outcomes for DHH children with A/DD.

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Budget Narrative: July 1, 2026– June 30, 2031
A./B. Personnel. Requested support includes partial salary/benefit support for 12 months/year: 1. Project Director (Sandra Gabbard, PhD, .10 FTE); 2. D/HH+Autism Content Expert (Deborah Mood, PhD, .10 FTE); 3. Audiologist (Tammy Fredrickson, PhD .05 FTE) and 4. Training Co-Director (Dina Johnson, MA, .10 FTE). Personnel forecasted to receive a 3% salary increase/year. **(Yr1 $43,629; Yr2 $44,536; Yr3 $45,471; Yr4 $46,434; Yr5 $47,426). Benefits** are based on actual cost and vary with the person based on individual selection of options, but include health, life, dental, and disability insurance, and either TIAA retirement and social security or PERA depending on classification. The standard CU-Anschutz benefit rates: full-time faculty: 27.64%. **(Yr1 $8,289; Yr2 $8,538; Yr3 $8,794; Yr4 $9,058; Yr5 $9,330).**
C. Equipment. None requested.
D. Travel. Funds are requested for trainees & faculty to attend the EHDI conference. Eight trips at $1480/trip (airfare $700/person; hotel $275/night for 2 nights; per diem $230 for 3 days. Funds requested for 3 LT trainees & 1 faculty supervisor for two 2-day pediatric rural AuD clinical rotation at $776/trip (mileage $161, lodging $400, per diem $215) **(Y1-5 $11,840/yr).**
E. Trainee Support Costs. Stipends. Support requested for three LT trainees per yr (>300 hrs) $1000/mo. Requirements for LT trainees are pediatric AuD practicum, LEND coursework, monthly AuD-specific meetings/mentorship, completion of a scholarly project to disseminate in pediatric AuD, grand rounds, & interaction with interdisciplinary trainees. Support requested for 1 Advanced MT trainee (150-299 hours), who is a third-year AuD doctoral student at CU-Boulder. Requirements for MT trainees are a pediatric AuD practicum and completion of a pediatric scholarly project to increase exposure earlier in doctorate to pediatric AuD practices and systems. **(Y1-5 $39,151/yr)**
F. Other Direct Costs. Consultation funds requested for consultants with specific expertise to enhance curricula, support for rural pediatric AuD clinics hosting LT trainee rotations, speakers to enhance the Colorado pediatric AuD workforce, and expertise in how to implement developmentally appropriate practices within AuD clinics. **(Y1 $13,919; Y2 $12,456; Yr3 $12,943; Yr4 $13,382; Yr5 $11,769).**
Printing. Funds are requested for printing posters for EDHI Conference. **(Y1-5 $500/yr)**
Conference registration fees. Funds requested for 4 trainees ($325/pp) & 4 faculty ($500/pp) to attend the EDHI Conference to enhance pediatric AuD specialization. **(Y1-5 $3,300/year)**
Funds requested for **Materials** for developmentally informed practices in AuD clinics to collect more accurate testing data and provide individualized intervention. **(Y1-2 $4,000, Y3 $2,000)**
G. Total Direct Costs include allowable cost requests in the categories of Personnel, Subcontract, Travel, Trainee Support Costs, Consultation, Printing, Conference Registration, and Materials. **(Y1$139,534, Y2$139,558, Y3$139,584, Y4$139,611, Y5$139,639)**
H. Facilities and Administration of 8 percent is applied to the modified total direct costs. **(Y1$10,466, Y2$10,442, Y3$10,416, Y4$10,389, Y5$10,362)**
Subcontract to CU-Boulder .05 FTE, benefits, and 8% indirect costs for 12 mos/year (Lauren Pontis, PhD, Audiologist) to recruit and mentor medium- and short-term trainees to increase the number of pediatric audiologists with clinical & leadership skills/expertise in delivering care to children with autism/DD. **(Y1 $8,704, Y2 $9,034, Y3 $9,381, Y4 $9,743, Y5 $10,120)**
Justification for Level of Request. We request $150,000/yr to support a comprehensive program that will increase the number of LEND-trained pediatric AuD trainees, advance innovations that address emerging needs, and build workforce expertise in delivering care to deaf or hard of hearing (DHH) infants and young children with autism/DD and their families.

RESEARCH & RELATED BUDGET - Budget Period 1

OMB Number: 4040-0001
Expiration Date: 11/30/2025

*** UEI:** MW8JHK6ZYEX8 **Enter name of Organization:** University of Colorado Denver

*** Budget Type:** ☒ Project ☐ Subaward/Consortium **Budget Period: 1** *** Start Date:** 07/01/2026 *** End Date:** 06/30/2027

A. Senior/Key Person

Prefix	* First	Middle	* Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	* Requested Salary ($)	* Fringe Benefits ($)	* Funds Requested ($)
	Reaven		Judith						0.00	0.00	0.00

*** Project Role:** PI

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 0.00

B. Other Personnel

* Number of Personnel	* Project Role	Months Cal.	Months Acad.	Months Sum.	* Requested Salary ($)	* Fringe Benefits ($)	* Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
1	Sandra Gabbard LEND AuD Project Director & Audiologist	12.00			13,375.00	0.00	13,375.00
1	Deborah Mood DHH+ASD Expert	1.20			13,350.00	3,658.00	17,008.00
1	Tammy Fredrickson Audiologist	1.00			7,330.00	2,008.00	9,338.00
1	Dina Johnson Training Co-Director	1.20			9,573.00	2,623.00	12,196.00

4 **Total Number Other Personnel**

Total Other Personnel 51,917.00

Total Salary, Wages and Fringe Benefits (A+B) 51,917.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	* Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file	
Total Equipment	

D. Travel

	Funds Requested ($)
1. Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	18,044.00
2. Foreign Travel Costs	
Total Travel Cost	18,044.00

E. Participant/Trainee Support Costs

	Funds Requested ($)
1. Tuition/Fees/Health Insurance	
2. Stipends	
3. Travel	
4. Subsistence	
5. Other	
Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	
2.	Publication Costs	
3.	Consultant Services	
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	8,704.00
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.	Consultation	13,919.00
9.	Printing (trainee posters)	500.00
10.	Conference Registration -- EHDI Conference	3,300.00
11.	Materials for Developmentally Informed Practices	4,000.00
12.	Pre-doc Trainee (LT >300 hours); Pre-doc Trainee (Advanced MT 150-299 hours)	39,150.00
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	69,573.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	139,534.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	* Funds Requested ($)
MTDC	8.00	130,830.00	10,466.00
		Total Indirect Costs	10,466.00

Cognizant Federal Agency (Agency Name, POC Name, and POC Phone Number): DHHS, Arif Karim, (415) 437-7820

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	150,000.00

J. Fee

	Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	150,000.00

L. * Budget Justification

(Only attach one file.) Add Attachment Delete Attachment View Attachment

RESEARCH & RELATED BUDGET - Budget Period 2

OMB Number: 4040-0001
Expiration Date: 11/30/2025

*** UEI:** MW8JHK6ZYEX8 **Enter name of Organization:** University of Colorado Denver

*** Budget Type:** ☒ Project ☐ Subaward/Consortium **Budget Period: 2** *** Start Date:** 07/01/2027 *** End Date:** 06/30/2028

A. Senior/Key Person

Prefix	* First	Middle	* Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	* Requested Salary ($)	* Fringe Benefits ($)	* Funds Requested ($)
	Reaven		Judith						0.00	0.00	0.00

*** Project Role:** PI

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 0.00

B. Other Personnel

* Number of Personnel	* Project Role	Months Cal.	Months Acad.	Months Sum.	* Requested Salary ($)	* Fringe Benefits ($)	* Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
1	Sandra Gabbard LEND AuD Project Director & Audiologist	12.00			13,375.00	0.00	13,375.00
1	Deborah Mood DHH+ASD Expert	1.20			13,750.00	3,768.00	17,518.00
1	Tammy Fredrickson Audiologist	1.00			7,550.00	2,069.00	9,619.00
1	Dina Johnson Training Co-Director	1.20			9,860.00	2,702.00	12,562.00

4 **Total Number Other Personnel**

Total Other Personnel 53,074.00

Total Salary, Wages and Fringe Benefits (A+B) 53,074.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	* Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

		Funds Requested ($)
1.	Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	18,044.00
2.	Foreign Travel Costs	
	Total Travel Cost	18,044.00

E. Participant/Trainee Support Costs

		Funds Requested ($)
1.	Tuition/Fees/Health Insurance	
2.	Stipends	
3.	Travel	
4.	Subsistence	
5.	Other	
	Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	
2.	Publication Costs	
3.	Consultant Services	
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	9,034.00
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.	Consultation	12,456.00
9.	Printing (trainee posters)	500.00
10.	Conference Registration -- EHDI Conference	3,300.00
11.	Materials for Developmentally Informed Practices	4,000.00
12.	Pre-doc Trainee (LT >300 hours); Pre-doc Trainee (Advanced MT 150-299 hours)	39,150.00
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	68,440.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	139,558.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	* Funds Requested ($)
MTDC	8.00	130,524.00	10,442.00
		Total Indirect Costs	10,442.00

Cognizant Federal Agency (Agency Name, POC Name, and POC Phone Number): DHHS, Arif Karim, (415) 437-7820

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	150,000.00

J. Fee

	Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	150,000.00

L. * Budget Justification

(Only attach one file.) Add Attachment Delete Attachment View Attachment

RESEARCH & RELATED BUDGET - Budget Period 3

OMB Number: 4040-0001
Expiration Date: 11/30/2025

*** UEI:** MW8JHK6ZYEX8 **Enter name of Organization:** University of Colorado Denver

*** Budget Type:** ☒ Project ☐ Subaward/Consortium **Budget Period: 3** *** Start Date:** 07/01/2028 *** End Date:** 06/30/2029

A. Senior/Key Person

Prefix	* First	Middle	* Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	* Requested Salary ($)	* Fringe Benefits ($)	* Funds Requested ($)
	Reaven		Judith						0.00	0.00	0.00

*** Project Role:** PI

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 0.00

B. Other Personnel

* Number of Personnel	* Project Role	Months Cal.	Months Acad.	Months Sum.	* Requested Salary ($)	* Fringe Benefits ($)	* Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
1	Sandra Gabbard LEND AuD Project Director & Audiologist	12.00			13,375.00	0.00	13,375.00
1	Deborah Mood DHH+ASD Expert	1.20			14,163.00	3,881.00	18,044.00
1	Tammy Fredrickson Audiologist	1.00			7,777.00	2,131.00	9,908.00
1	Dina Johnson Training Co-Director	1.20			10,156.00	2,783.00	12,939.00

4 **Total Number Other Personnel**

Total Other Personnel 54,266.00

Total Salary, Wages and Fringe Benefits (A+B) 54,266.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	* Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file	
Total Equipment	

D. Travel

	Funds Requested ($)
1. Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	18,044.00
2. Foreign Travel Costs	
Total Travel Cost	18,044.00

E. Participant/Trainee Support Costs

	Funds Requested ($)
1. Tuition/Fees/Health Insurance	
2. Stipends	
3. Travel	
4. Subsistence	
5. Other	
Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	
2.	Publication Costs	
3.	Consultant Services	
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	9,381.00
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.	Consultation	12,943.00
9.	Printing (trainee posters)	500.00
10.	Conference Registration -- EHDI Conference	3,300.00
11.	Materials for Developmentally Informed Practices	2,000.00
12.	Pre-doc Trainee (LT >300 hours); Pre-doc Trainee (Advanced MT 150-299 hours)	39,150.00
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	67,274.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	139,584.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	* Funds Requested ($)
MTDC	8.00	130,203.00	10,416.00
		Total Indirect Costs	10,416.00

Cognizant Federal Agency (Agency Name, POC Name, and POC Phone Number): DHHS, Arif Karim, (415) 437-7820

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	150,000.00

J. Fee

	Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	150,000.00

L. * Budget Justification

(Only attach one file.) Add Attachment Delete Attachment View Attachment

RESEARCH & RELATED BUDGET - Budget Period 4

OMB Number: 4040-0001
Expiration Date: 11/30/2025

*** UEI:** MW8JHK6ZYEX8 **Enter name of Organization:** University of Colorado Denver

*** Budget Type:** ☒ Project ☐ Subaward/Consortium **Budget Period: 4** *** Start Date:** 07/01/2029 *** End Date:** 06/30/2030

A. Senior/Key Person

Prefix	* First	Middle	* Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	* Requested Salary ($)	* Fringe Benefits ($)	* Funds Requested ($)
	Reaven		Judith						0.00	0.00	0.00

*** Project Role:** PI

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 0.00

B. Other Personnel

* Number of Personnel	* Project Role	Months Cal.	Months Acad.	Months Sum.	* Requested Salary ($)	* Fringe Benefits ($)	* Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
1	Sandra Gabbard LEND AuD Project Director & Audiologist	12.00			13,375.00	0.00	13,375.00
1	Deborah Mood DHH+ASD Expert	1.20			14,588.00	3,997.00	18,585.00
1	Tammy Fredrickson Audiologist	1.00			8,010.00	2,195.00	10,205.00
1	Dina Johnson Training Co-Director	1.20			10,461.00	2,866.00	13,327.00

4 **Total Number Other Personnel**

Total Other Personnel 55,492.00

Total Salary, Wages and Fringe Benefits (A+B) 55,492.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	* Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

		Funds Requested ($)
1.	Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	18,044.00
2.	Foreign Travel Costs	
	Total Travel Cost	18,044.00

E. Participant/Trainee Support Costs

		Funds Requested ($)
1.	Tuition/Fees/Health Insurance	
2.	Stipends	
3.	Travel	
4.	Subsistence	
5.	Other	
	Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	
2.	Publication Costs	
3.	Consultant Services	
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	9,743.00
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.	Consultation	13,382.00
9.	Printing (trainee posters)	500.00
10.	Conference Registration -- EHDI Conference	3,300.00
11.		
12.	Pre-doc Trainee (LT >300 hours); Pre-doc Trainee (Advanced MT 150-299 hours)	39,150.00
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	66,075.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	139,611.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	* Funds Requested ($)
MTDC	8.00	129,868.00	10,389.00
		Total Indirect Costs	10,389.00

Cognizant Federal Agency (Agency Name, POC Name, and POC Phone Number): DHHS, Arif Karim, (415) 437-7820

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	150,000.00

J. Fee

	Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	150,000.00

L. * Budget Justification

(Only attach one file.) Add Attachment Delete Attachment View Attachment

RESEARCH & RELATED BUDGET - Budget Period 5

OMB Number: 4040-0001
Expiration Date: 11/30/2025

*** UEI:** MW8JHK6ZYEX8 **Enter name of Organization:** University of Colorado Denver

*** Budget Type:** ☒ Project ☐ Subaward/Consortium **Budget Period: 5** *** Start Date:** 07/01/2029 *** End Date:** 06/30/2031

A. Senior/Key Person

Prefix	* First	Middle	* Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	* Requested Salary ($)	* Fringe Benefits ($)	* Funds Requested ($)
	Reaven		Judith						0.00	0.00	0.00

*** Project Role:** PI

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 0.00

B. Other Personnel

* Number of Personnel	* Project Role	Months Cal.	Months Acad.	Months Sum.	* Requested Salary ($)	* Fringe Benefits ($)	* Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
1	Sandra Gabbard LEND AuD Project Director & Audiologist	12.00			13,375.00	0.00	13,375.00
1	Deborah Mood DHH+ASD Expert	1.20			15,026.00	4,117.00	19,143.00
1	Tammy Fredrickson Audiologist	1.00			8,250.00	2,261.00	10,511.00
1	Dina Johnson Training Co-Director	1.20			10,775.00	2,952.00	13,727.00

4 **Total Number Other Personnel**

Total Other Personnel 56,756.00

Total Salary, Wages and Fringe Benefits (A+B) 56,756.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	* Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

		Funds Requested ($)
1.	Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	18,044.00
2.	Foreign Travel Costs	
	Total Travel Cost	18,044.00

E. Participant/Trainee Support Costs

		Funds Requested ($)
1.	Tuition/Fees/Health Insurance	
2.	Stipends	
3.	Travel	
4.	Subsistence	
5.	Other	
	Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	
2.	Publication Costs	
3.	Consultant Services	
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	10,120.00
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.	Consultation	11,769.00
9.	Printing (trainee posters)	500.00
10.	Conference Registration -- EHDI Conference	3,300.00
11.		0.00
12.	Pre-doc Trainee (LT >300 hours); Pre-doc Trainee (Advanced MT 150-299 hours)	39,150.00
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	64,839.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	139,639.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	* Funds Requested ($)
MTDC	8.00	129,519.00	10,361.00
		Total Indirect Costs	10,361.00

Cognizant Federal Agency (Agency Name, POC Name, and POC Phone Number): DHHS, Arif Karim, (415) 437-7820

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	150,000.00

J. Fee

	Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	150,000.00

L. * Budget Justification

(Only attach one file.) Add Attachment Delete Attachment View Attachment

RESEARCH & RELATED BUDGET - Cumulative Budget

		Totals ($)
Section A, Senior/Key Person		0.00
Section B, Other Personnel		271,505.00
Total Number Other Personnel	20	
Total Salary, Wages and Fringe Benefits (A+B)		271,505.00
Section C, Equipment		
Section D, Travel		90,220.00
1. Domestic	90,220.00	
2. Foreign		
Section E, Participant/Trainee Support Costs		
1. Tuition/Fees/Health Insurance		
2. Stipends		
3. Travel		
4. Subsistence		
5. Other		
6. Number of Participants/Trainees		
Section F, Other Direct Costs		336,201.00
1. Materials and Supplies		
2. Publication Costs		
3. Consultant Services		
4. ADP/Computer Services		
5. Subawards/Consortium/Contractual Costs	46,982.00	
6. Equipment or Facility Rental/User Fees		
7. Alterations and Renovations		
8. Other 1	64,469.00	
9. Other 2	2,500.00	
10. Other 3	16,500.00	
11. Other 4	10,000.00	
12. Other 5	195,750.00	
13. Other 6		
14. Other 7		
15. Other 8		
16. Other 9		
17. Other 10		

Section G, Direct Costs (A thru F)	697,926.00
Section H, Indirect Costs	52,074.00
Section I, Total Direct and Indirect Costs (G + H)	750,000.00
Section J, Fee	
Section K, Total Costs and Fee (I + J)	750,000.00

RESEARCH & RELATED BUDGET - Budget Period 1

OMB Number: 4040-0001
Expiration Date: 11/30/2025

*** UEI:** SPVKK1RC2MZ3 **Enter name of Organization:** University of Colorado Boulder

*** Budget Type:** ☐ Project ☒ Subaward/Consortium **Budget Period: 1** *** Start Date:** 07/01/2026 *** End Date:** 06/30/2027

A. Senior/Key Person

Prefix	* First	Middle	* Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	* Requested Salary ($)	* Fringe Benefits ($)	* Funds Requested ($)
	Lauren		Pontis		111,830.00	0.61			5,688.00	2,370.00	8,058.00

*** Project Role:** Site PI

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 8,058.00

B. Other Personnel

* Number of Personnel	* Project Role	Months Cal.	Months Acad.	Months Sum.	* Requested Salary ($)	* Fringe Benefits ($)	* Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						

Total Number Other Personnel

Total Other Personnel

Total Salary, Wages and Fringe Benefits (A+B) 8,058.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	* Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

	Funds Requested ($)
1. Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	
2. Foreign Travel Costs	
Total Travel Cost	

E. Participant/Trainee Support Costs

	Funds Requested ($)
1. Tuition/Fees/Health Insurance	
2. Stipends	
3. Travel	
4. Subsistence	
5. Other	
Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	
2.	Publication Costs	
3.	Consultant Services	
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.		
9.		
10.		
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	8,058.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	* Funds Requested ($)
MTDC	8.00	8,058.00	646.00
		Total Indirect Costs	646.00

Cognizant Federal Agency (Agency Name, POC Name, and POC Phone Number): DHHS, Arif Karim, (415) 437-7820

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	8,704.00

J. Fee

	Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	8,704.00

L. * Budget Justification

(Only attach one file.) Add Attachment | Delete Attachment | View Attachment

RESEARCH & RELATED BUDGET - Budget Period 2

OMB Number: 4040-0001
Expiration Date: 11/30/2025

*** UEI:** SPVKK1RC2MZ3 **Enter name of Organization:** University of Colorado Boulder

*** Budget Type:** ☐ Project ☒ Subaward/Consortium **Budget Period: 2** *** Start Date:** 07/01/2027 *** End Date:** 06/30/2028

A. Senior/Key Person

Prefix	* First	Middle	* Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	* Requested Salary ($)	* Fringe Benefits ($)	* Funds Requested ($)
	Lauren		Pontis		115,408.00	0.61			5,869.00	2,496.00	8,365.00

*** Project Role:** Site PI

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 8,365.00

B. Other Personnel

* Number of Personnel	* Project Role	Months Cal.	Months Acad.	Months Sum.	* Requested Salary ($)	* Fringe Benefits ($)	* Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						

Total Number Other Personnel

Total Other Personnel

Total Salary, Wages and Fringe Benefits (A+B) 8,365.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	* Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

	Funds Requested ($)
1. Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	
2. Foreign Travel Costs	
Total Travel Cost	

E. Participant/Trainee Support Costs

	Funds Requested ($)
1. Tuition/Fees/Health Insurance	
2. Stipends	
3. Travel	
4. Subsistence	
5. Other	
Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	
2.	Publication Costs	
3.	Consultant Services	
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
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	Total Other Direct Costs	

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	8,365.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	* Funds Requested ($)
MTDC	8.00	8,365.00	669.00
		Total Indirect Costs	669.00

Cognizant Federal Agency (Agency Name, POC Name, and POC Phone Number): DHHS, Arif Karim, (415) 437-7820

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	9,034.00

J. Fee

	Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	9,034.00

L. * Budget Justification

(Only attach one file.) Add Attachment Delete Attachment View Attachment

RESEARCH & RELATED BUDGET - Budget Period 3

OMB Number: 4040-0001
Expiration Date: 11/30/2025

*** UEI:** SPVKK1RC2MZ3 **Enter name of Organization:** University of Colorado Boulder

*** Budget Type:** ☐ Project ☒ Subaward/Consortium **Budget Period: 3** *** Start Date:** 07/01/2028 *** End Date:** 06/30/2029

A. Senior/Key Person

Prefix	* First	Middle	* Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	* Requested Salary ($)	* Fringe Benefits ($)	* Funds Requested ($)
	Lauren		Pontis		119,101.00	0.61			6,056.00	2,630.00	8,686.00

*** Project Role:** Site PI

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 8,686.00

B. Other Personnel

* Number of Personnel	* Project Role	Months Cal.	Months Acad.	Months Sum.	* Requested Salary ($)	* Fringe Benefits ($)	* Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						

Total Number Other Personnel

Total Other Personnel

Total Salary, Wages and Fringe Benefits (A+B) 8,686.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	* Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

	Funds Requested ($)
1. Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	
2. Foreign Travel Costs	
Total Travel Cost	

E. Participant/Trainee Support Costs

	Funds Requested ($)
1. Tuition/Fees/Health Insurance	
2. Stipends	
3. Travel	
4. Subsistence	
5. Other	
Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	
2.	Publication Costs	
3.	Consultant Services	
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
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	Total Other Direct Costs	

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	8,686.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	* Funds Requested ($)
MTDC	8.00	8,686.00	695.00
		Total Indirect Costs	695.00

Cognizant Federal Agency (Agency Name, POC Name, and POC Phone Number): DHHS, Arif Karim, (415) 437-7820

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	9,381.00

J. Fee

	Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	9,381.00

L. * Budget Justification

(Only attach one file.) Add Attachment Delete Attachment View Attachment

RESEARCH & RELATED BUDGET - Budget Period 4

OMB Number: 4040-0001
Expiration Date: 11/30/2025

*** UEI:** SPVKK1RC2MZ3 **Enter name of Organization:** University of Colorado Boulder

*** Budget Type:** ☐ Project ☒ Subaward/Consortium **Budget Period: 4** *** Start Date:** 07/01/2029 *** End Date:** 06/30/2030

A. Senior/Key Person

Prefix	* First	Middle	* Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	* Requested Salary ($)	* Fringe Benefits ($)	* Funds Requested ($)
	Lauren		Pontis		122,912.00	0.61			6,250.00	2,771.00	9,021.00

*** Project Role:** Site PI

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 9,021.00

B. Other Personnel

* Number of Personnel	* Project Role	Months Cal.	Months Acad.	Months Sum.	* Requested Salary ($)	* Fringe Benefits ($)	* Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						

Total Number Other Personnel

Total Other Personnel

Total Salary, Wages and Fringe Benefits (A+B) 9,021.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	* Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

	Funds Requested ($)
1. Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	
2. Foreign Travel Costs	
Total Travel Cost	

E. Participant/Trainee Support Costs

	Funds Requested ($)
1. Tuition/Fees/Health Insurance	
2. Stipends	
3. Travel	
4. Subsistence	
5. Other	
Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	
2.	Publication Costs	
3.	Consultant Services	
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
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	Total Other Direct Costs	

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	9,021.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	* Funds Requested ($)
MTDC	8.00	9,021.00	722.00
		Total Indirect Costs	722.00

Cognizant Federal Agency (Agency Name, POC Name, and POC Phone Number): DHHS, Arif Karim, (415) 437-7820

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	9,743.00

J. Fee

	Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	9,743.00

L. * Budget Justification

(Only attach one file.) Add Attachment Delete Attachment View Attachment

RESEARCH & RELATED BUDGET - Budget Period 5

OMB Number: 4040-0001
Expiration Date: 11/30/2025

*** UEI:** SPVKK1RC2MZ3 **Enter name of Organization:** University of Colorado Boulder

*** Budget Type:** ☐ Project ☒ Subaward/Consortium **Budget Period: 5** *** Start Date:** 07/01/2030 *** End Date:** 06/30/2031

A. Senior/Key Person

Prefix	* First	Middle	* Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	* Requested Salary ($)	* Fringe Benefits ($)	* Funds Requested ($)
	Lauren		Pontis		126,845.00	0.61			6,450.00	2,920.00	9,370.00

*** Project Role:** Site PI

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 9,370.00

B. Other Personnel

* Number of Personnel	* Project Role	Months Cal.	Months Acad.	Months Sum.	* Requested Salary ($)	* Fringe Benefits ($)	* Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						

Total Number Other Personnel

Total Other Personnel

Total Salary, Wages and Fringe Benefits (A+B) 9,370.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	* Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

	Funds Requested ($)
1. Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	
2. Foreign Travel Costs	
Total Travel Cost	

E. Participant/Trainee Support Costs

	Funds Requested ($)
1. Tuition/Fees/Health Insurance	
2. Stipends	
3. Travel	
4. Subsistence	
5. Other	
Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	
2.	Publication Costs	
3.	Consultant Services	
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
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17.		
	Total Other Direct Costs	

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	9,370.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	* Funds Requested ($)
MTDC	8.00	9,370.00	750.00
		Total Indirect Costs	750.00

Cognizant Federal Agency (Agency Name, POC Name, and POC Phone Number): DHHS, Arif Karim, (415) 437-7820

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	10,120.00

J. Fee

	Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	10,120.00

L. * Budget Justification

(Only attach one file.) Add Attachment Delete Attachment View Attachment

RESEARCH & RELATED BUDGET - Cumulative Budget

		Totals ($)
Section A, Senior/Key Person		43,500.00
Section B, Other Personnel		
Total Number Other Personnel		
Total Salary, Wages and Fringe Benefits (A+B)		43,500.00
Section C, Equipment		
Section D, Travel		
1. Domestic		
2. Foreign		
Section E, Participant/Trainee Support Costs		
1. Tuition/Fees/Health Insurance		
2. Stipends		
3. Travel		
4. Subsistence		
5. Other		
6. Number of Participants/Trainees		
Section F, Other Direct Costs		
1. Materials and Supplies		
2. Publication Costs		
3. Consultant Services		
4. ADP/Computer Services		
5. Subawards/Consortium/Contractual Costs		
6. Equipment or Facility Rental/User Fees		
7. Alterations and Renovations		
8. Other 1		
9. Other 2		
10. Other 3		
11. Other 4		
12. Other 5		
13. Other 6		
14. Other 7		
15. Other 8		
16. Other 9		
17. Other 10		

Section G, Direct Costs (A thru F)	43,500.00
Section H, Indirect Costs	3,482.00
Section I, Total Direct and Indirect Costs (G + H)	46,982.00
Section J, Fee	
Section K, Total Costs and Fee (I + J)	46,982.00

JFK Partners LEND Budget Narrative 2026-2031

A. Personnel - Non-Trainee Expenses. Requested support includes partial salary and benefits for 16 faculty with faculty appointments at CU-SOM. Positions include: 1) Project Director (J. Reaven, PhD, .30 FTE on LEND as required). Dr. Reaven serves as Project Director, core curriculum lead and represents the Psychology Discipline Director; 2) Training Co-Director, Ed/SpEd Discipline Director, Co-lead Key Concepts, Leadership, and Lead-Policy Course (D. Johnson, MA Yr1-4 .40 and Yr5 .322 on LEND); 3) Training Co-Director, Speech-Language Pathology Discipline Director, Co-lead Key Concepts Course, Lead Intervention Course (R. Charlifue-Smith, MA, CCC-SLP .10 on LEND) 4) Screening & Assessment Course Co-lead (A. Blakeley-Smith, PhD .05 on LEND); 5) Screening and Assessment Course Co-lead (A. Reynolds, MD .025 on LEND); 6) Research Manager (V. Postorino, PhD .10 FTE on LEND); 7-15) Discipline Directors/Content Experts: Occupational Therapy (TBA, .10 on LEND); Nutrition (L. Diab, MD .05 FTE on LEND); Self-Advocacy (R. Ennis, DSW .10 FTE on LEND); Family (K. Kaiser, MA, MPH .10 FTE on LEND); Physical Therapy (M. Hernandez, PT, DPT, EdD, PCS), .10 FTE on LEND); Public Health (K. Hightshoe, MSPH, .10 FTE on LEND); Social Work (S. McSwegin, LCSW .10 FTE on LEND); Developmental Behavioral Pediatrics (V. Novoa, MD .10 FTE on LEND); and Rehabilitation Counseling (TBA pending Project Officer approval) TBA .10 FTE on LEND; 16) Evaluation (G. Steinke, PhD, .20 FTE on LEND); Years 2 through 5 include a 3% cost of living increase in salary for all personnel. Named personnel have additional university or other funding. Audiology faculty on the Supplement contribute to the curriculum by providing instruction about the CO-EHDI program for all disciplines (S. Gabbard, PhD; D. Mood, PhD; & T. Fredrickson, PhD). (Total Disciplines=11, not including Audiology or Nutrition due to faculty being a pediatrician and not duplicated in discipline count.

B. Other Personnel. Support is also requested for Training Coordinator (C. Cook Yrs1-5: .40 on LEND), Exempt Professional. **Fringe Benefits** are based on actual cost and vary with the person based on individual selection of options, but include health, life, dental, and disability insurance, and either TIAA retirement and social security or Public Employee Retirement depending on classification. The standard benefit rates: full-time Professors, Assoc Professors, Asst Professors, Instructors: 27.64%; Part-time faculty (50% or less): 14.42%; Exempt Prof Staff: 40.30%. (Total Personnel Y1 $368,703, Y2 $382,505, Y3 $395,785, Y4 $405,936 , Y5 $403,238.)

D. Travel. Funds requested for Project Director to attend grantee meetings and Autism CARES meeting every other year as specified in the guidance. Funds requested for 6 faculty/trainees to travel to the AUCD Conf and/or Disability Policy Seminar. Air travel $400/person. Hotel, ground transportation, & related travel expenses are typically $1,200/person for 3-day conf. Local mileage 1000 miles at .50/mile are requested for faculty in-state travel to meetings. Funds requested for trainee travel to scholarly conf/mtgs where trainees present research, engage in interdisciplinary leadership development, and enhance competencies aligned with LEND program goals. (Y1&Y2 $17,300/year; Y3 $8,500; Y4 $5,300; Y5 $5,300)

E. Trainee Support Costs. Pre-doctoral Fellows – Support requested for 11 part-time .30 FTE on LEND (>300 hours) stipends of $7,341 each using the 2026-NRSA predoc base of $29,364; 3 full-time salaried positions (1,800 hours+) include: 1) Developmental Behavioral Pediatric Fellow at .20 FTE on LEND using the 2026 NIH-PGY-4 base of $69,180, 2) Pediatric Physical Therapy Resident .50 FTE on LEND using the 2026 NIH-PGY0 base of $63,480, and 3) Psychology Intern .65 FTE on LEND using 2026-NRSA pre-doctoral base of $29,364 to complete LEND-specific curricula, including leadership training. These 3 full-time trainees receive additional university or other funds to make up their 1.0 FTE salary. The three full-time

trainee positions are eligible for fringe benefits which are based on actual cost and vary with the person based on individual selection of options, but include health, life, dental, and disability insurance, and either TIAA retirement and social security or Public Employee Retirement depending on classification. The standard benefit rates for one DBP GME-fellow: 26.50% and two full-time Instructor Fellows: 27.64%.Trainee eligibility requirements for these stipends and salaries will be met by ensuring that funds from this award are used only for the training purposes for which this grant is awarded. The following assurances apply: 1) trainees are U.S. citizens, lawfully admitted for permanent U.S. residence, or otherwise classified as "qualified aliens"; 2) there is no duplication of hours or activities for other financial support related to training or employment for the same hours; 3) stipend/salary support does not exceed allowable thresholds; 4) trainees represent approved disciplines listed in the NOFO; 5) trainees are enrolled in a graduate program or have completed the educational preparation required by their discipline, including licensure if it is a prerequisite of the profession, with the exception of individuals with disabilities and family members, for whom there is no educational requirement for participation; 6) no single trainee discipline will make up more than 25% of the LEND cohort; and 7) trainees are interested in serving children, youth, and adults with autism/DD when they enter the workforce. Trainee Support Costs total of $174,464/yr.

F. Other Direct Costs. Consultation. Funds are requested for consultants with specific expertise to provide curricula enhancements pertaining to adult topics e.g., transition to adulthood and customized employment. Incentives for families/self-advocates serving as Family Mentors, speaking in courses or serving as panelists are requested. These families provide expertise in content not otherwise represented among faculty. Youth with autism/DD, as well as family members, will be involved in presentation of parts of our curricula. Compensation for these activities will be at a standard rate of $50 per hour. (Y1-Y5 $1950/yr) **Instructional materials**. Leadership-building self-reflection assessment tools and other instructional materials are requested. (Y1 $5,013, Y2 $3,825, Y3-5 $1,000/yr); **Printing/Graphic Design.** Funds are requested to assist in printing and/or creating high-quality products developed by LEND faculty and trainees. (Y1 $5,075, Y2 $933, Y3 $1,799, Y4 $351, Y5 $263); **Conference reg fees**. Funds are requested to support trainees & faculty to attend national, state, and local conferences to enhance their specialization in A/DD such as Disability Policy Seminar. Travel to scholarly conferences where faculty/trainees present research and engage in interdisciplinary leadership development. (Y1-2 $7,230, Y3 $4,620 , Y4-5 $2,920)

G. Direct Costs. Total Direct Costs include allowable cost requests in the categories of Personnel, Other Personnel, Travel, Trainee Support Costs, Consultation, Instructional Materials, Printing, and Conference Registration for a total of $593,519 per year.

H. Facilities & Admin of 8 percent indicated in the NOFO has been applied to the Total Direct Costs for a total of $47,481 per year.

Justification for Level of Request. We request Tier 3 funding of $641,000 per year for 18 long-term trainees (≥300 hours). Trainees represent 10 different disciplines and faculty represent 11 disciplines, thereby meeting the Tier 3 requirements.